Growing Together

access Care Planning

A Guide to Assessment, Care Planning, Writing Care Plans on Access Care Planning Application

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Forward

The purpose of this document is to give a guide to managers, Care Co-ordinators and area supervisors, on how to assess care, devise care plans and implement and review.

Regulation 9 – Person Centred Care

A clear care and/or treatment plan, which includes agreed goals, must be developed and made available to all staff and others involved in providing the care. Where relevant, the plan should include ways in which the person can maintain their independence. Plans should include an agreed review date.

The above is the CQC [Care Quality Commission] – statement on care plans and comes from the *Health and Social Care Act 2008 – 2022*. [Appendix One]

A care plan is not a one size fits all document and should be personalised to the Service User. Therefore, cutting and pasting, from one to another should be avoided, unless adding the bits that are relevant to the person being cared for.

The document will also explain how to use the Care Planner application and give a good practice reference guide on how to construct and lay out the care plans for Service Users and Staff alike.

This manual is designed to be used as a reference manual, and follows policy, procedure and good practice in relation to providing care.

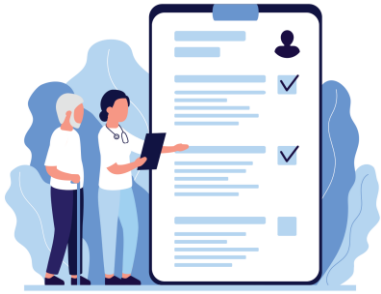
If you require any further help or guidance in developing care plans feel free to contact you Training Manager, Operations Manager, compliance manager, or Registered Manager for Support.

I hope you find this guide useful. Any comments for improvements or additional guidance you would like included, please do not hesitate to contact me andrew.charles@crtcare.co.uk to discuss.



Andy Charles. 😊

What is a Care Plan?



A Care Plan is a document that demonstrates how to care for the person. This is done in a person centred and should be devised from their wants and wishes [this should be gained from the initial assessment that is carried out when the package of care is accepted.]

We have two common types of care plan that we will deal with the first type of care plan is a general care plan that covers the tasks and support that a person needs whilst the carers are in the visit. The second type of care plan is the Advanced Care Plan. This is a care plan that supports the person through end-of-life care.



The care plan should give staff the relevant information to enable them to care for the people they are sent to. On the Care Planner Application that we use the whole element of the visit is shown when carers log in to the application and they will see a list of all tasks including medication that needs to be carried out whilst they are with the individual.

The care plan should be written with the person at the centre, family members and other professionals i.e. physiotherapists, occupational therapists or District Nurses / General Practitioners [Doctors] having an input in to the process. It is important to remember that the care plan is the persons and not ours. This is why asking at the initial assessment how a person would like something done is so important. Also allowing the person to be involved in their care gives them ownership and promotes their independence.

This is why the basis of the best care plans come from a robust assessment that gathers all the relevant information. We need to be careful as sometimes documentation that is received from the local Authority is not always accurate, and cutting and pasting goes on. This is why the initial assessment / visit is important to ensure that carers have the correct information and know the level of care that the person requires. There are two ways of carrying out the assessment there is one that is built in to the care planner app. This assessment is good but basic, and you may not capture enough information to make the care plan robust. There is also a word-based document which gives you more information. [Appendices Two]

This will let you capture things like which items to use for personal care such as shower gel, shampoo, perfume or deodorant that the person may whish to use.

The Care plan should be easy to read and not overload the carer as there maybe multiple task and activities that need to be carried out.

Gathering Information for the Care Plan

This should be done face to face with the person being cared for [where the person has dementia, it maybe necessary to have family members, advocate, friends (if authorised to do so, think GDPR) present to fill in gaps that the person may struggle with.]

Ideally the initial assessment should be carried out when the package is accepted or within 72 hours of the package starting.



As discussed on page two we can carry out the assessment and gather the relevant information to ensure that we create a Care Plan that is person Centred and enables the carers to deliver quality care in a way that the person requires it.

We also use this assessment process to carry out the health and safety aspect of the care. This will include Risk Assessments, of the environment, knowing where emergency cut offs such as electricity, water or gas are located. Identifying problems with the flooring, fire hazards i.e., hording, unclean / unkept environments, Hazards towards mobility such as holes in the carpet or objects in the way such as coffee tables.

It is important to remember that we need to gather information on the items used in care such as the hoist [has it been serviced], Slings [have they been tested], and when are they next due to be tested.



Equipment such as slings and hoist should be LOLLER [Lifting Operations and Lifting Equipment Regulation 1998] checked as a minimum every 12 months. Profiling beds should also be safety checked every year, and this recorded in the care plan.

The main purpose of the care plan is to inform the carer how the individual being supported wishes to be cared for and the level of detail should ensure that care is delivered in a person centred way and in the safest way possible.

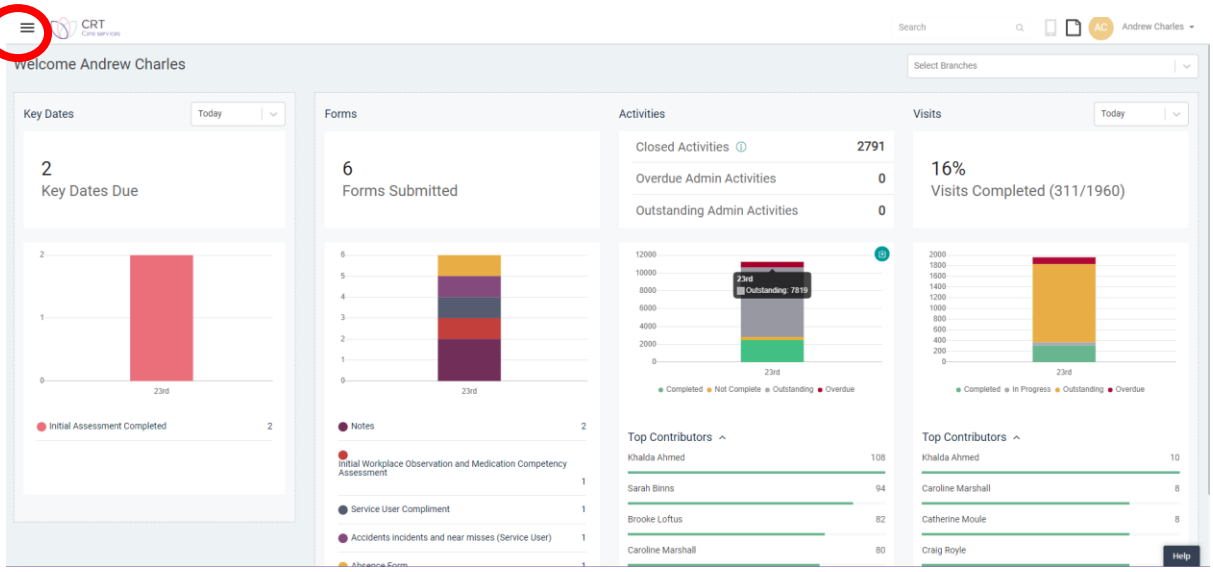
This is why carrying out the assessment and gathering as much detail about the person is invaluable that we get to know the person, their wishes wants and more importantly the care is delivered in a way that promotes independence and person centred based care.



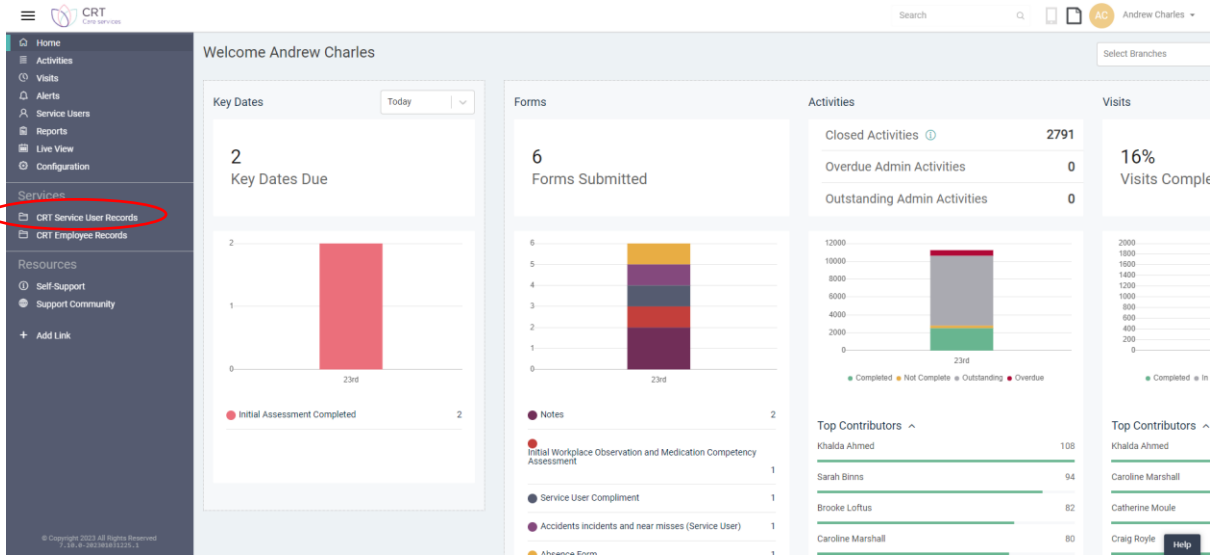
To clarify information at the start of the package we can gather essential information via telephone and then clarify when the full assessment is carried out. We should only use the telephone call to check essential information if the assessment is not being carried out before the care starts. A telephone call can not let you gather all the essential information to ensure that the care plan is robust, and risk assessments identify all hazards and how to overcome them. If you have not been into the property and met with the individual and seen there living environment how can you ensure that your care plans and risk assessments are accurate and meet the persons needs.

We can use the online version or the paper version of the assessment.

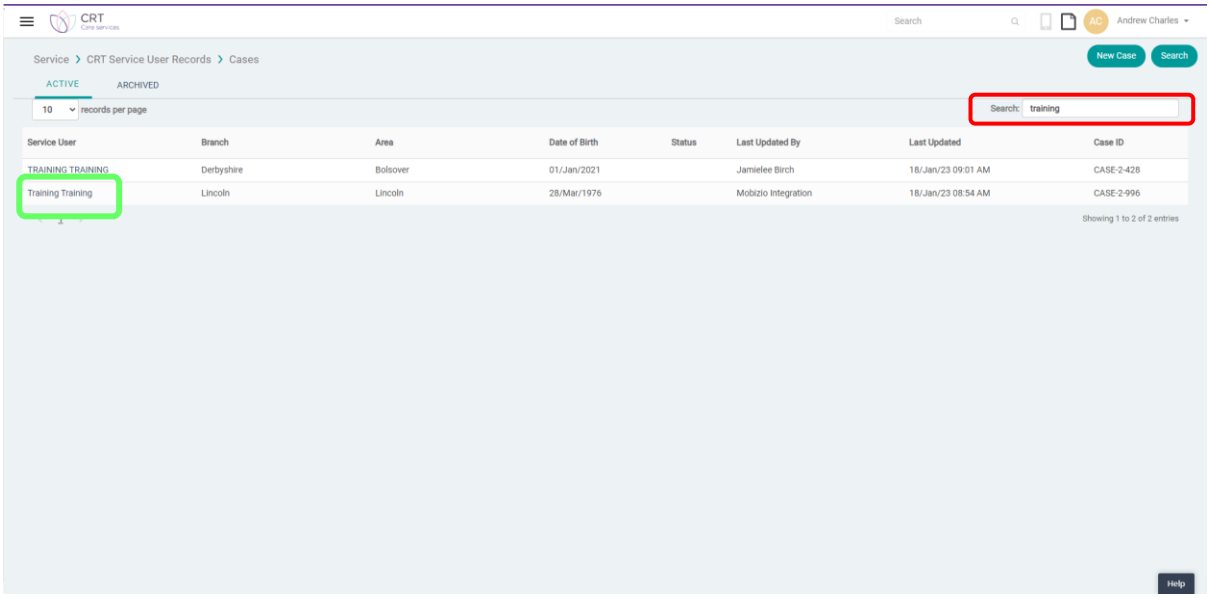
The paper version can then be scanned and added to the documents section on Care Planer.



Click the 3 bars as indicated by the red circle.



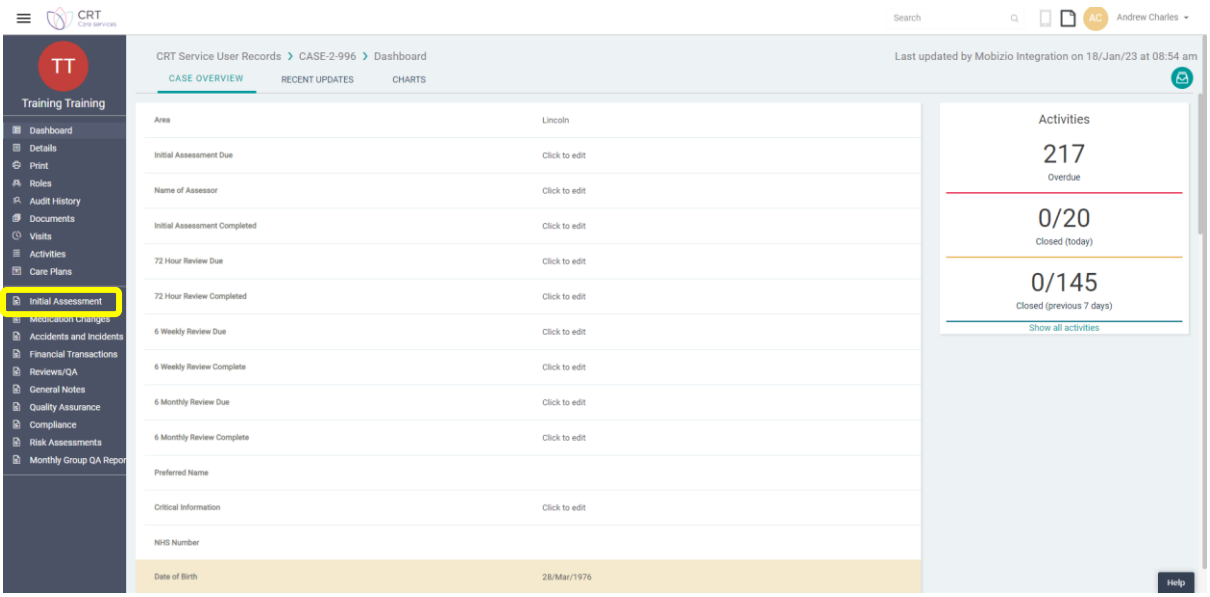
Then Select the service user records. This will take you to a full list of service users. It is better to use the search bar at the top of the screen to find the person we are looking for. As shown in the picture below.



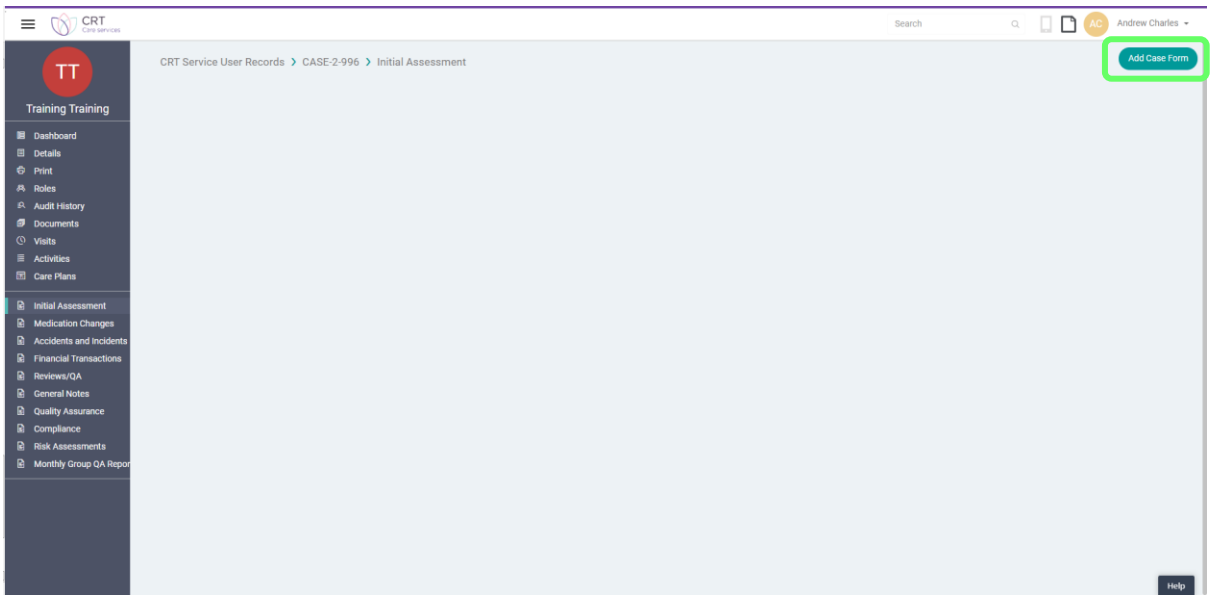
In the Search bar [indicated by the red box above] enter the service users name that we wish to assess.

The you can select the person from the list that is populated after the search. Select the service user by clicking on the name of the person. [as indicated by the green box above]

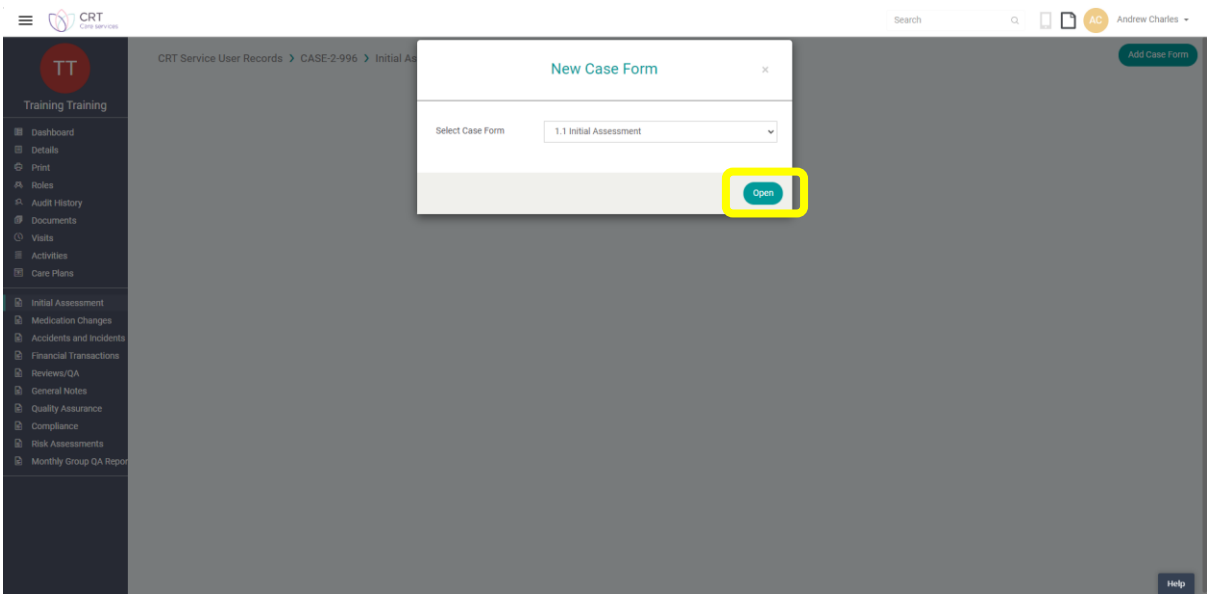
It will then take you the dashboard screen for the service user and then we can select the initial assessment, to complete the documentation. [Indicated in the Yellow box]



It will then take you to the screen below, and you will need to select add case forms. [indicated by the green box]

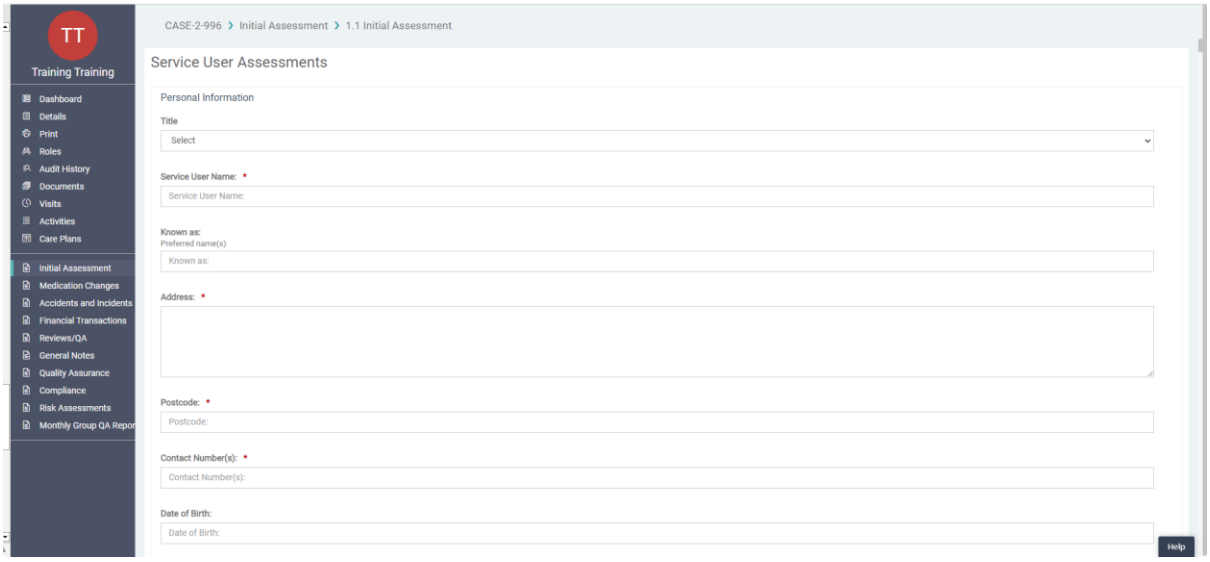


You will then be presented with this screen.



You need to click open to start the assessment. [As indicated by the yellow box]

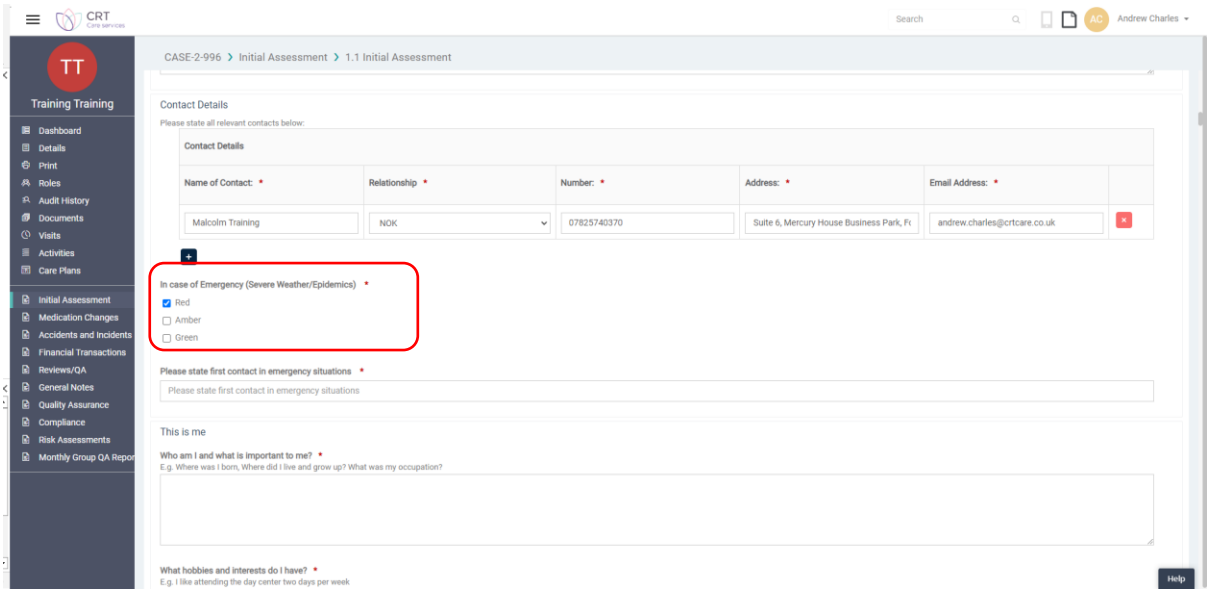
You Should now see this screen.



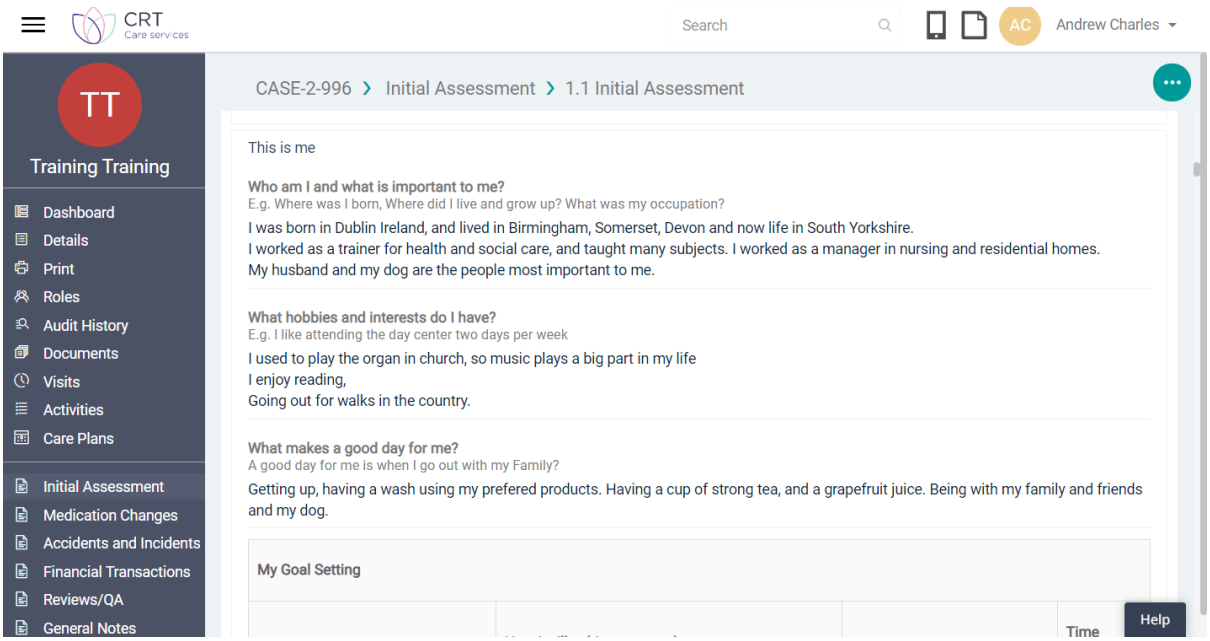
All the box's on the screen should be completed, and the form will not complete if any of the box's that are marked with a * are not completed.

As you populate the assessment, you will come across the section in case of emergency. You have to select if the service user, will:

a) Able to manage, no concern – Green
b) Family member or other person may be able to step in – Amber
c) No one to support, requires care – Red

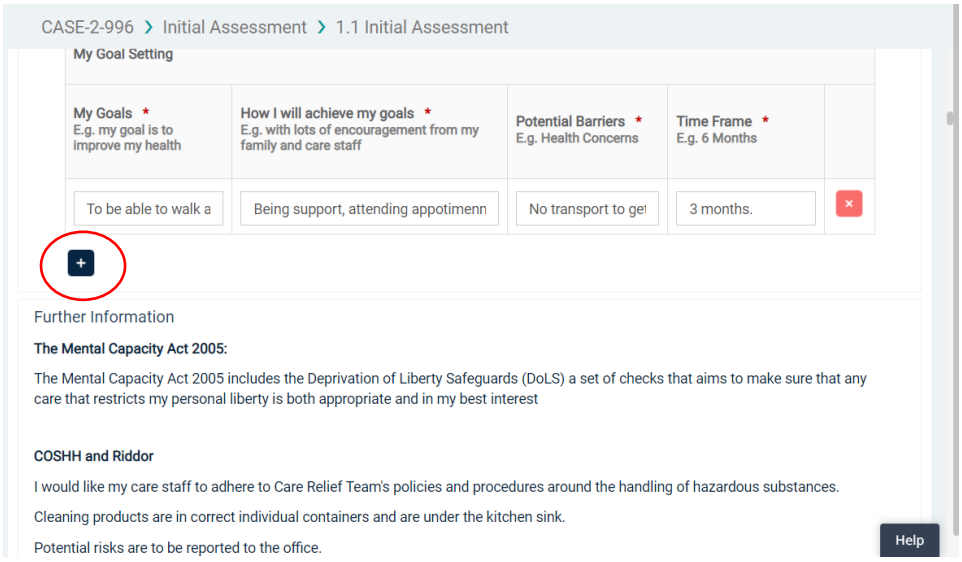


The next section is the This is me document, the section is important as it gives an overview of the person and lets the carers know what is important to the person i.e. family, friends, pets and key people.



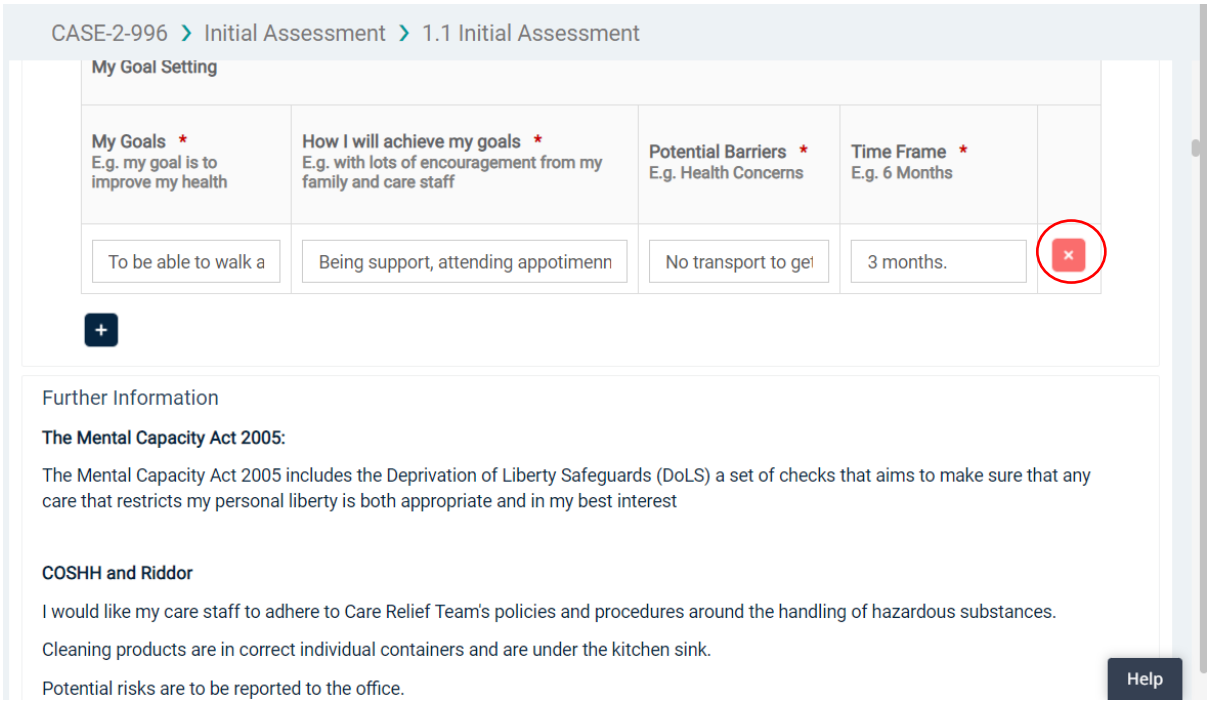
Next Section is goal setting and this is where we can add in goals for the service user, this can be rehabilitation goals, it may be a goal that the person wants to achieve. It maybe something as simple as “I want to be cared for and supported to continue to live in my own home.”

If the person has more than one goal then this should be added using the + button as indicated by the red circle.



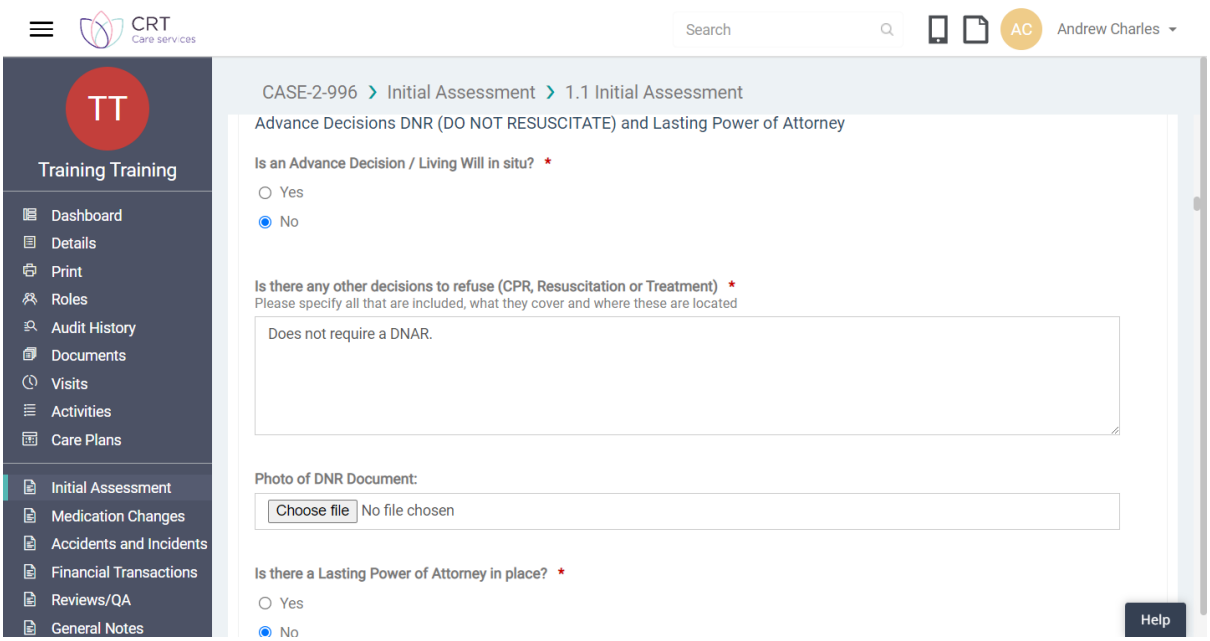
Through all the assessment there should not be any section that the questions are not answered. If there is a section that there is nothing relevant to add

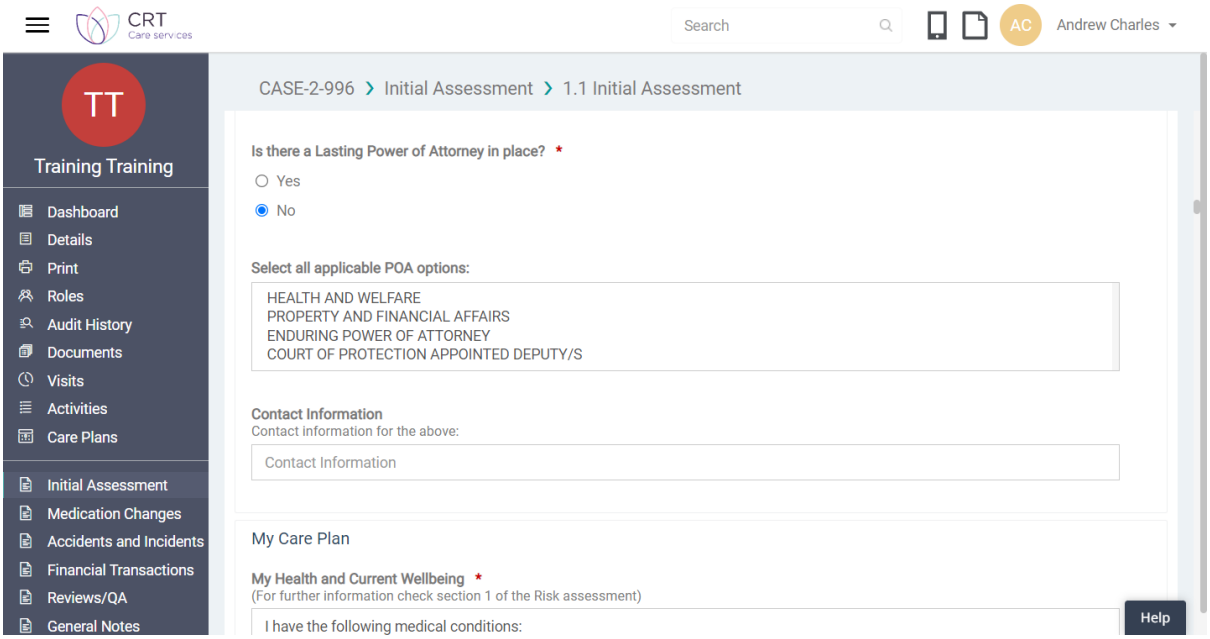
anything to then we can use the orange cross to remove the section. See red circle below.



The further information section, should be discussed with the service user, as this is already populated and completed for you.

The next section to be completed is the Advanced decision section, and this covers DNAR and Lasting Power of Attorney.

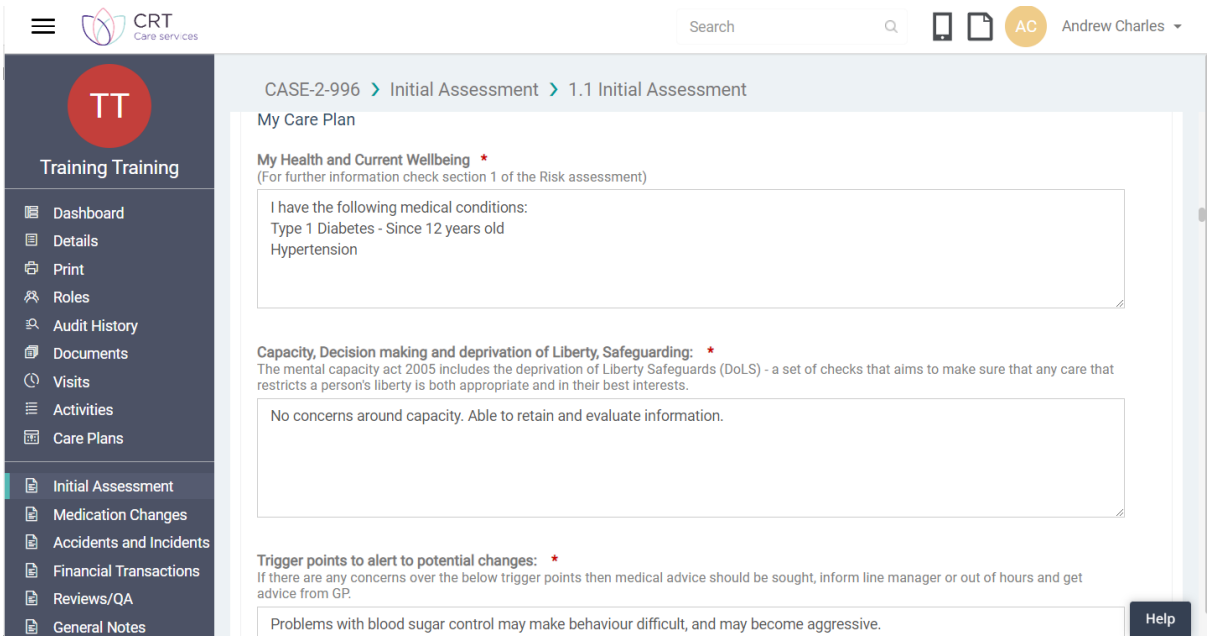


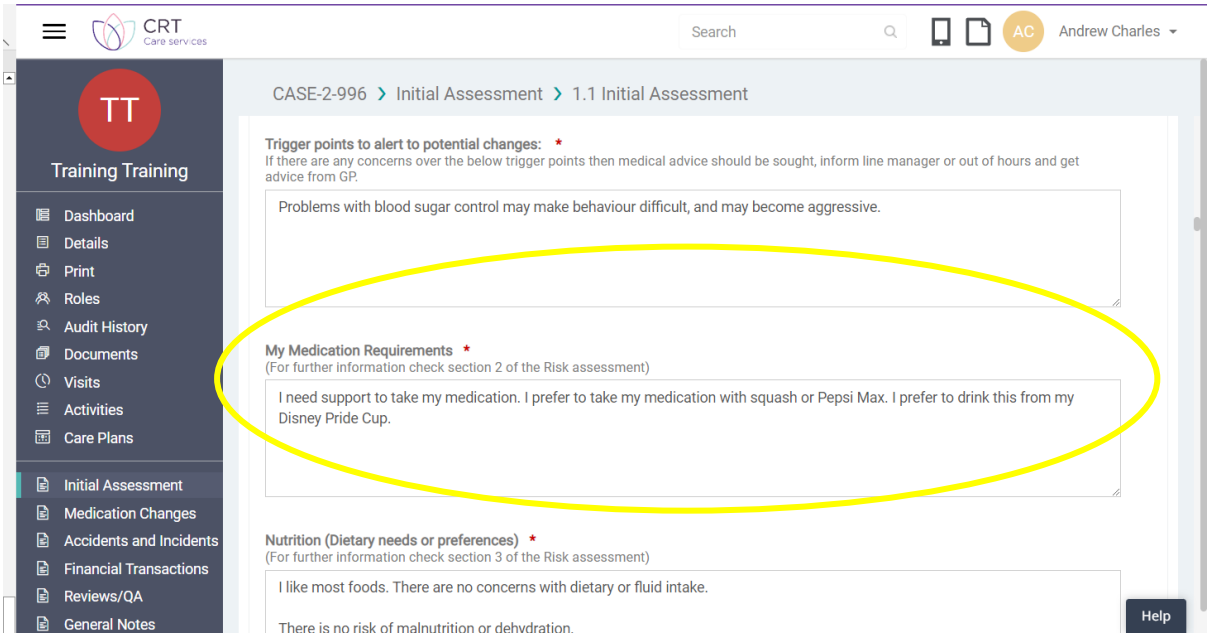


It is important that if someone declares they have POA [Power of Attorney] for either Finance / Health and Wellbeing, then we need to see the evidence of this. A photograph of the front page will be the evidence.

If someone holds a POA for Finance and Property, this does not given them the legal right to make decisions around the persons care. This can only be done if the person has POA for Health and Welfare.

So ensure that you are aware which is in place before discussing care and making decisions around the support required.

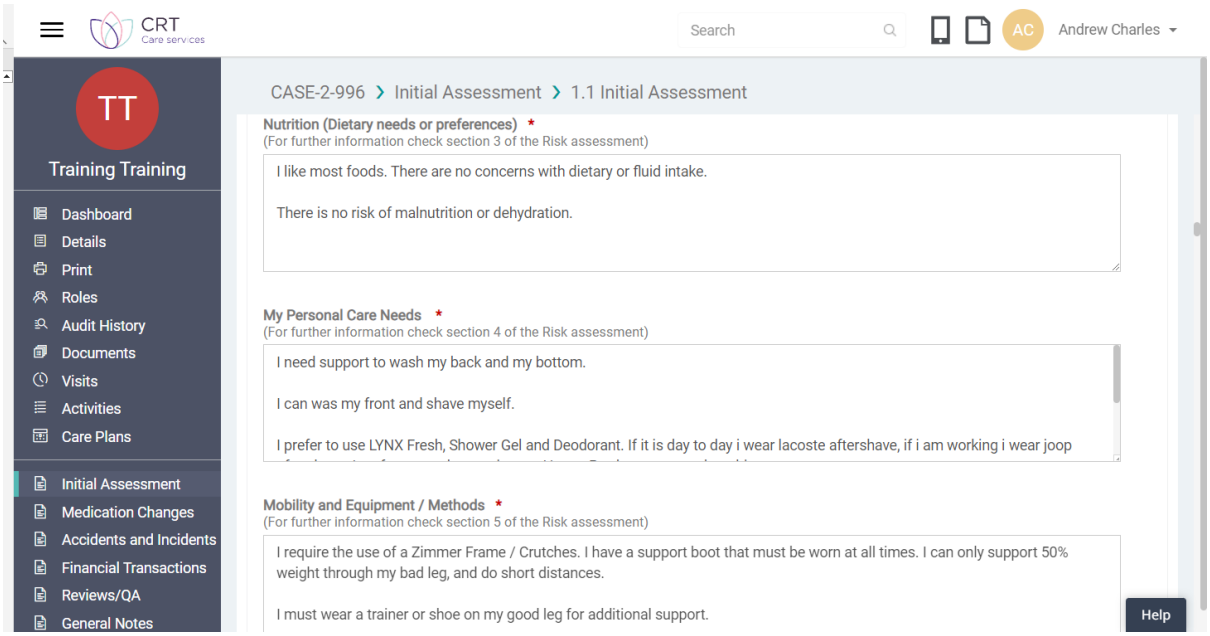


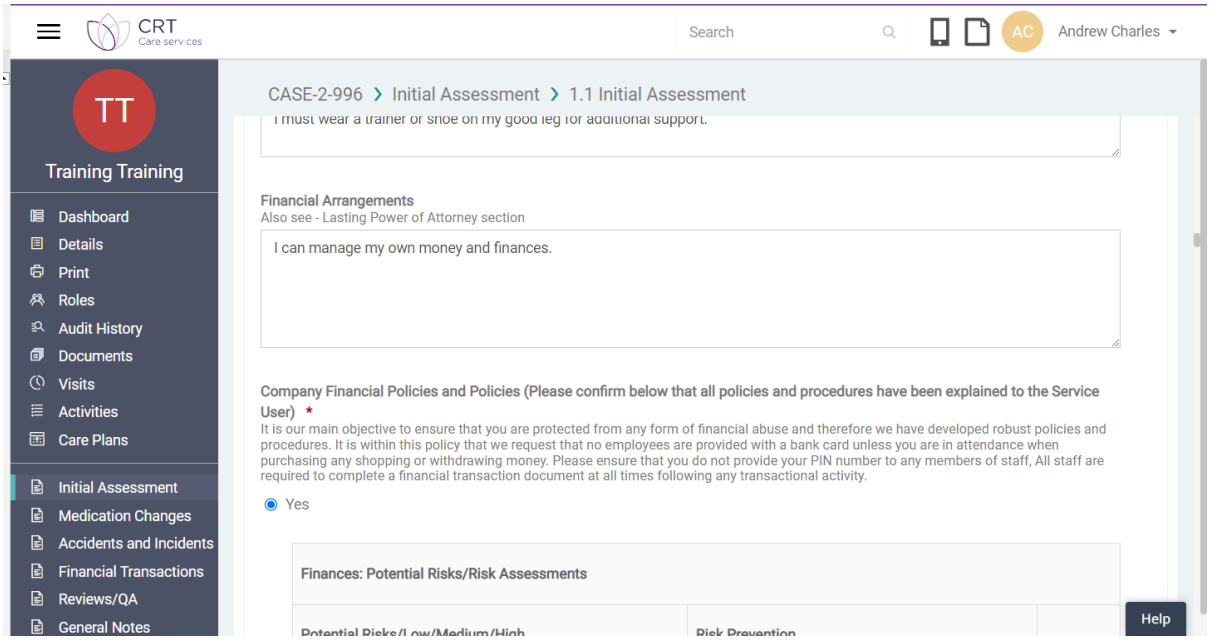


We can past medical history and diagnosis in the sections above. Also if there are triggers that may cause issues with the care these can be entered in to the sections as demonstrated above.

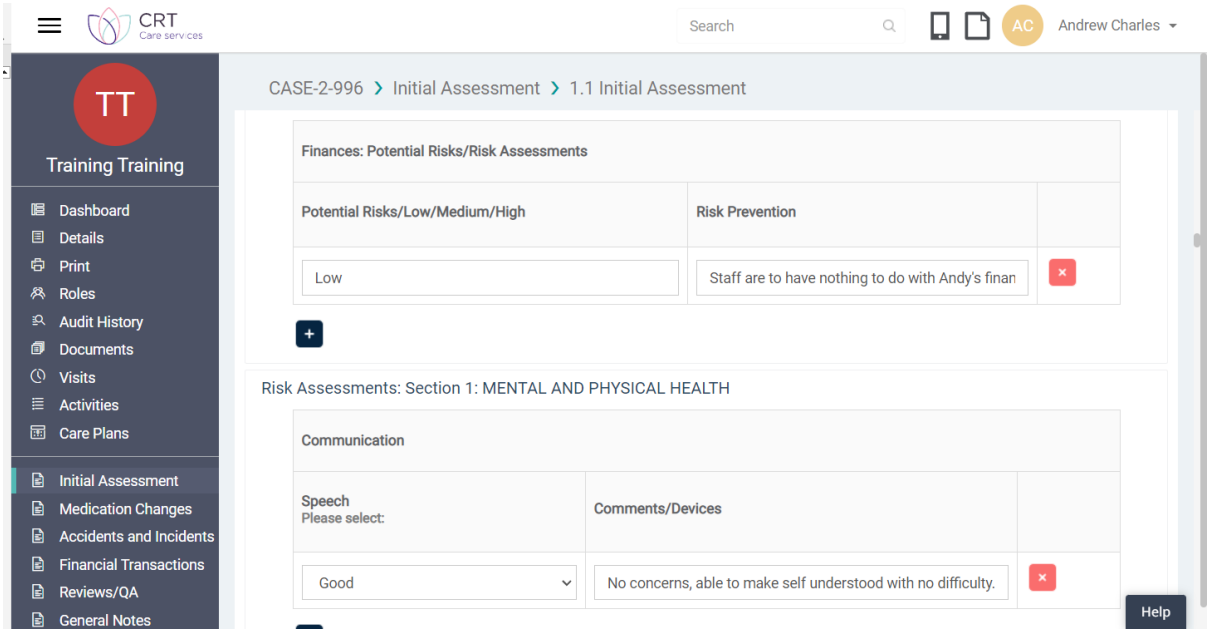
When writing into the care plan about the service users medication information about how the person wants to take their medication is important i.e. do they prefer a glass or a cup, do they prefer water, squash, soft drink, tea or coffee. Try to avoid fruit juice as grapefruit and cranberry can ruin certain medications. See the yellow circled information above.

The next few sections are straight forward and again add as much detail as you can to the section to ensure that the information is there, to enable you to write a detailed care plan.

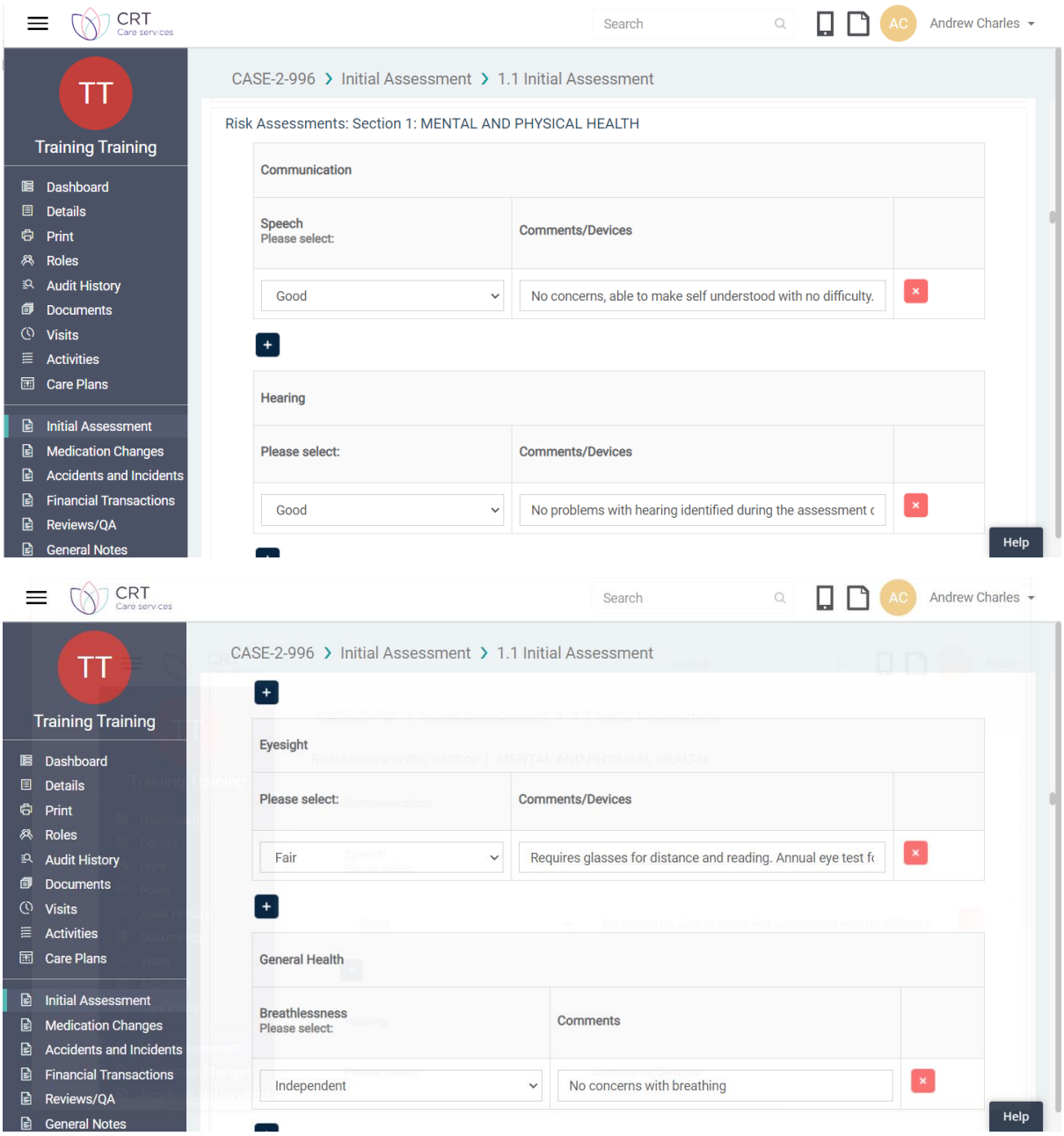




The next section is Risk Assessments for the Care Plan, each section MUST be completed and not left Blank.



If there is no identified issues, record this as a low risk, and explain the Risk Prevention, that the person or family manages this for the individual being supported.



As you can see from the sections above, if there are additional issues that you wish to cover as part of the assessment we can use the black + and this will add a new line for you to add to the risk assessment.

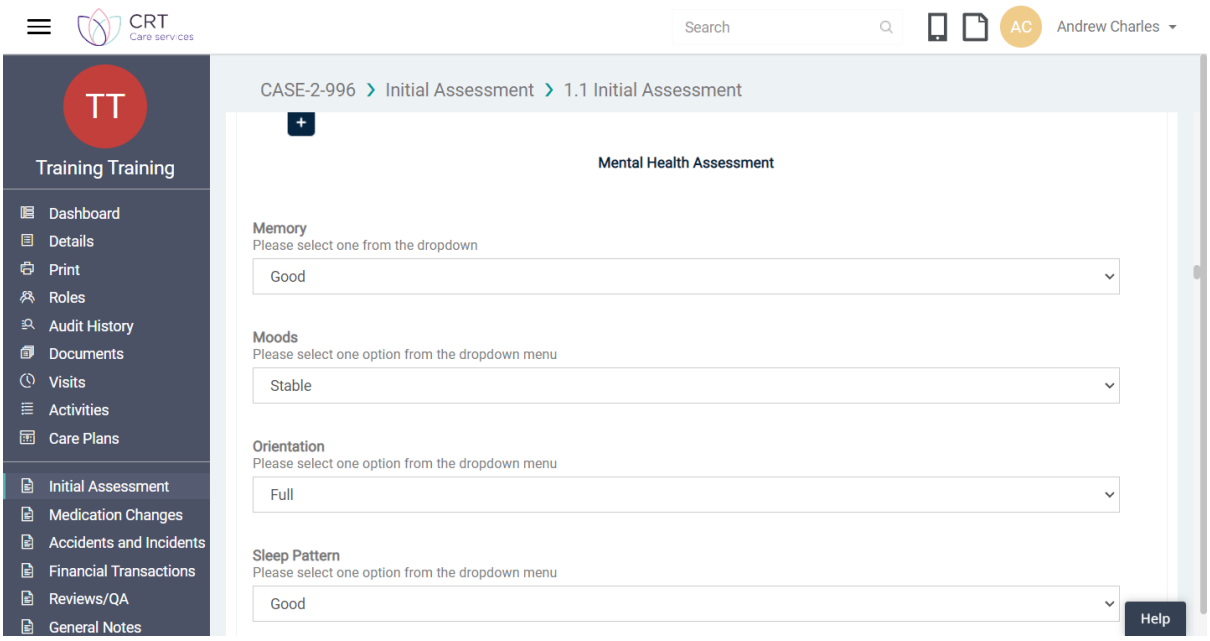
All the sections should be completed as mentioned above with as much detail as possible to ensure that you have enough information to ensure you care plan is robust.

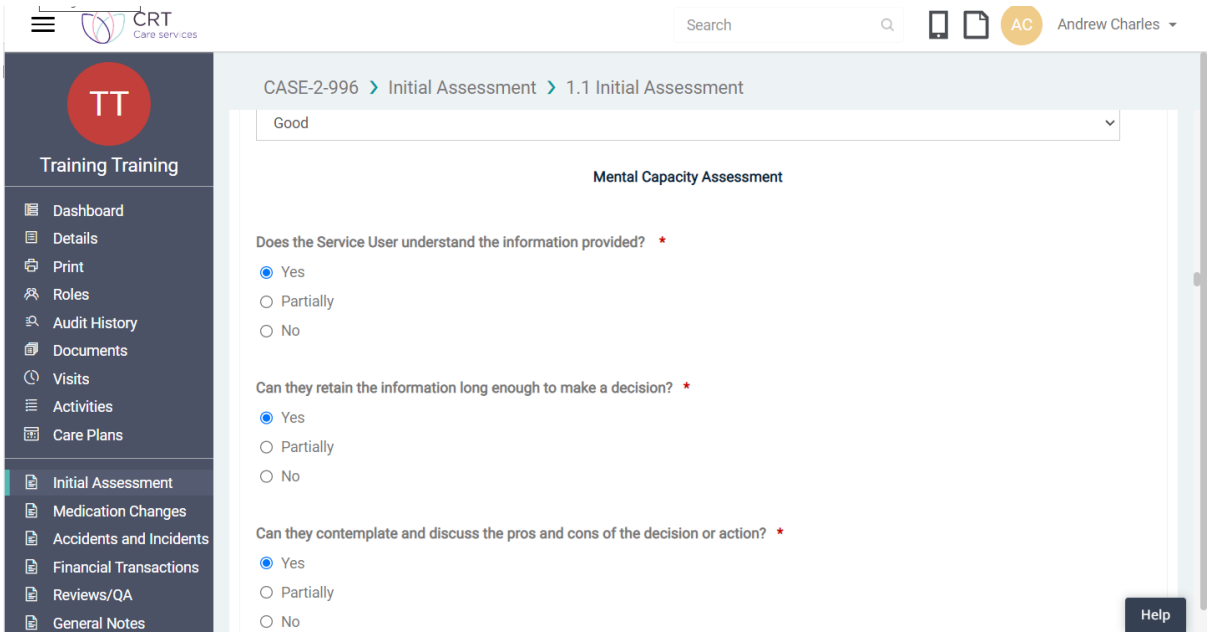
We now move on to the Mental Health Assessment.

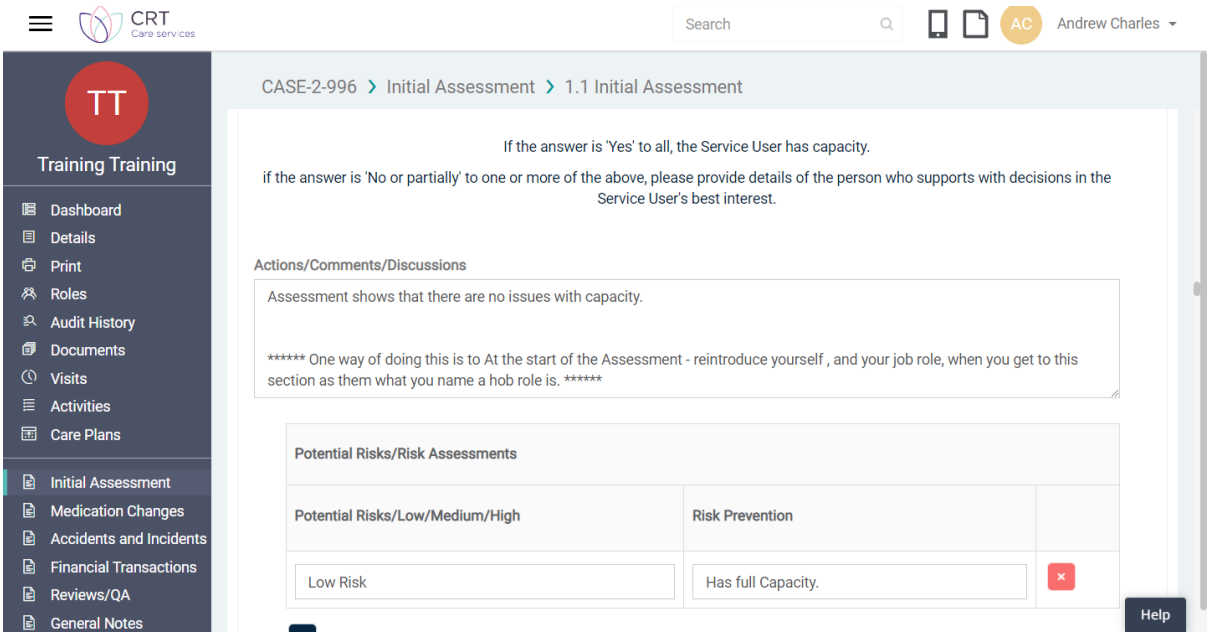
One way of checking if the person can retain information and evaluate is this.

At the start of the assessment, reintroduce yourself, your job role and what the purpose of your visit is. When you get to this section, as them what it was you said to them at the start and see if the person can relay the information back to you.

This helps fill the following section in:







The Nex section is the medication section. And please ensure that you detail all the medications correctly. We need to know the following:

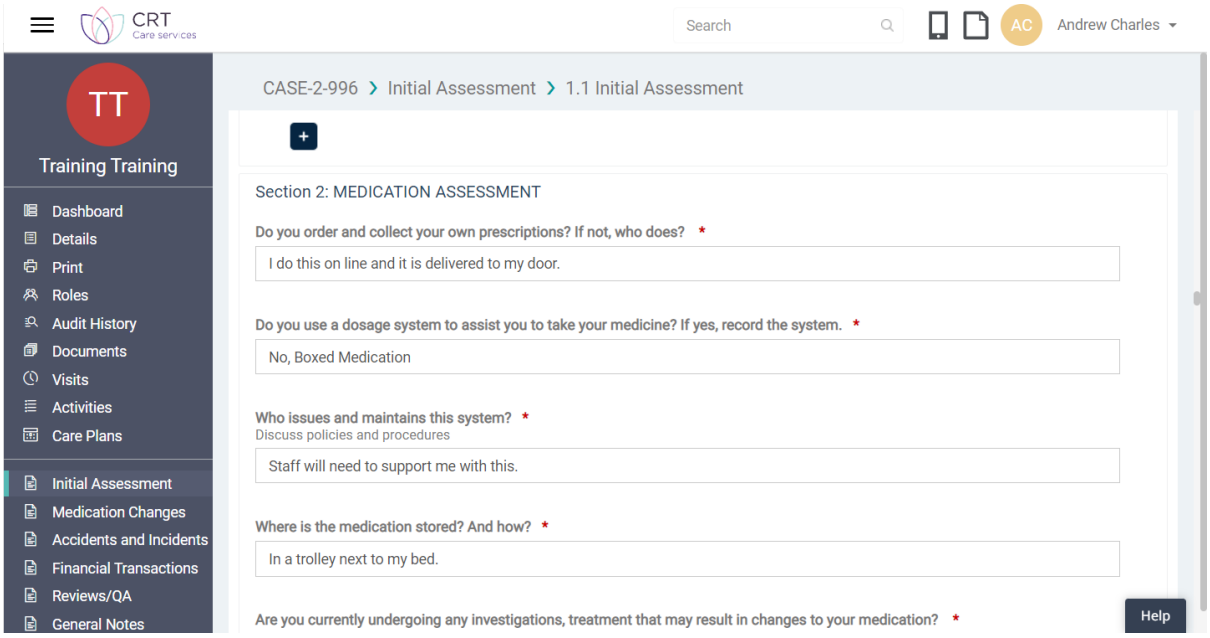
1- Name of Medication [Generic], make a note in the medication notes sections of the brand name. I.e Lithium Carbonate, can also be know as Piradel.

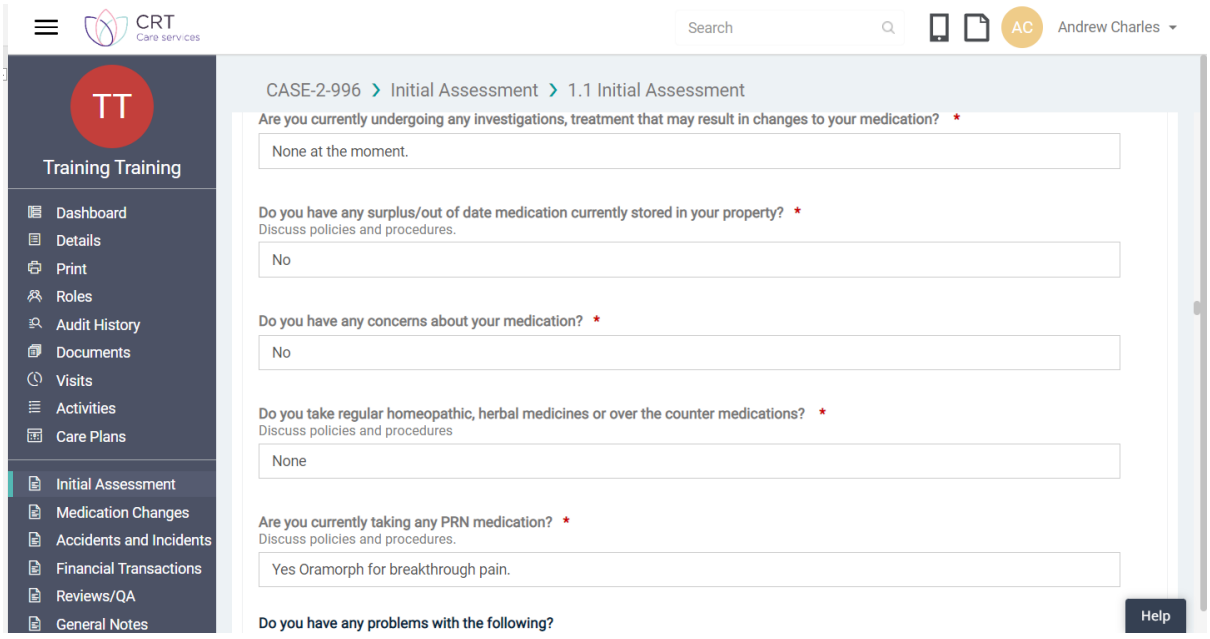
2- The strength of the medication i.e. 500mg, 25mcg, 10mg/5mls and 1g

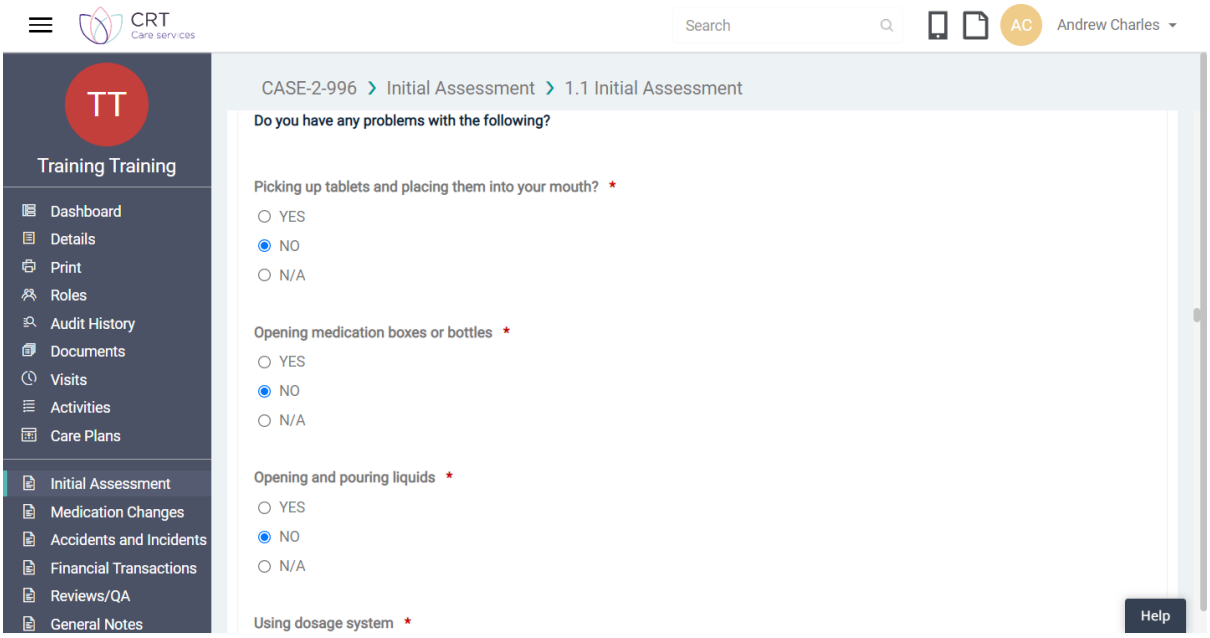
3- The Dose, Take 1 tablet three times a day

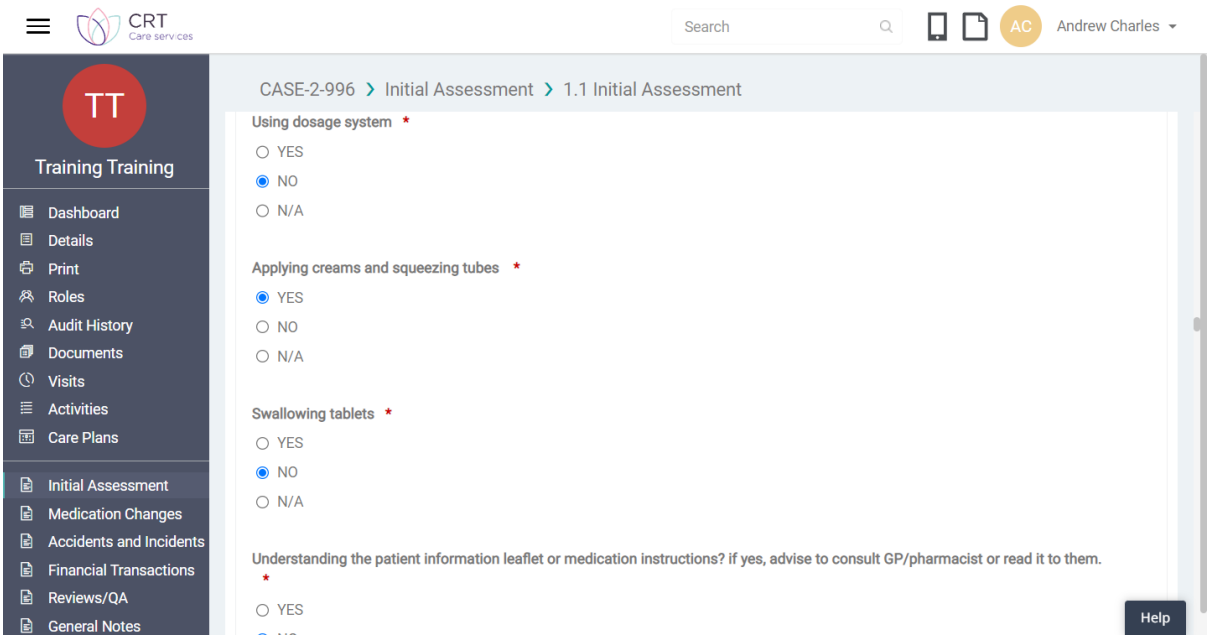
4- The time of day that the medication is taken. I.e. Morning, Lunch, Tea, Bed

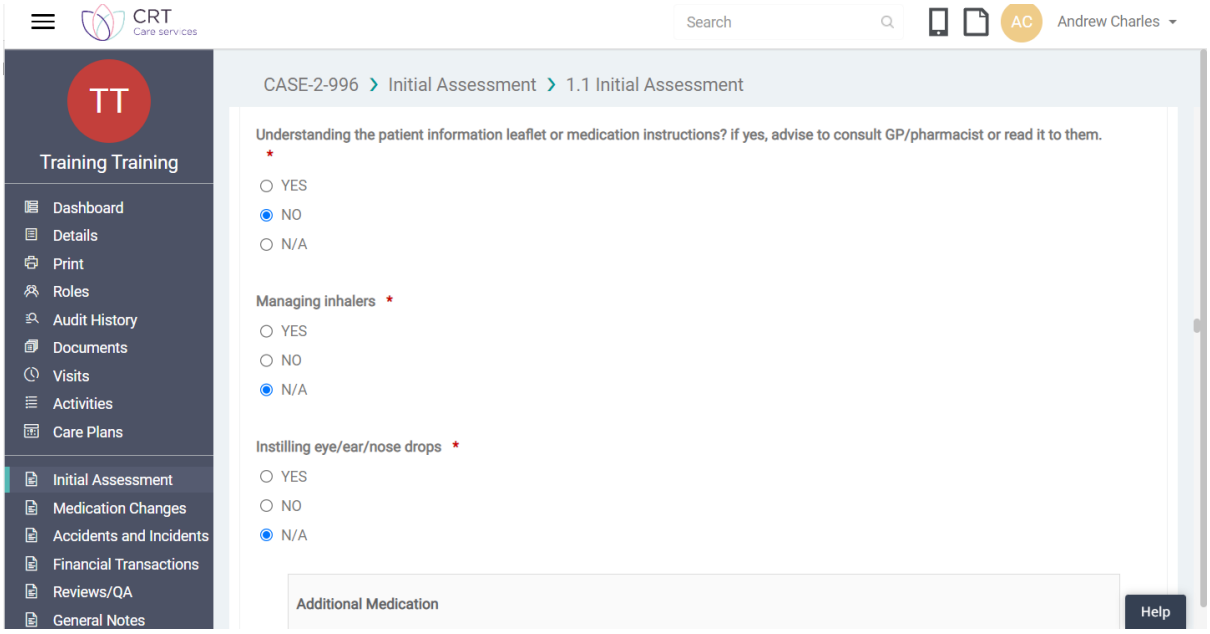
5- Special Instructions, with food, before food, before breakfast, and after food.


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Initial Assessment
Medication Changes
Accidents and Incidents
Financial Transactions
Reviews/QA
General Notes
CASE-2-996 › Initial Assessment › 1.1 Initial Assessment
Section 2: MEDICATION ASSESSMENT
Do you order and collect your own prescriptions? If not, who does? *
I do this on line and it is delivered to my door.
Do you use a dosage system to assist you to take your medicine? If yes, record the system. *
No, Boxed Medication
Who issues and maintains this system? *
Discuss policies and procedures
Staff will need to support me with this.
Where is the medication stored? And how? *
In a trolley next to my bed.
Are you currently undergoing any investigations, treatment that may result in changes to your medication? *
Help


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CASE-2-996 › Initial Assessment › 1.1 Initial Assessment
Are you currently undergoing any investigations, treatment that may result in changes to your medication? *
None at the moment.
Do you have any surplus/out of date medication currently stored in your property? *
Discuss policies and procedures.
No
Do you have any concerns about your medication? *
No
Do you take regular homeopathic, herbal medicines or over the counter medications? *
Discuss policies and procedures
None
Are you currently taking any PRN medication? *
Discuss policies and procedures.
Yes Oramorph for breakthrough pain.
Do you have any problems with the following?
Help

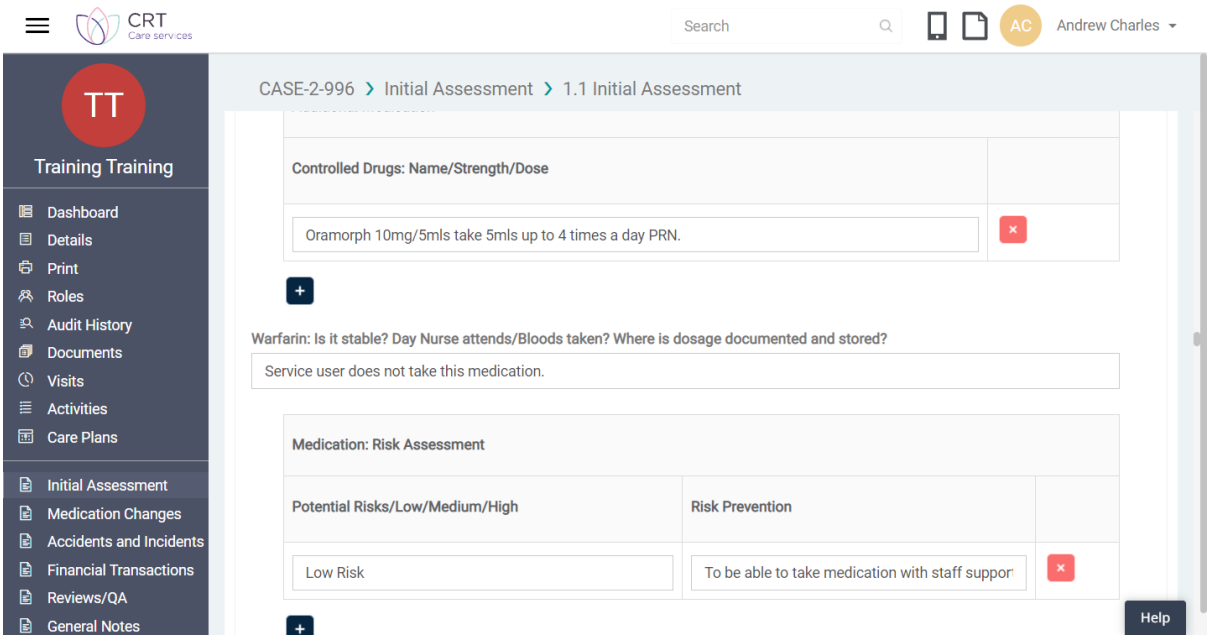

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CASE-2-996 › Initial Assessment › 1.1 Initial Assessment
Do you have any problems with the following?
Picking up tablets and placing them into your mouth? *
YES
NO
N/A
Opening medication boxes or bottles *
YES
NO
N/A
Opening and pouring liquids *
YES
NO
N/A
Using dosage system *
Help


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Financial Transactions
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General Notes
CASE-2-996 › Initial Assessment › 1.1 Initial Assessment
Using dosage system *
YES
NO
N/A
Applying creams and squeezing tubes *
YES
NO
N/A
Swallowing tablets *
YES
NO
N/A
Understanding the patient information leaflet or medication instructions? if yes, advise to consult GP/pharmacist or read it to them. *
YES
Help

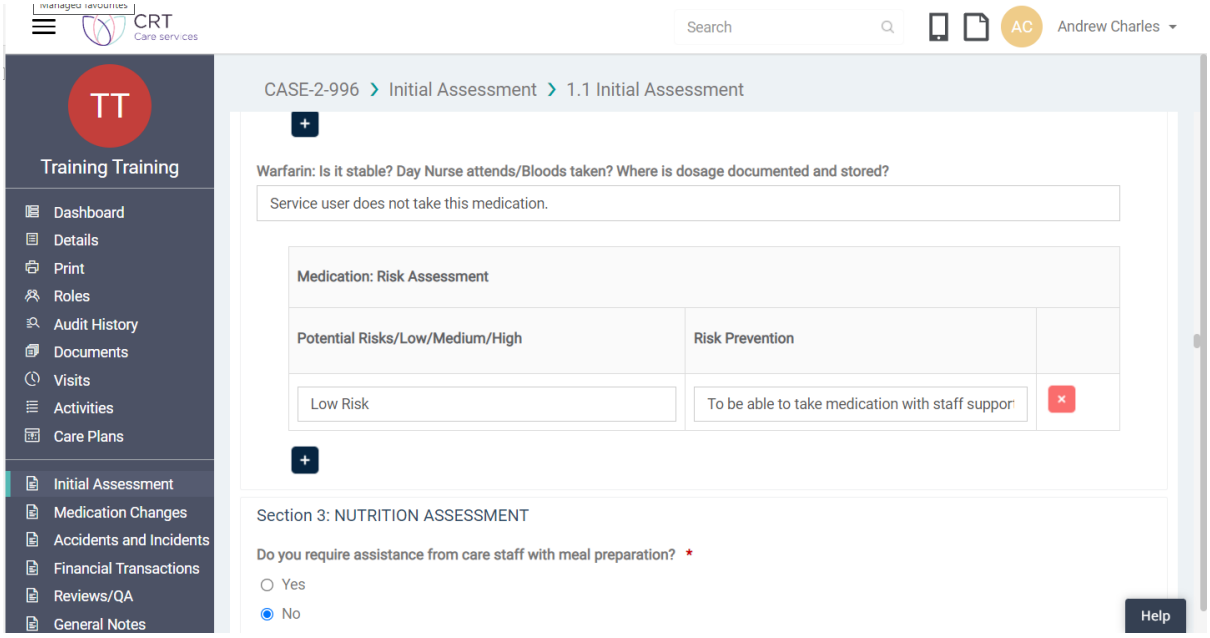


As you can see from the above pictures the questions are straight forward and easily answered.

Next section is controlled medications such as Morphine. If you are unsure if the medication is a controlled drug, please refer to the controlled medications reference guide.



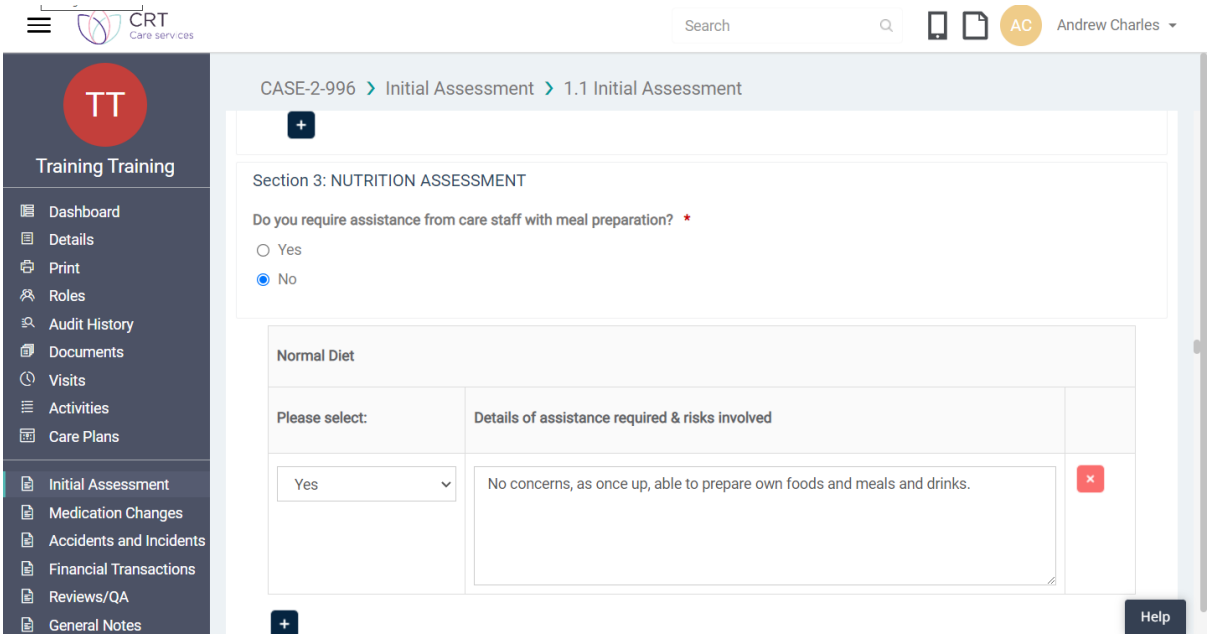
If there are not controlled medications click the orange cross, if the person takes more than one controlled medication click the black + to add the additional controlled medication.



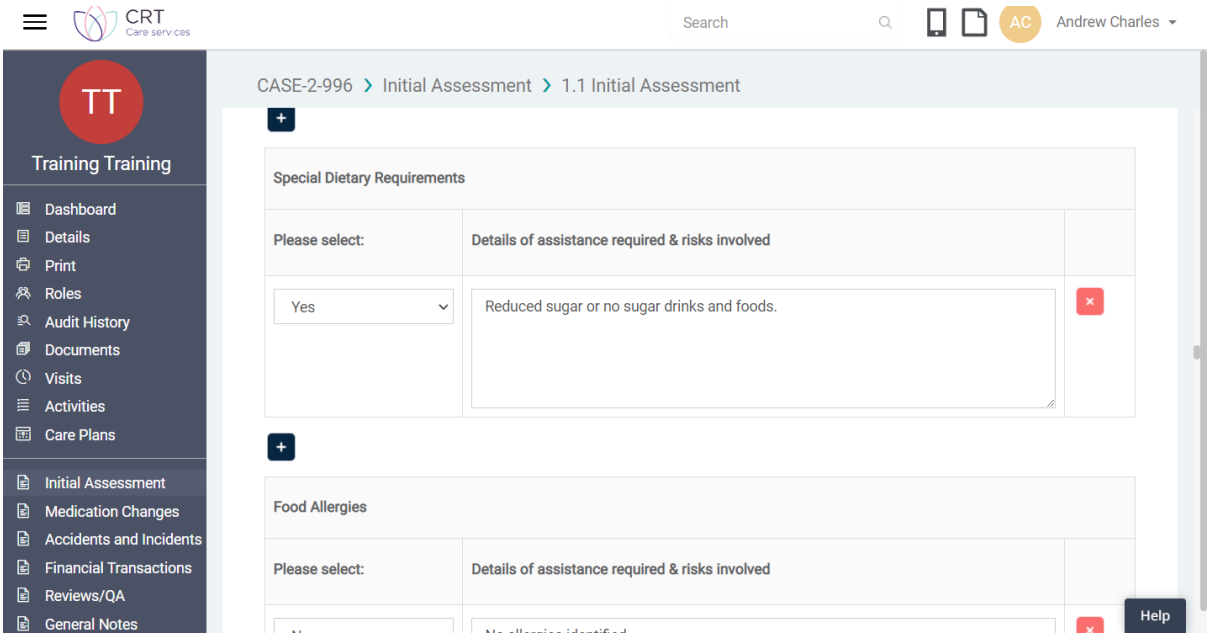
The medication risk assessment should include how support is given, how the person takes their medication and actions to take if it goes wrong.

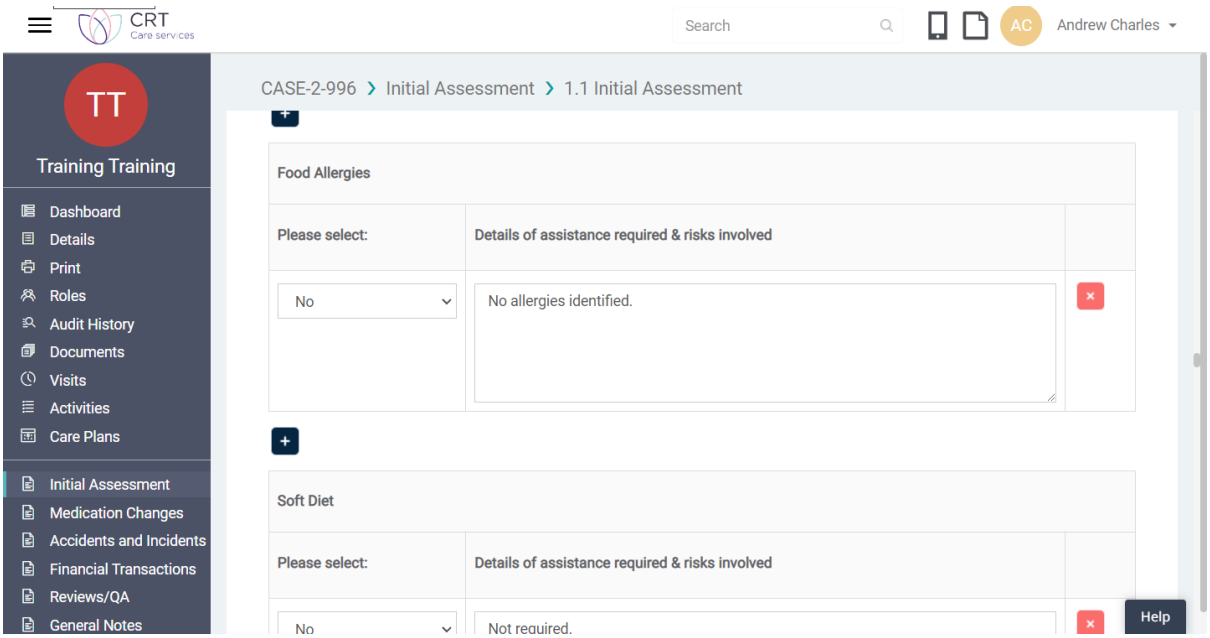
If there have been previous safeguarding or medication errors in the past we can add an additional row and risk assess this in this section.

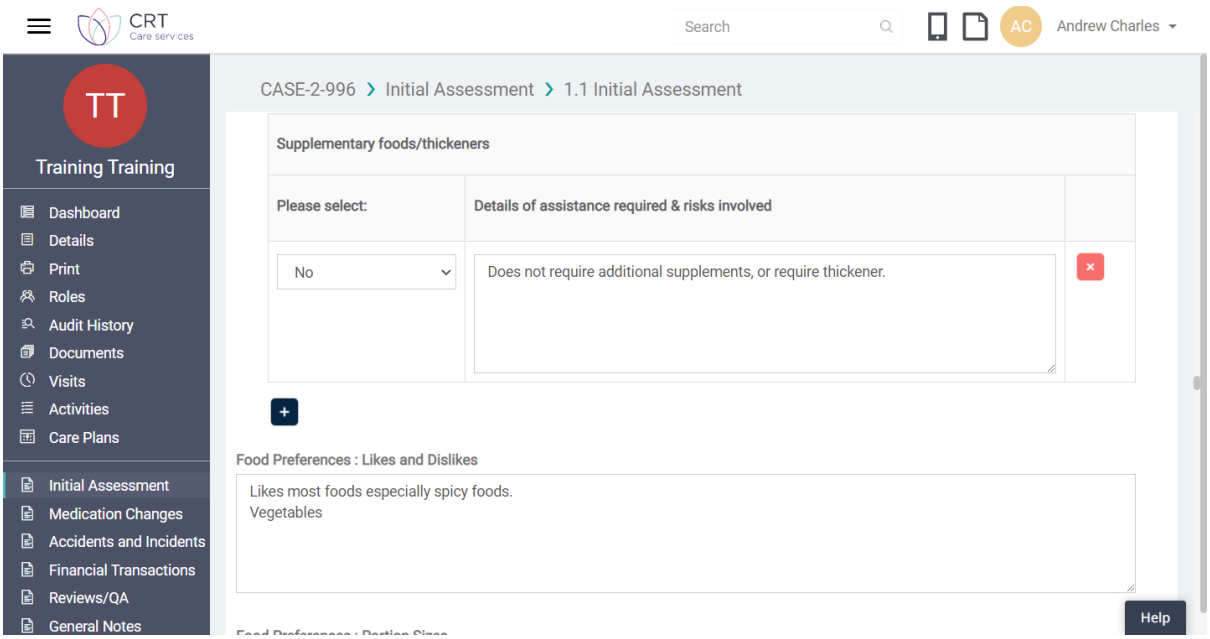
The next section that we move on to is the Nutrition section, and ensure that you complete all the information in as much detail as possible to ensure that the care plan is detailed.

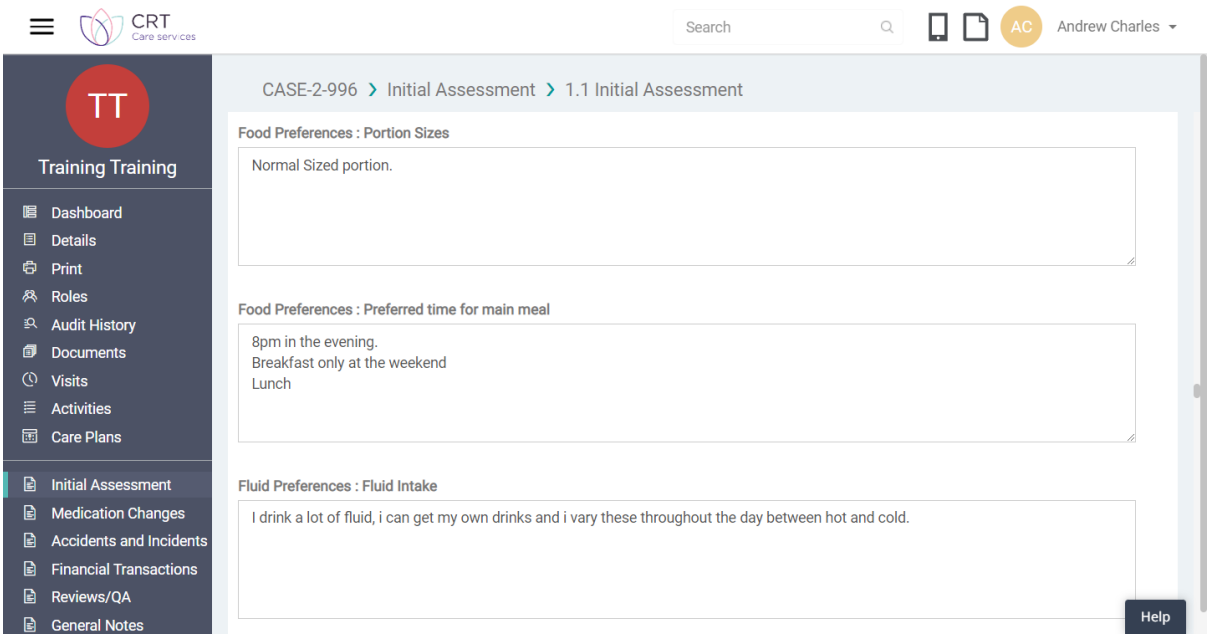


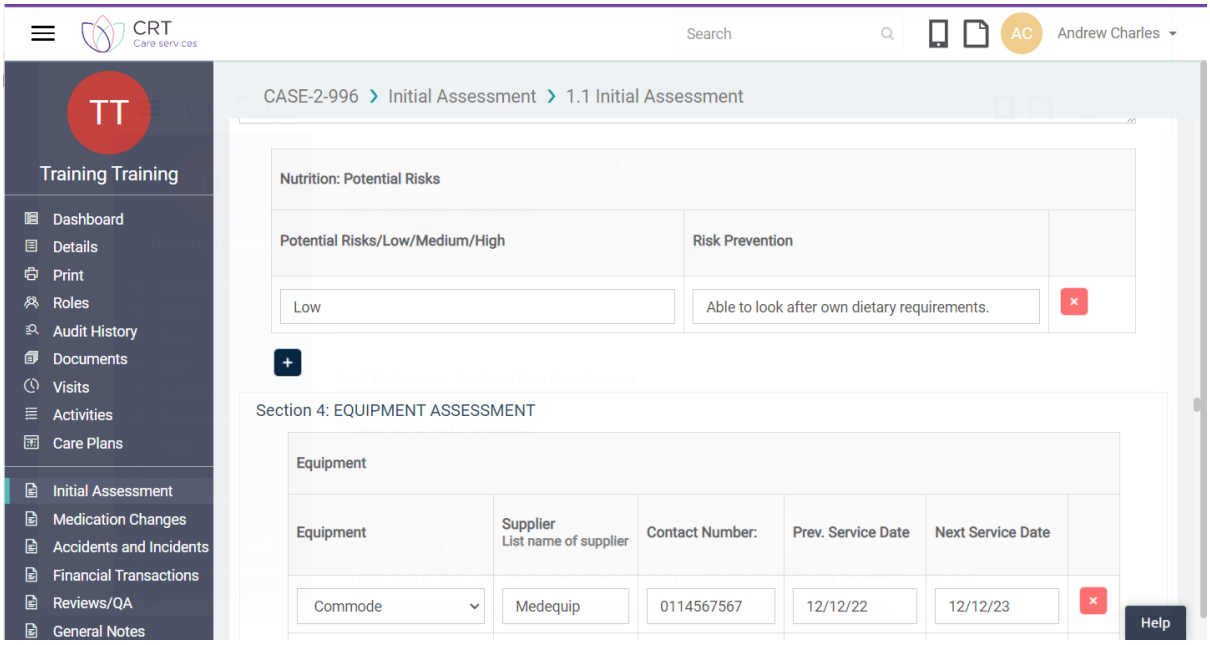
Again if we need to add additional information we can do this by clicking the black + to add an additional line.


CRT
Care services
Search
AC
Andrew Charles
TT
Training Training
Dashboard
Details
Print
Roles
Audit History
Documents
Visits
Activities
Care Plans
Initial Assessment
Medication Changes
Accidents and Incidents
Financial Transactions
Reviews/QA
General Notes
CASE-2-996 › Initial Assessment › 1.1 Initial Assessment
Special Dietary Requirements
Please select:
Details of assistance required & risks involved
Yes
Reduced sugar or no sugar drinks and foods.
Food Allergies
Please select:
Details of assistance required & risks involved
Help


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Medication Changes
Accidents and Incidents
Financial Transactions
Reviews/QA
General Notes
CASE-2-996 › Initial Assessment › 1.1 Initial Assessment
Food Allergies
Please select:
Details of assistance required & risks involved
No
No allergies identified.
Soft Diet
Please select:
Details of assistance required & risks involved
No
Not required.
Help

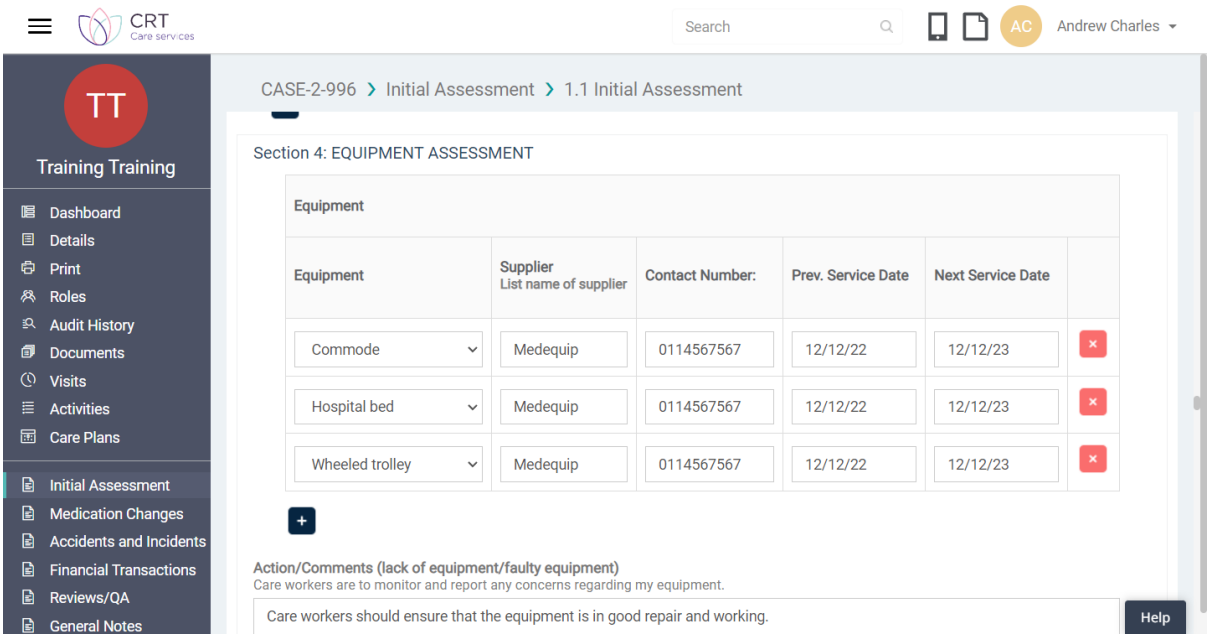

CRT
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Search
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Care Plans
Initial Assessment
Medication Changes
Accidents and Incidents
Financial Transactions
Reviews/QA
General Notes
CASE-2-996 › Initial Assessment › 1.1 Initial Assessment
Supplementary foods/thickeners
Please select:
Details of assistance required & risks involved
No
Does not require additional supplements, or require thickener.
Food Preferences : Likes and Dislikes
Likes most foods especially spicy foods.
Vegetables
Help


CRT
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Training Training
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Initial Assessment
Medication Changes
Accidents and Incidents
Financial Transactions
Reviews/QA
General Notes
CASE-2-996 › Initial Assessment › 1.1 Initial Assessment
Food Preferences : Portion Sizes
Normal Sized portion.
Food Preferences : Preferred time for main meal
8pm in the evening.
Breakfast only at the weekend
Lunch
Fluid Preferences : Fluid Intake
I drink a lot of fluid, i can get my own drinks and i vary these throughout the day between hot and cold.
Help

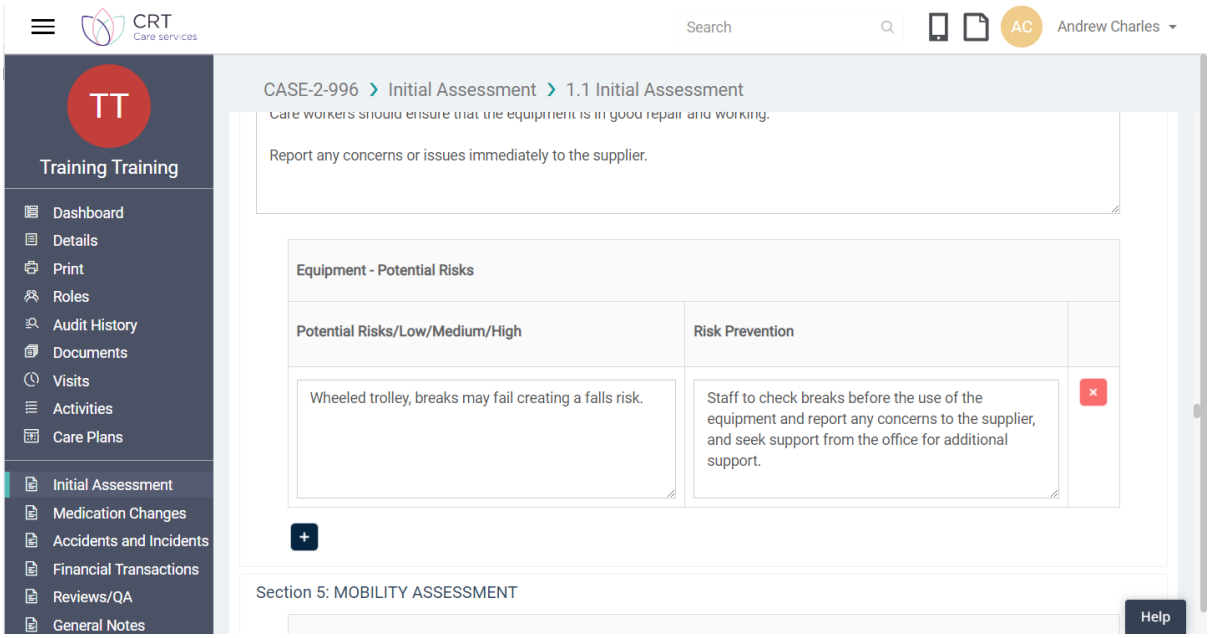


Once all the above sections have been completed ensure that you complete the risk assessment section at the bottom. If there are no concerns record it as a low risk and explain in the risk prevention section why it is low risk.

Next Section looks at the equipment that the person uses.

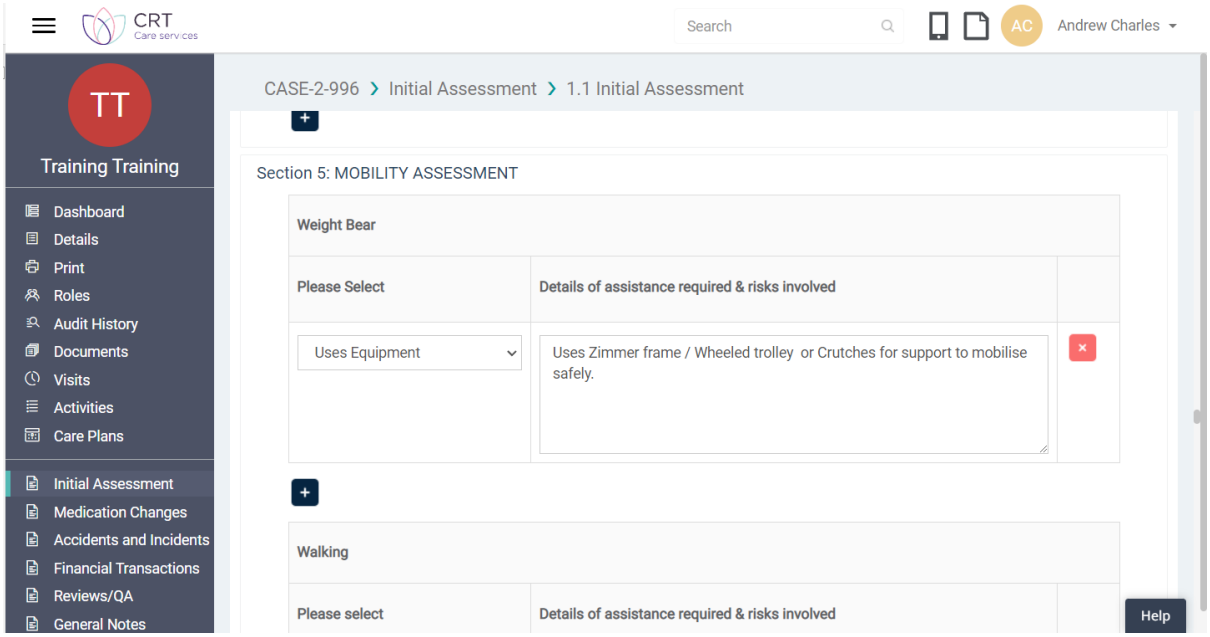


It is important that we record what equipment is used, who supplied it, when was it serviced, when the next service is due. Especially important when we are using hoists, slings and profiling beds.

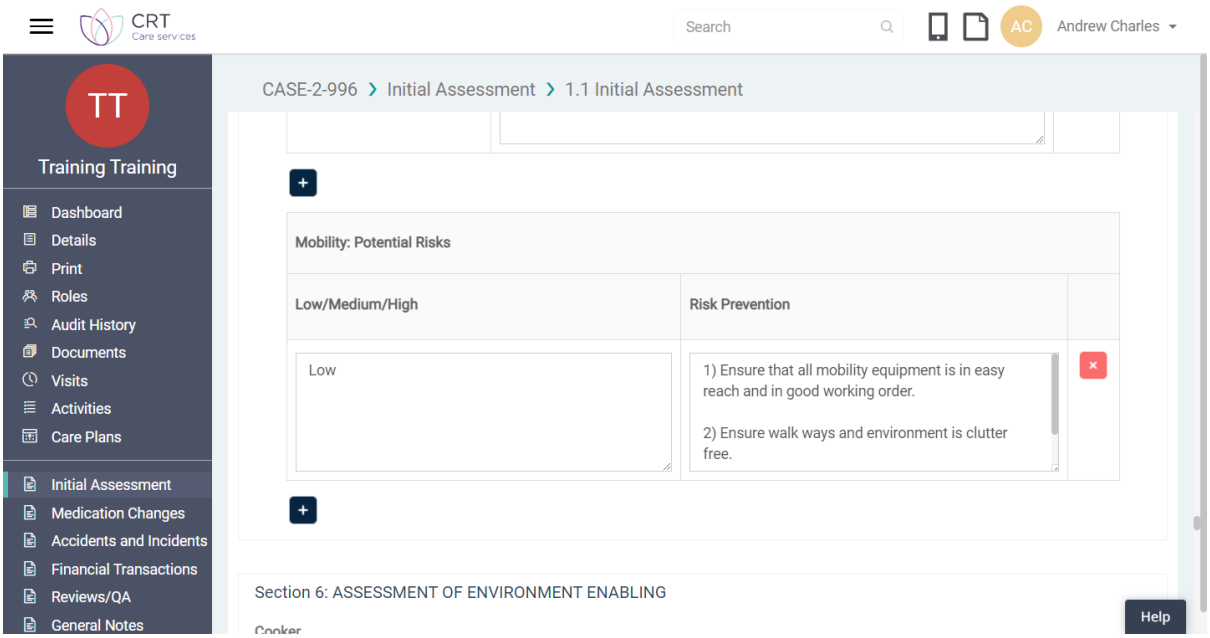


Again any risks need to be recorded and documented in the section as outlined above.

The Next section is the mobility section and needs to be completed.



Ensure that we are completing all the sections, and remember if equipment is required then make sure that you document in all the sections what equipment is required. Sometimes the person may have 2 different pieces of equipment for the same task but different times. i.e. inside the house uses a wheeled Zimmer frame, but when out and about in community uses a wheeled trolley frame.



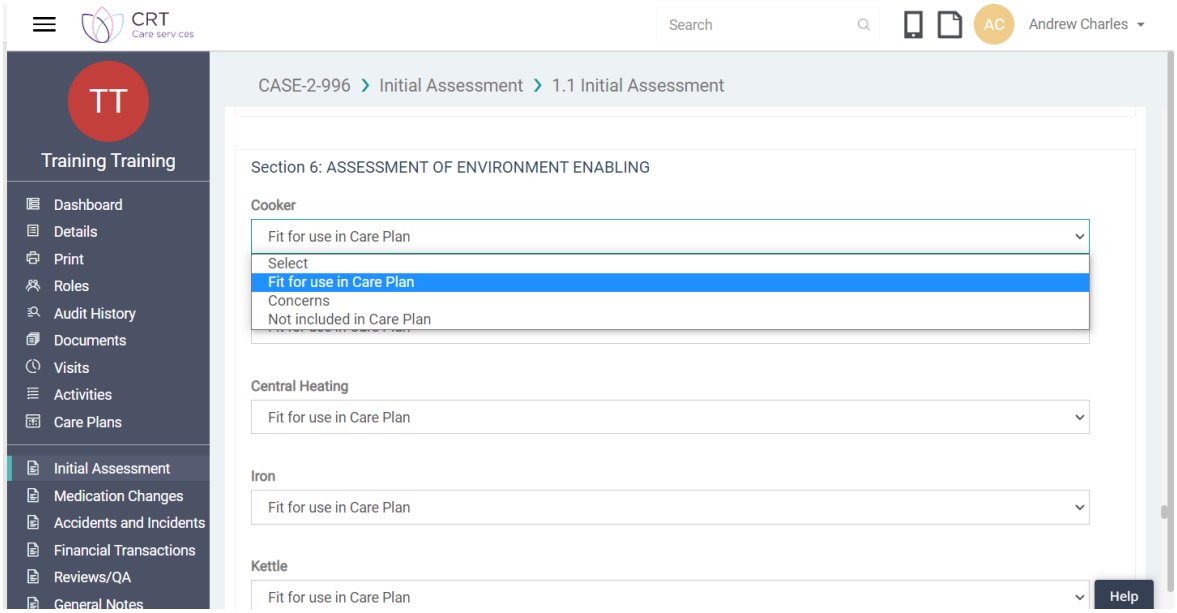
Ensure that we are covering all risks and how to overcome them in the above sections.

When we are writing in relation to hoisting, ensure that there is something in about which coloured loops are to be used to get the person in and out of the bed, bed to commode, bed to chair and chair to bed. Ensure that we are writing in the care plan to check status of the sling, hoist is charged and put back on to charge at the end on the day.

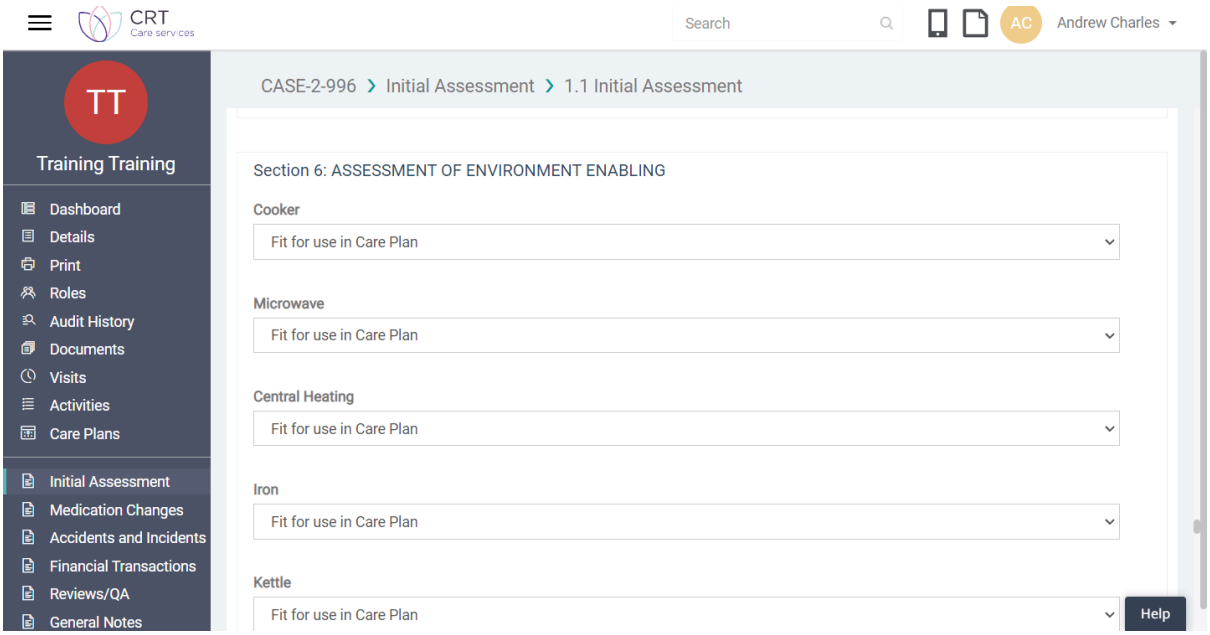
Ensuring the way we hoist and the sling type, and how to attach to the hoist is documented in the handling plan part of the risk assessment and the care plan.

The next section is the Assessment of the living environment.

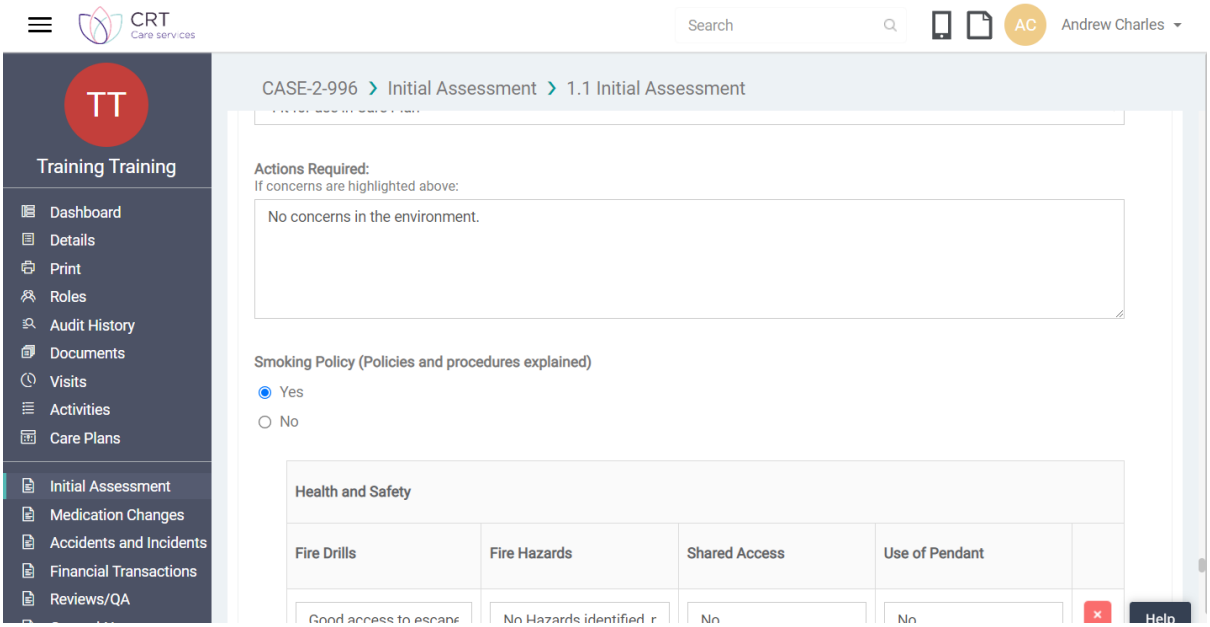
For each question there are 3 options as outlined below.



Each section is required to be completed.

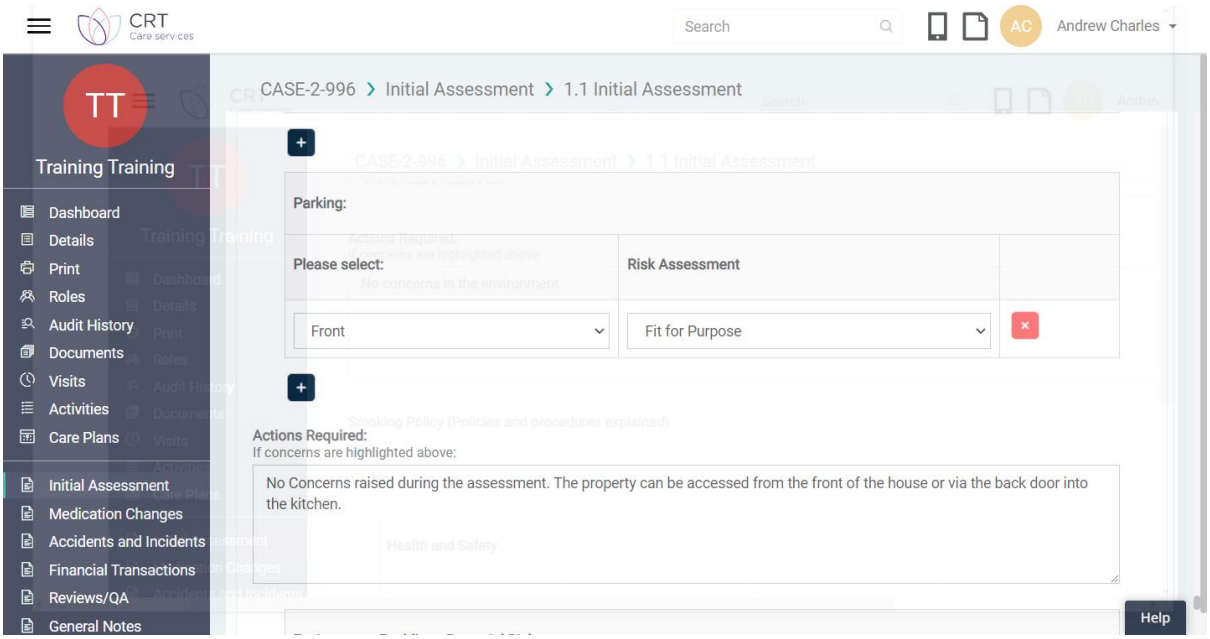


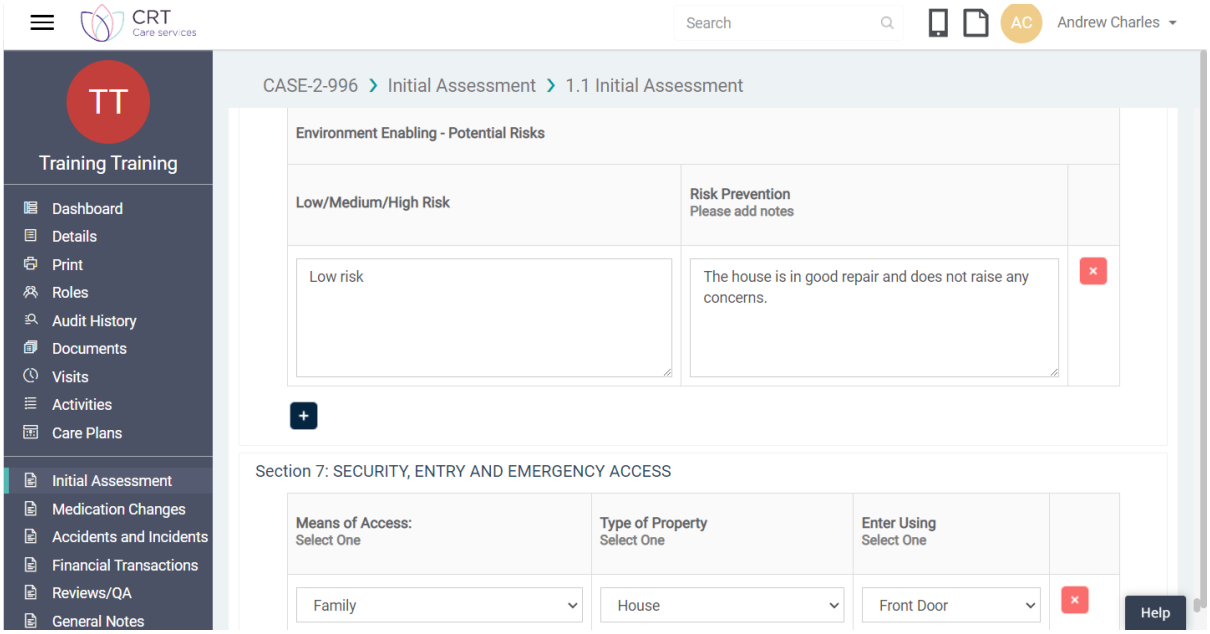
If you identify any section that there is cause for concern or a problems. Ensure that you document that in the section as shown on the next page, and this is raised with the relevant people and documented.

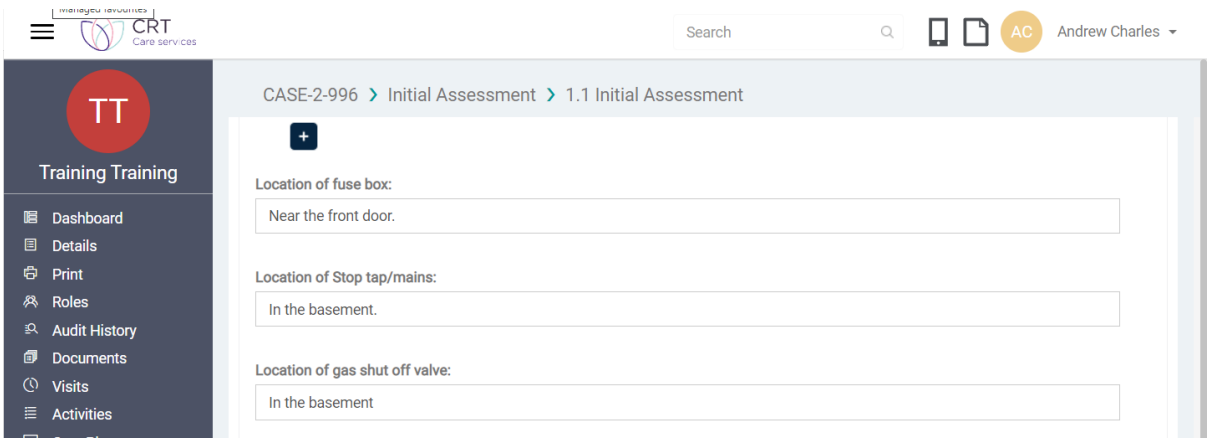


There are a number of risk assessments to be completed for the environments.

Complete each section in as much detail as possible.

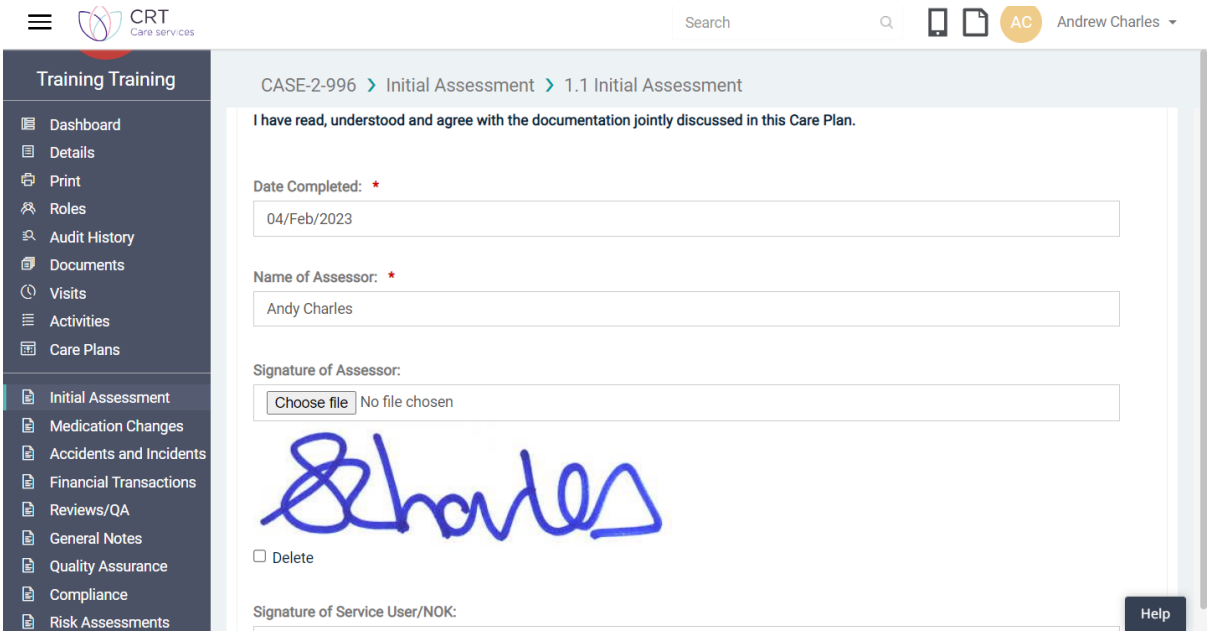






It is important that we complete the sections that we identify emergency cut off points in case there is an emergency in the persons home while the carers are visiting.

You then need to complete the Consent to care and data protection sections and get the individual to sign, and sign yourself.



On you phone or tablet there will also be the consent forms for care etc which will need to be completed while you are doing the assessment.

Roper, Logan and Tierney

The Roper-Logan-Tierney Model for Nursing is a theory of nursing care based on activities of daily living, which are often abbreviated ADLs or ALs. The model is widespread in the United Kingdom, especially in the public sector. The model is named after its developers: Nancy Roper, Winifred W. Logan, and Alison J. Tierney. The inspiration comes from the work of Virginia Henderson.

The purpose of the theory is as an assessment used throughout the service user's care. In the United Kingdom, where the model is prevalent, it has been reduced to being used simply as a checklist. It is often used to assess how the life of a service user has changed due to illness, injury, or admission to a hospital rather than as a way of planning for increasing independence and quality of life.

The theory attempts to define what living means. It categorises the discoveries into activities of living through complete assessment, which leads to interventions that support independence in areas that may be difficult for the service user to address alone. The goal of the assessment and interventions is to promote maximum independence for the service user.

The assessor uses the model to assess the patient's relative independence and potential for independence in the activities of daily living. The service user's independence is looked at on a continuum that ranges from complete dependence to complete independence. This helps the assessor determine what interventions will lead to increased independence as well as what ongoing support is needed to offset any dependency that still exists.

The activities of daily living should not be used as a checklist. Instead, Roper states they should be viewed "As a cognitive approach to the assessment and care of the service user, not on paper as a list of boxes, but in the assessor's approach to and organisation of care," and that carers deepen their understanding of the model and its application. The service user should be assessed on start of service, and his or her dependence and independence should be reviewed throughout the care plan and evaluation. By looking at changes in the dependence-independence continuum, the carer's can see whether the patient is improving or not, and make changes to the care provided based on the evidence presented.

The activities of living listed in the Roper-Logan-Tierney Model of Nursing are:

- **Maintaining a safe environment**



This is probably one of the activities which if totally independent in all twelve activities of living the patient will often take for granted. However when illness or disability is present then maintaining a safe environment is vital to protect the patient from harm. Where a person is on the lifespan will contribute to this. Younger patients may not be aware of the dangers present within a healthcare setting or as a result of their illness. At the same time an elderly patient who is suffering from confusion may be disorientated by an unfamiliar environment. Issues around pressure care and preventing falls as well as infection control measures need to be considered within this activity.

- **Communication**

It is important to assess a patient's communication skills. How do they normally communicate and is this being effected by their recent illness or accident. Aids to communication can be identified and implemented. Roper et al (2006) state that this activity is a good point to assess a person's pain. The reasoning for this is that communication, both verbal and non-verbal will play a large part here in how a person conveys their pain. Pain will also come into other activities of living as it has an effect upon them.

- **Breathing**

  This should have already been assessed as part of your initial airway, breathing and circulation assessment (Stonehouse, 2017). However at this point of the assessment you are finding out does the patient have any breathing difficulties. Do they take any medication to help them breath, such as inhalers? Do they smoke? Do they become breathless on movement?

- **Eating and drinking**

  A service user's normal diet can be assessed here. Are there any special dietary needs that need to be catered for while they are in your care. Are they eating normally or has this changed recently? How much and how often do they eat? For patients who are under or over weight, or who are diabetic this is very important.

- **Elimination**

This can often be a sensitive activity for people to talk about especially to a stranger, however through building up a therapeutic relationship (Doherty & Thompson, 2014) and acting in a professional manner, patients will hopefully become comfortable discussing this. What is their normal activity in respect to elimination? Is the patient experiencing any problems with their bowels and bladder? How often do they go to the toilet? Again, has this changed recently? Appropriate language needs to be used here as what an adult may understand and say may well be different to a child.

- **Washing and dressing**

  This activity is concerned with how self-caring a patient is in washing and dressing themselves. Do they normally wash, bath or shower and how often? Do they normally require assistance? Mouth care and teeth cleaning need to be considered. How do they like to dress? An assessment of a patient's skin and potential for developing pressure sores should be performed if relevant (Roper et al, 2006).

- **Controlling temperature**

  This can be done by observing the patient and by taking their temperature. Do they look flushed or are they shivering? Ask the patient what is normal for them, do they usually feel hot or cold? Are they able to take off or put on clothes to help control their temperature?

- **Mobilisation**

  Is the patient able to mobilise freely or do they require aids or assistance. This may have changed drastically as a part of their admission and they may need reassurance and support with this. This activity can link directly with maintaining a safe environment and elimination.

- **Working and playing**

  Finding out if a patient's recent or on-going illness is effecting or being effected by their work is important. Are they retired or working full or part time? Play also needs to be considered. Play is very important for both children and young people in hospital (Hubbuck, 2009, Stonehouse, 2014), but also for adults as well (Tonkin & Whitaker, 2016). It is important that this part of the assessment is not quickly passed over. Finding out what a patient likes to do, whether they call it play, or a pastime or a hobby, and then facilitating this, will help aid normality within a strange environment as well as many other therapeutic benefits (Bayliss and Etchells, 2016).

- **Sleeping**

  Asking how a patient normally sleeps and has it been effected by their illness is an important question. Asking what time do they normally settle down for bed and what time do they get up in a morning are key to identifying what their normal routine is. Do they take any medication for sleeping and has this been prescribed for them while they will be in hospital? However, the reality is that a patient's sleep is likely to be altered by the unfamiliar clinical environment.

- **Sexuality**

Again, this can be a sensitive activity to discuss with a patient you have only just
met. However, it can be as simple as observing how the patient expresses their
gender through how they dress. Do they have a partner with them or who will be
visiting? If the presenting problem is related to this activity then deeper questions may need to be asked. Does the illness or accident impact upon body image or function which may affect this activity? Think about how the person likes to dress, shave, wear makeup? What makes them feel good, and fulfilled.

- **Death**

Roper et al (2006) state that this activity though included as one of the twelve, is only essential if it is relevant to the patient or the patient wants to discuss this sensitive topic. However any patient could have experienced a loss or bereavement and may wish to open up about it. The support worker needs to be sensitive to this possibility, giving time for the patient to talk if they wish. Again, the development of a therapeutic relationship will assist with this.

The list also includes death as an activity of daily living, but this are often disregarded depending on the setting and situation for the individual service user.

According to the model, there are five factors that influence the activities of living.

The incorporation of these factors into the theory of nursing makes it a holistic model. If they aren't considered, the resulting assessment is incomplete and flawed. The factors are used to determine the individual service user's relative independence in regards to the activities of daily living.

They are: biological, psychological, sociocultural, environmental, and politicoeconomic. The biological factor addresses the impact of the overall health, of current injury and illness, and the scope of the service user's anatomy and physiology. The psychological factor addresses the impact of emotion,

cognition, spiritual beliefs, and the ability to understand. According to Roper, this is about “knowing, thinking, hoping, feeling and believing.”

The sociocultural factor is the impact of society and culture as experienced by the individual service user. This includes expectations and values based on class and status, and culture within the sociocultural factor relates to the beliefs, expectations, and values held by the individual patient for him or herself, as well as by others pertaining to independence in and ability to carry out the activities of daily living.

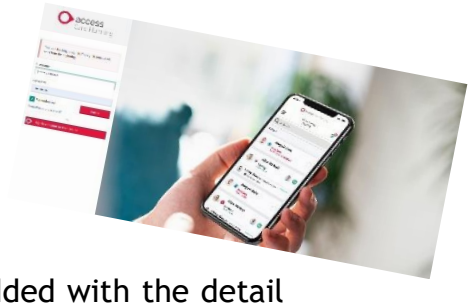
The environmental factor in Roper’s theory of nursing makes it a “green” model. The theory takes into consideration the impact of the environment on the activities of daily living, but also examines the impact of the activities of daily living on the environment. The politicoeconomic factor is the impact of the government, politics, and economy on the activities of daily living. This factor addresses issues such as funding, government policies and programs, war or conflict, availability to benefits, political reforms, interest rates, and availability of public and private funding, among others.

Roper herself objects to the model being used as a checklist. She states that if assessors are uncomfortable discussing certain factors, they assume the service users are, as well. This leads to the nurses attributing the lack of assessment to the service users’s preference, when in actuality, the service user’s preferences were not addressed.

Assesor assertion leads to the conclusion that rather than deleting or disregarding activities of daily living, it can benefit the individual being assessed if the carer uses the model more thoroughly and assesses the service user using the five factors in conjunction with the activities of daily living, regardless of the area in which the care is being received. Roper stated, “The service user is the service user, they are not a different service3 user because they are in a different care area. Their needs are the same- it’s who will meet those needs that changes.”

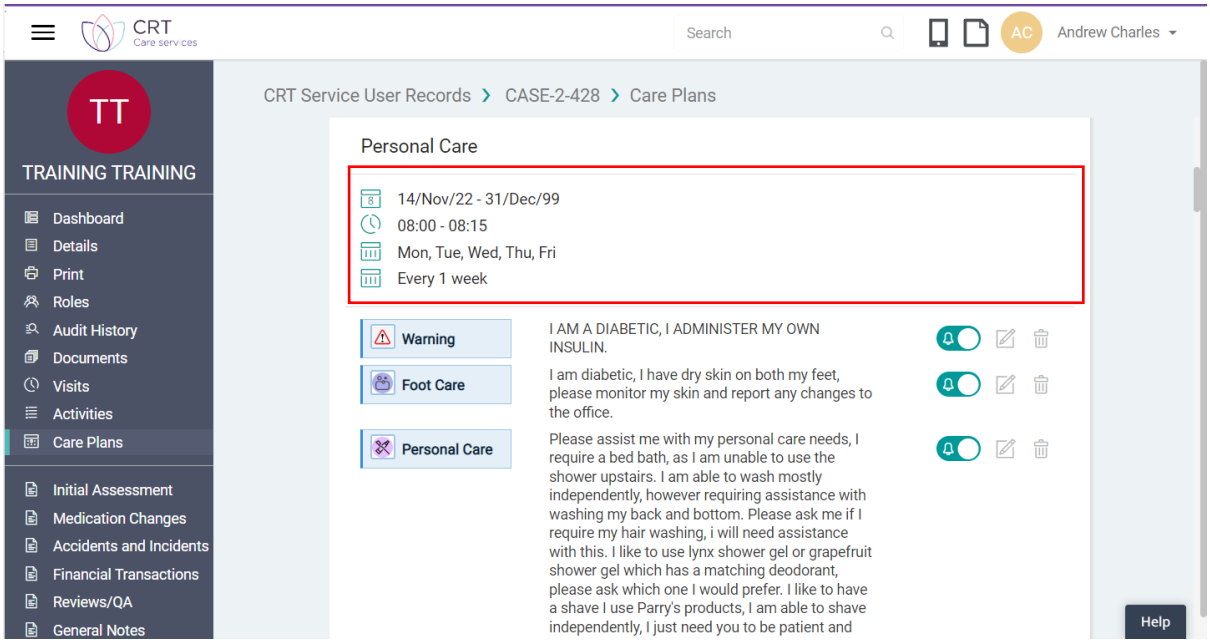
This is why we should ensure that all care plans follow the basics as outlined above.

Compiling the Care Plan



Once we have completed the initial assessment we need to complete the care plan based on the information that we have gathered.

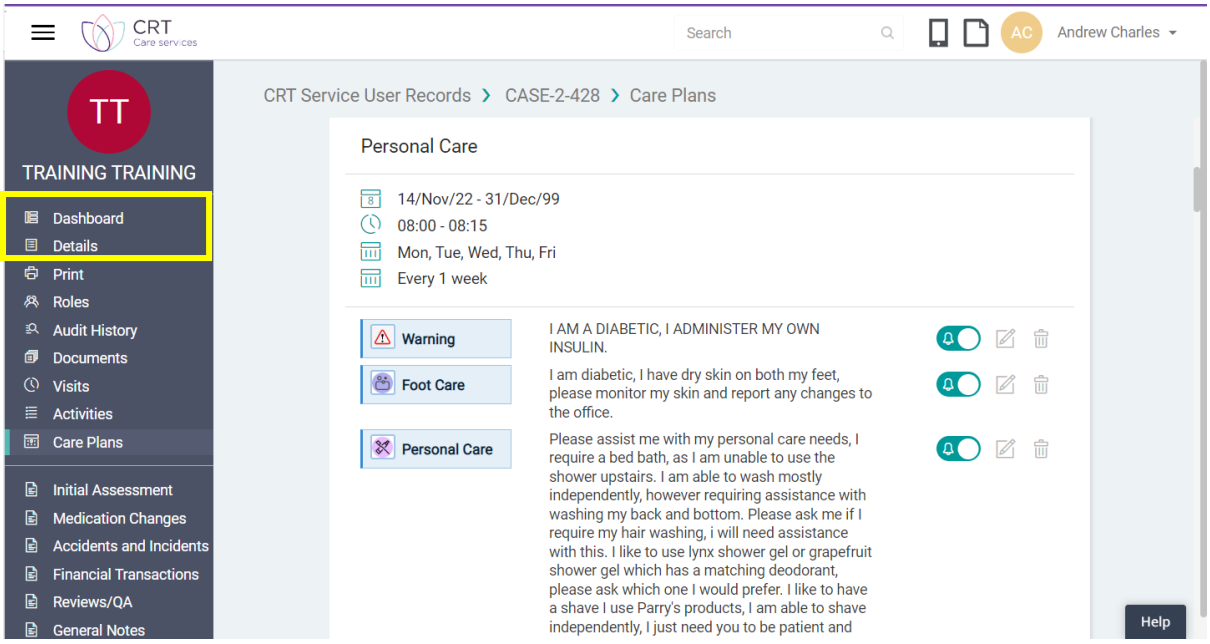
The Care Plan should be compiled as a list of tasks and added with the detail required to show the carer going in to the call what needs to be done and how the person prefers their care to be delivered.



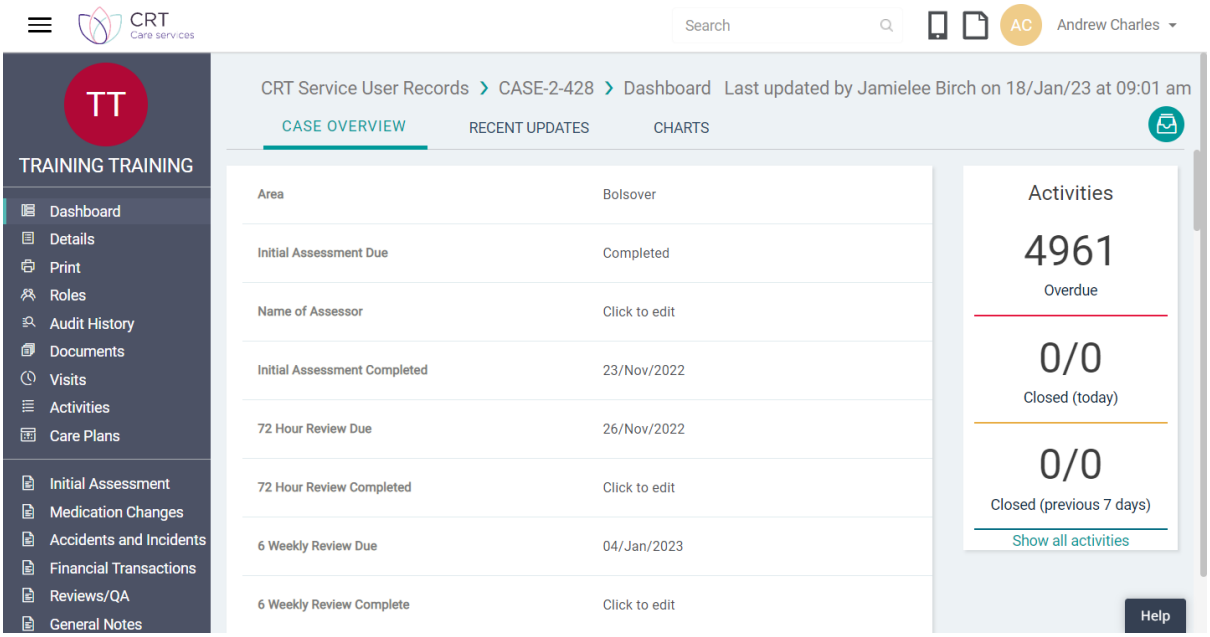
As you can see from the above, the top part shows what the call is for i.e. personal care / Domestic / Cleaning / Social. When the call was started, what time the call is to be carried out, days of the week that the call should be carried out, and is this every week /every other week / once a month. [This is indicted in the red rectangle]

A useful function that we have with the system, as well as being able to add information to the Critical information section, we can all add a warning to each call that shows essential information i.e. falls risk, epileptic or type one diabetic. We will see in a moment how we add a task to the system, and the warning box can be added as a task. Carers will have to click to say they have read this.

Before we start compiling the care plan, it is always worth double checking the dashboard / details. There are areas in these that additional information can be added.



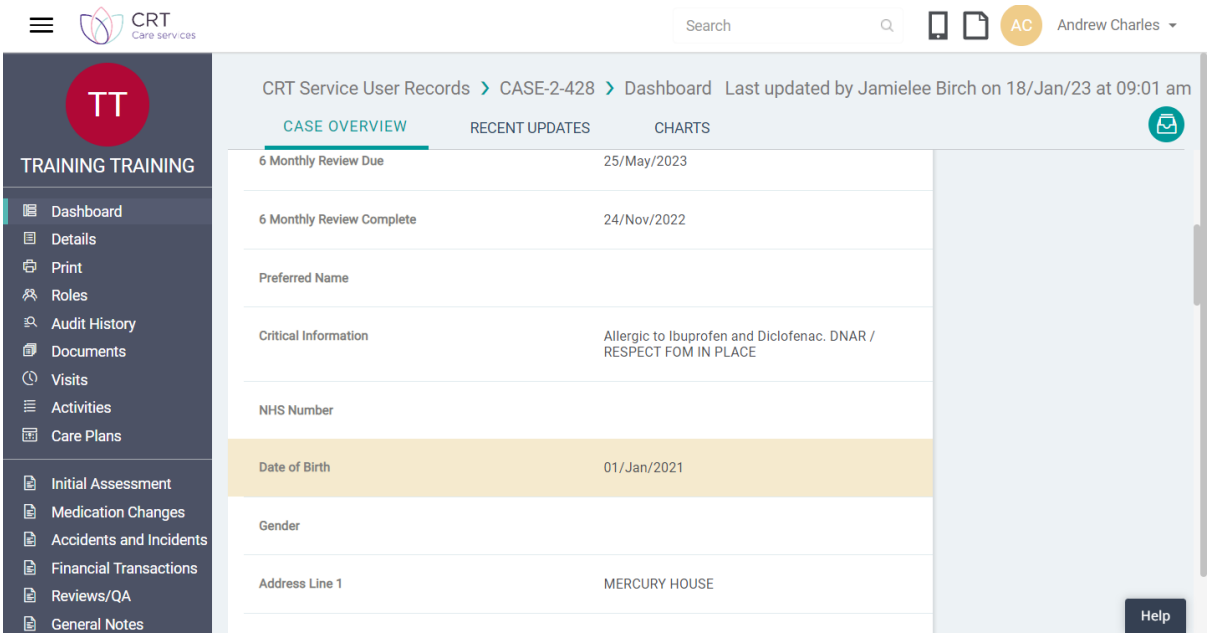
The sections are highlighted above in the yellow rectangle. Click on the dashboard option first, and this should show all the relevant information in relation to this service user.



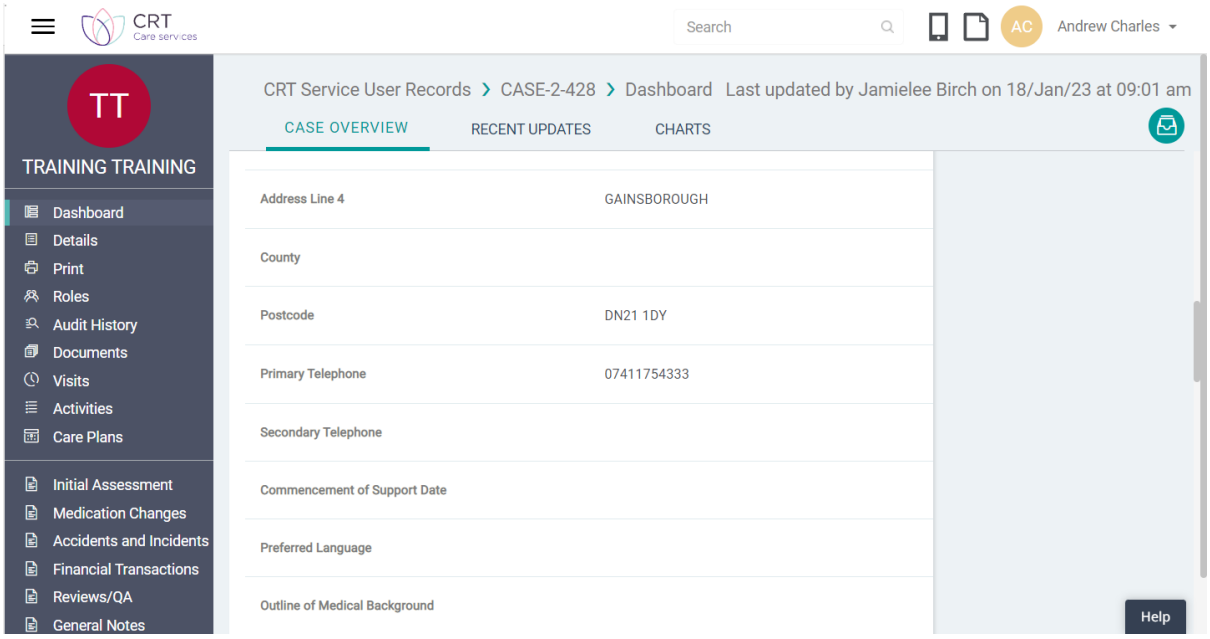
This is the information that is displayed on the dashboard screen, it shows the area that the service user lives, has their assessment been completed, name of the assessor.

Any section that has the line click to edit, means we can add additional information.

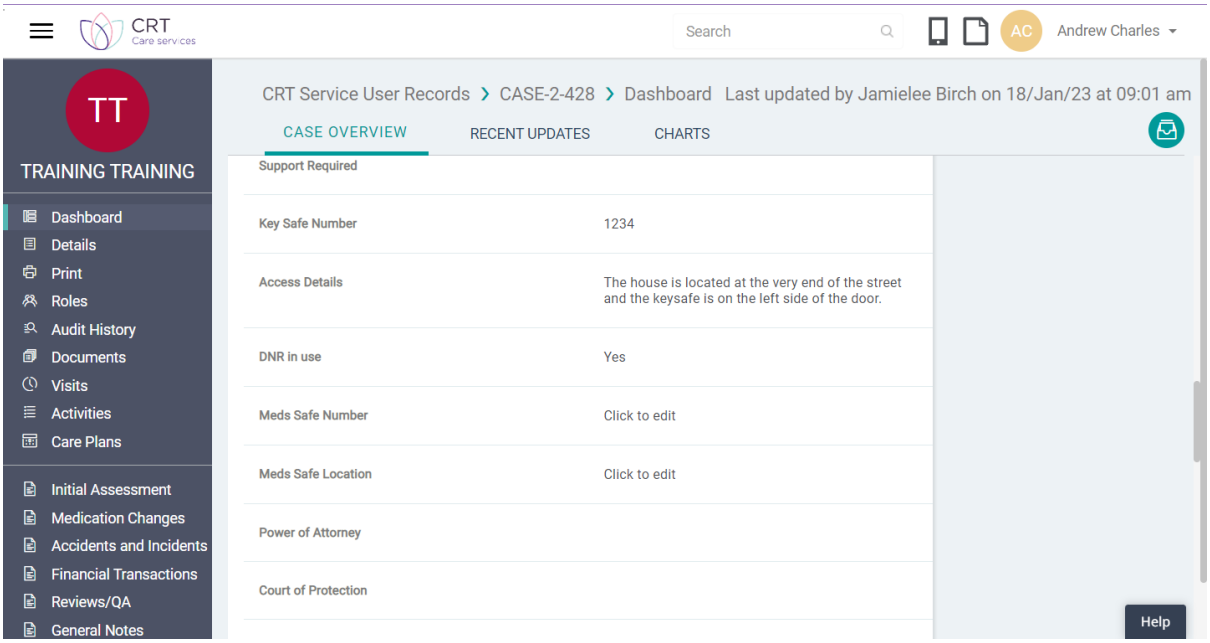
Most information is dragged across from People Planner when the package is setup.



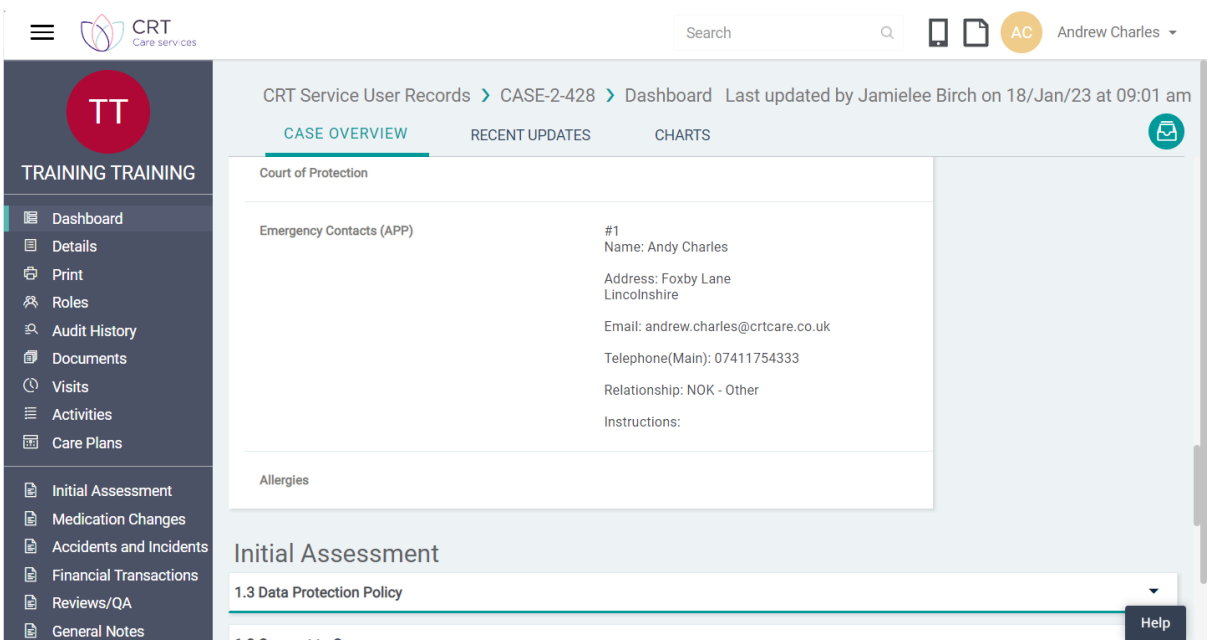
The basic information about the person i.e. name, DOB and their address is documented here.



Other important information is listed i.e. preferred language.

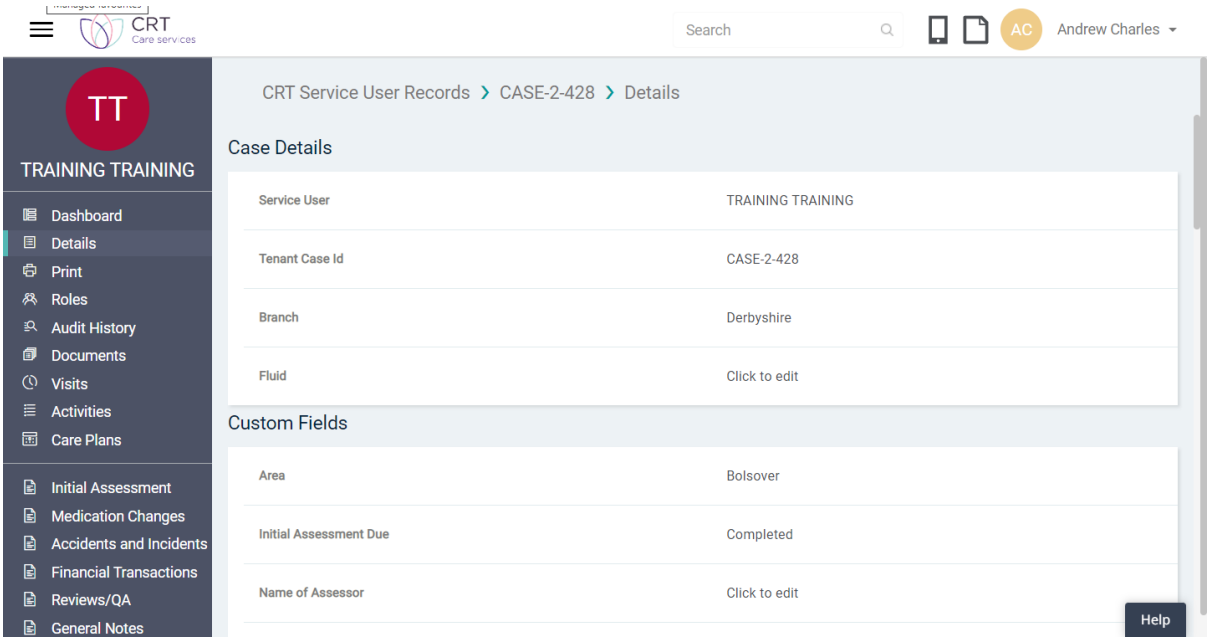


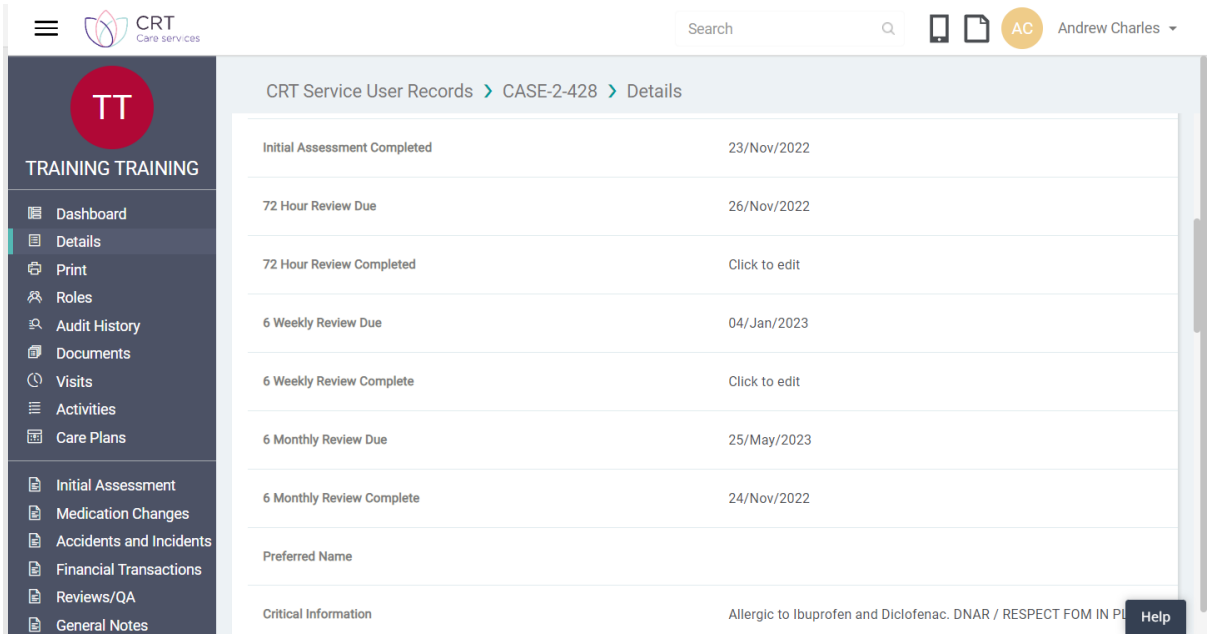
It is important to document things like the access code to the key safe so carers can gain access to the property, also please document under the section access details, not just which door to use or where to go, but also the location of the key safe. As they are not always readily visible i.e. by the front door. i.e. if the key is located under the windowsill on the left hand side of the bay window to the front of the house.

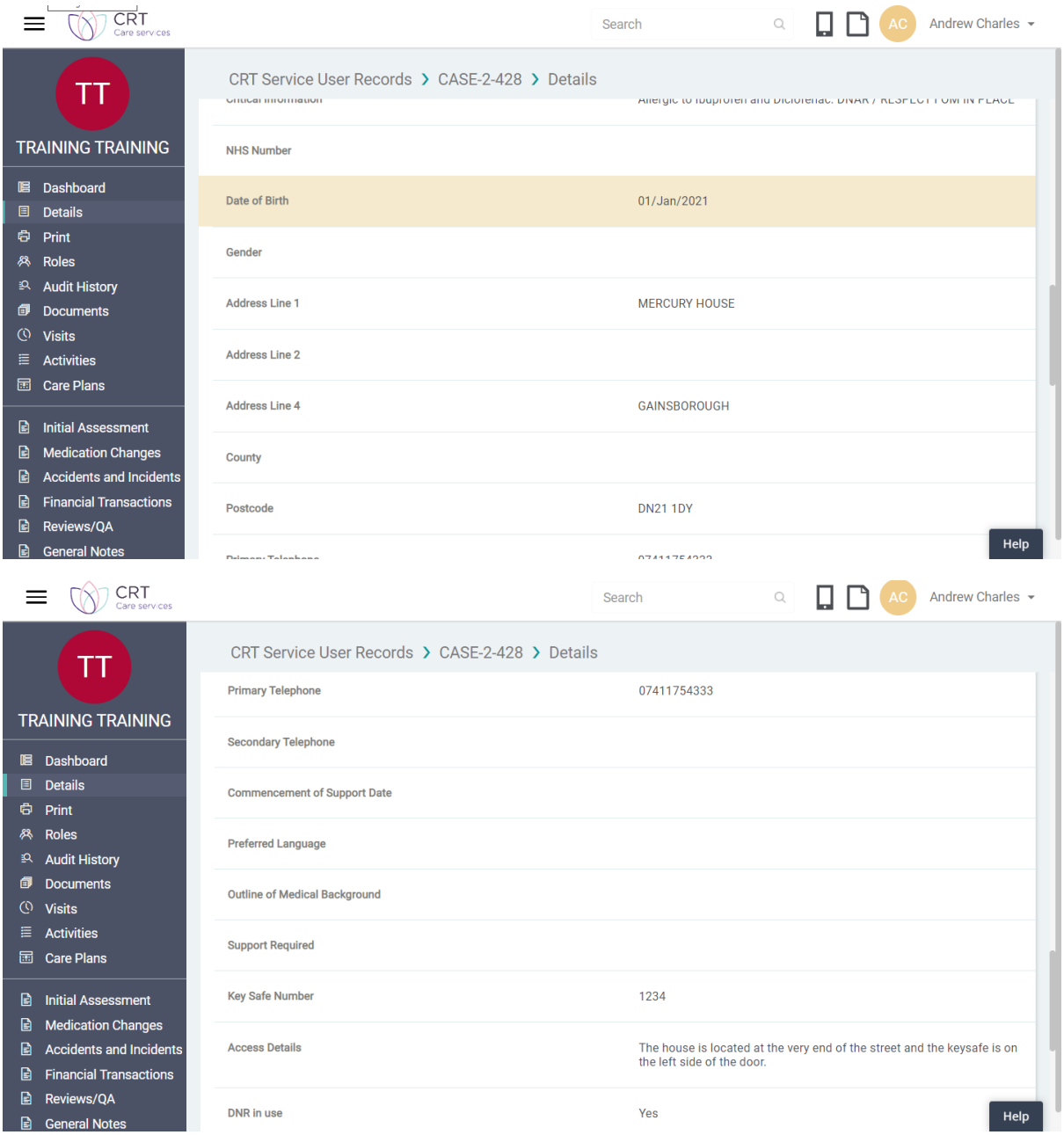


Contacts need to be added in people planner, and then they will appear here. Ensure that we are documenting all numbers i.e. family, friends, suppliers, Dr's, District nurses, SaLT, Physiotherapists, and Social Workers.

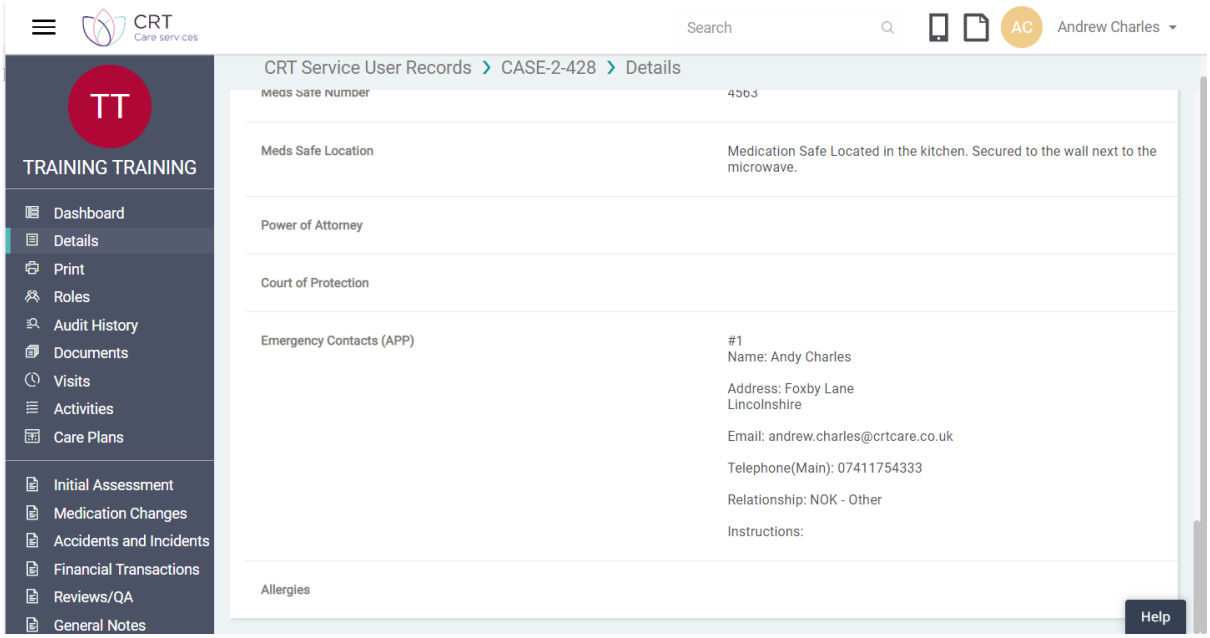
Under the Details section we are presented with the following information:


CRT
Care services
Search
AC
Andrew Charles
TT
TRAINING TRAINING
Dashboard
Details
Print
Roles
Audit History
Documents
Visits
Activities
Care Plans
Initial Assessment
Medication Changes
Accidents and Incidents
Financial Transactions
Reviews/QA
General Notes
CRT Service User Records › CASE-2-428 › Details
Case Details
Service User
TRAINING TRAINING
Tenant Case Id
CASE-2-428
Branch
Derbyshire
Fluid
Click to edit
Custom Fields
Area
Bolsover
Initial Assessment Due
Completed
Name of Assessor
Click to edit
Help


CRT
Care services
Search
AC
Andrew Charles
TT
TRAINING TRAINING
Dashboard
Details
Print
Roles
Audit History
Documents
Visits
Activities
Care Plans
Initial Assessment
Medication Changes
Accidents and Incidents
Financial Transactions
Reviews/QA
General Notes
CRT Service User Records › CASE-2-428 › Details
Initial Assessment Completed
23/Nov/2022
72 Hour Review Due
26/Nov/2022
72 Hour Review Completed
Click to edit
6 Weekly Review Due
04/Jan/2023
6 Weekly Review Complete
Click to edit
6 Monthly Review Due
25/May/2023
6 Monthly Review Complete
24/Nov/2022
Preferred Name
Critical Information
Allergic to Ibuprofen and Diclofenac. DNAR / RESPECT FOM IN PL
Help

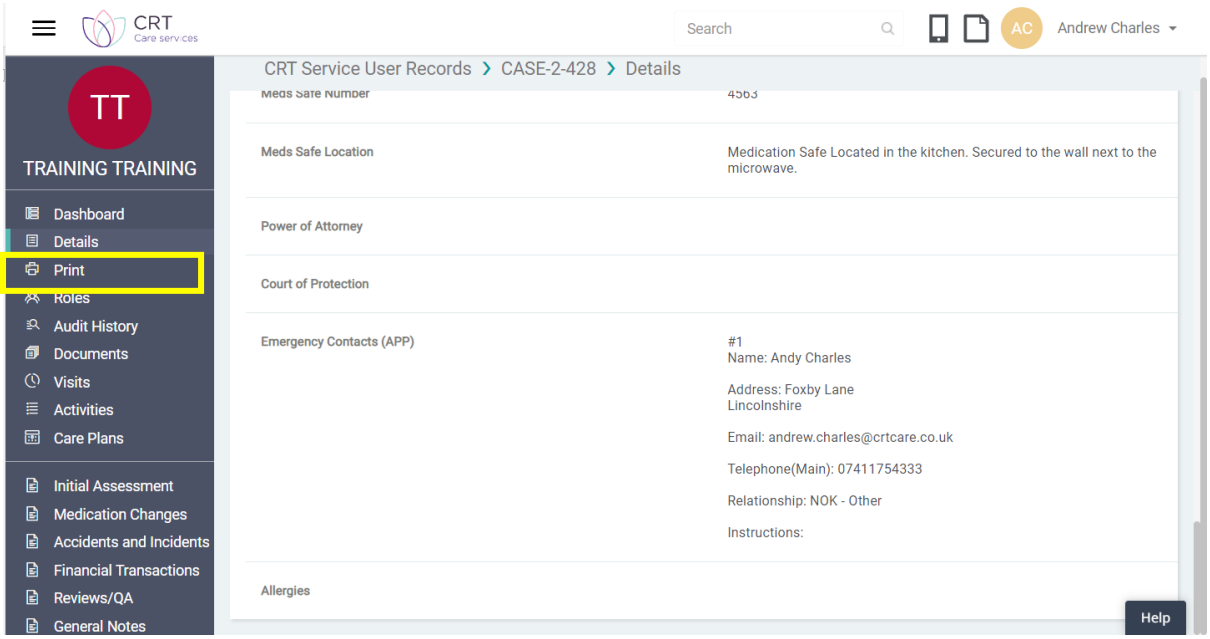


As you can see from the above if the person has a medication safe, this is listed here and the code needed to gain access and the location of the safe should be documented. In the care plan explain under a medication tab / warning tab, why the medication is locked away. This is an unusual step in a person's own home. And only carried out when there is a significate risk i.e. Dementia, Mental Health or Risk of overdose.

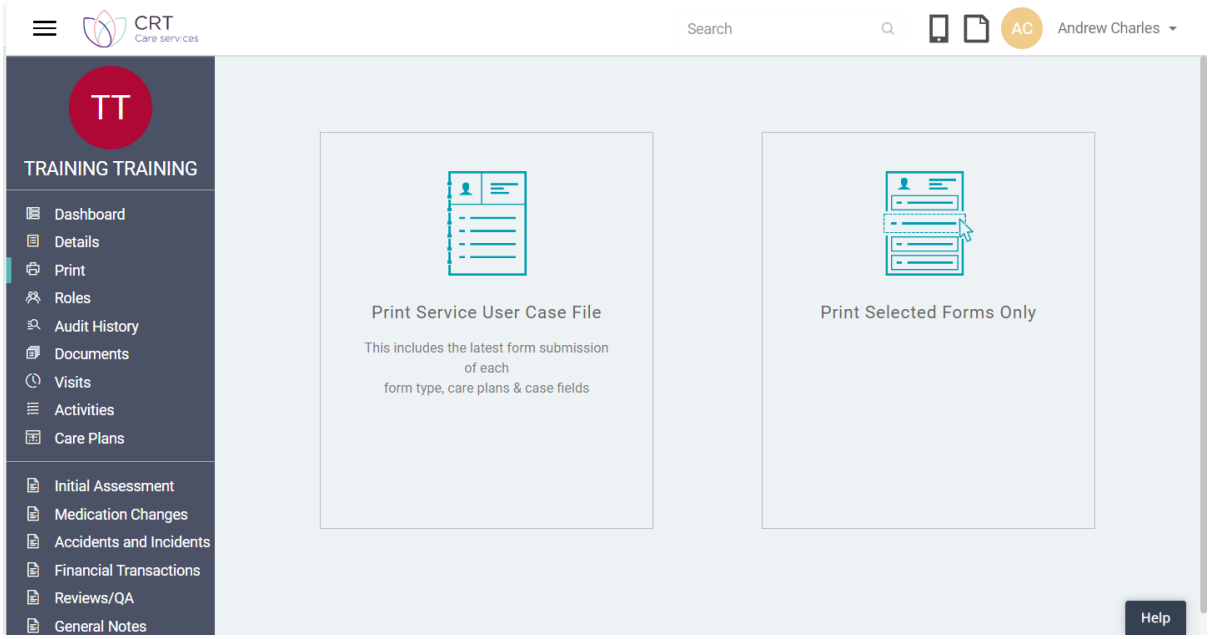


As can be seen from the above the emergency contacts are all listed above.

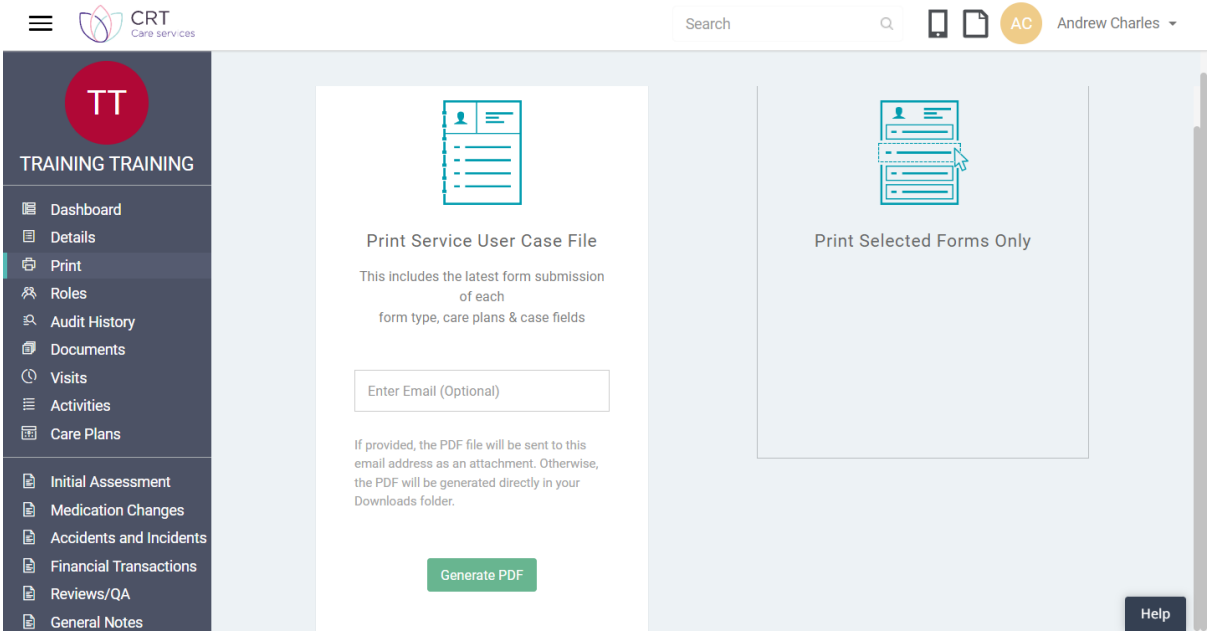
We can also print a simple hospital passport using the print function here. [Yellow rectangle]

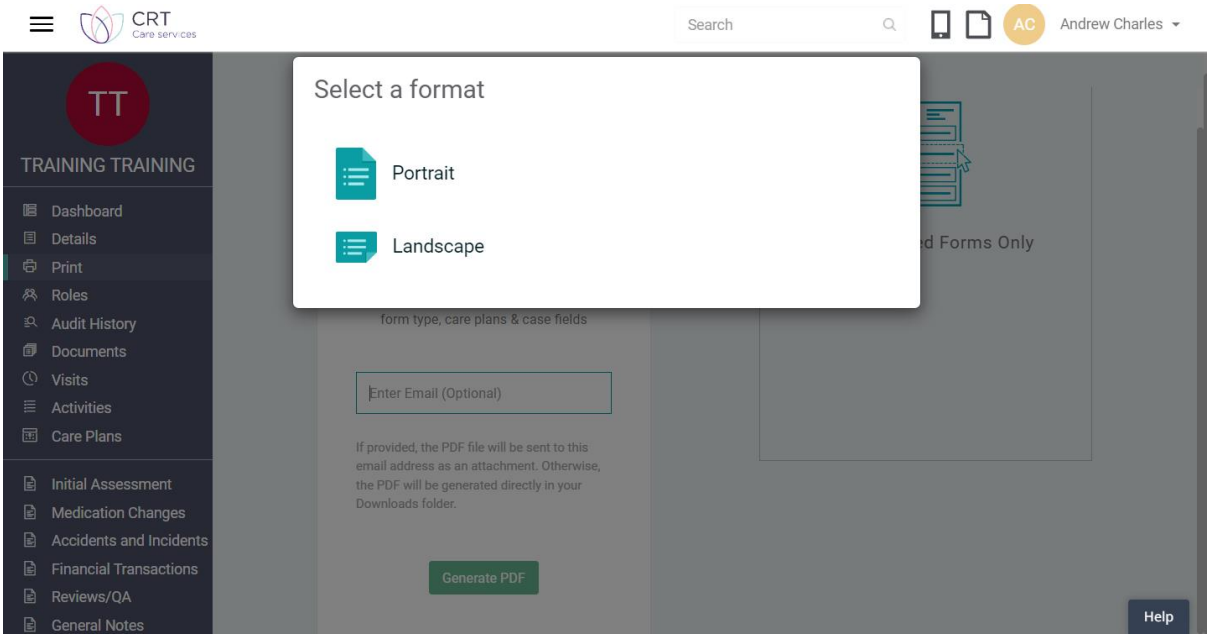


You will then be presented with the following screen.



Select the print the service user case file. You will then be given the option to enter an email to directly send the case file to someone. It is better not to do this, and just click the generate PDF option.

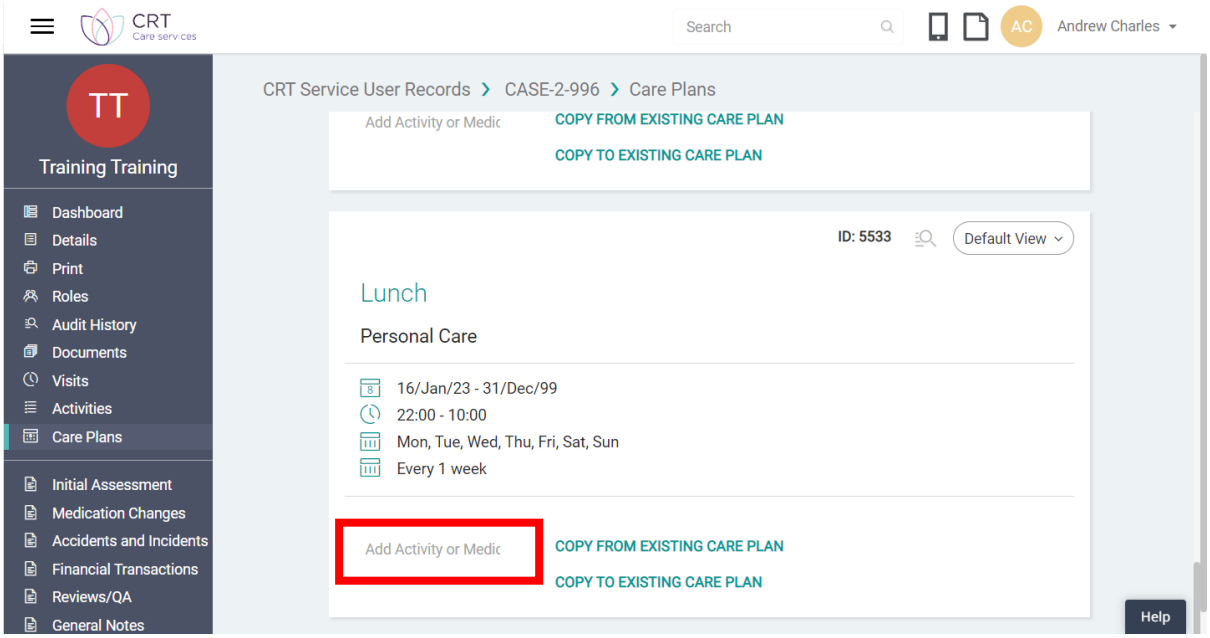




Select the portrait option. The system will display Generating PDF, please wait and then the file will automatically down load and show at the bottom of the screen, you can also access it from the folder option on the task bar. When the folder options come up look in the left-hand list, and click downloads, and your file should be there.

The example will be shown in the appendices to this document. You can remove the pages that are not relevant by deleting them from the PDF document. The version that is located in the appendices is not a true reflection as it has been converted to a word document to show information that it contains. [Appendices Three]

This is also a useful function to create a hard copy for the service user, should it be required.

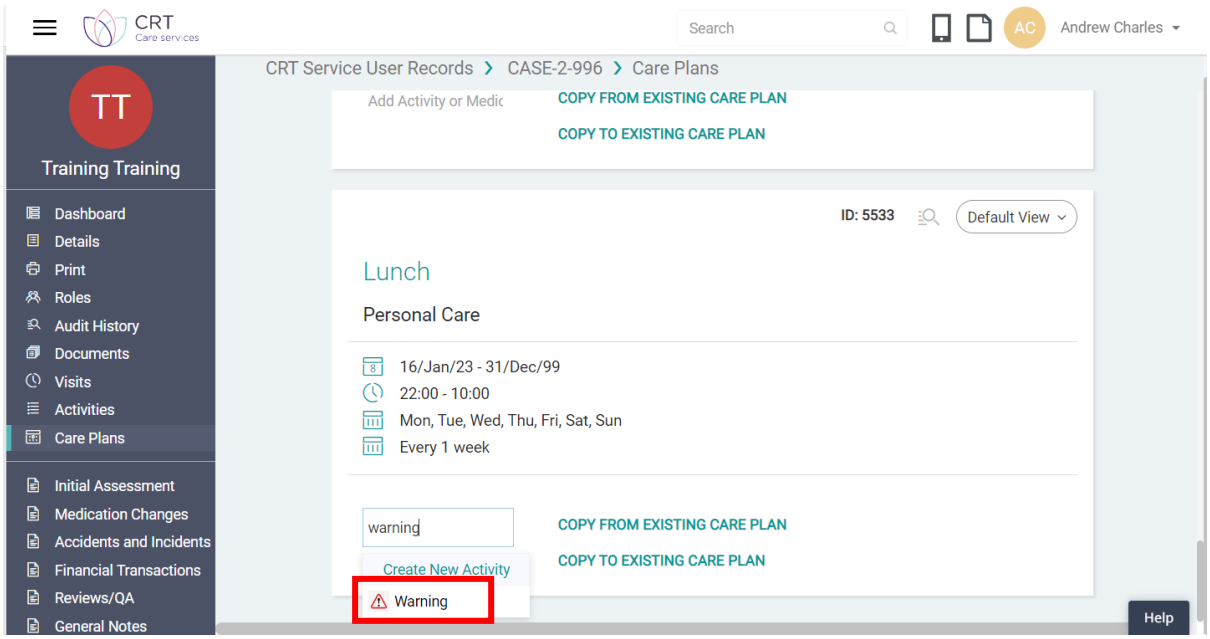


Go to the area that we need to generate the care plan. As we can see from above the call has no information, contained within to let the carers know how to complete tasks for service user.

We need to go to the Add activity or Medication, this will let us add the activity or suggestion medications to add.

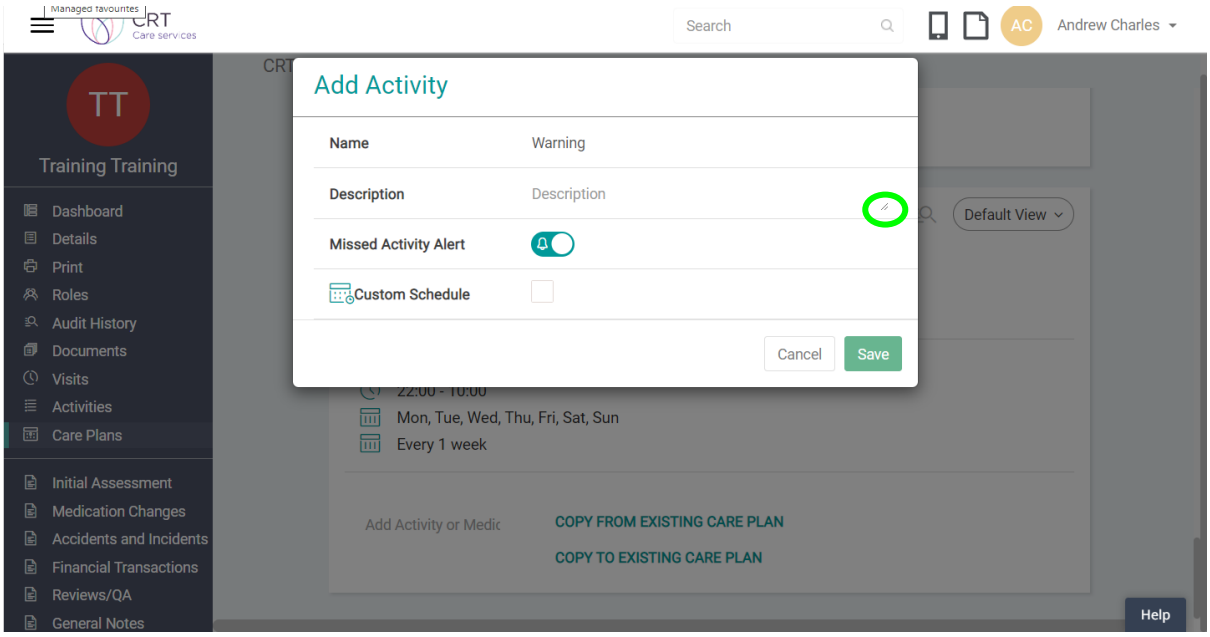
If we have critical information type in the word warning.

It will take you to this screen:

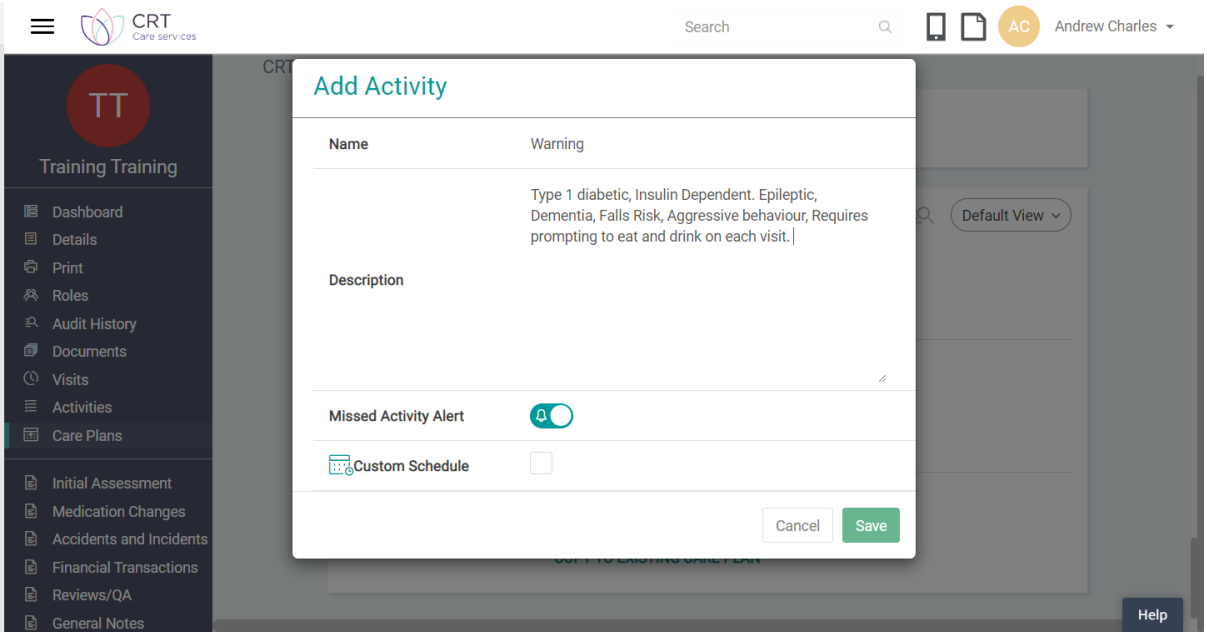


Click the red triangle indicated by the red rectangle above.

When you click the rectangle, it will show you this screen:

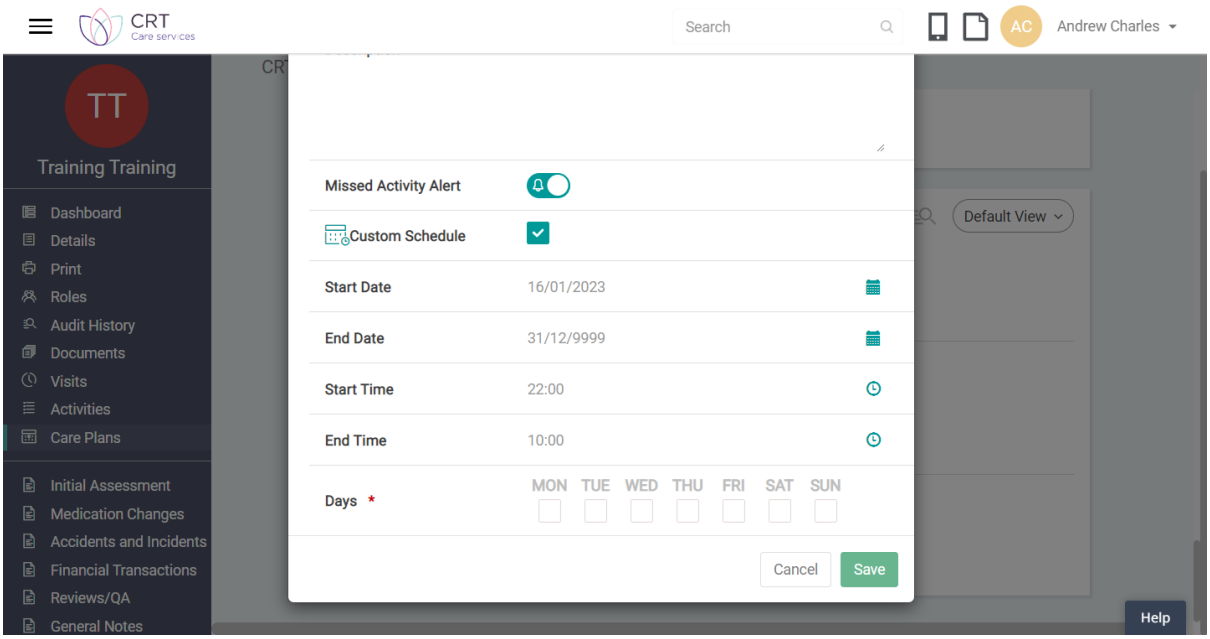


Click in to the description box, and you can type the information required into the box. There is the option to drag the box bigger, so you can read everything that you have written. See green circle above. It makes live easier and you don't have to keep scrolling up and down to read what has been written.



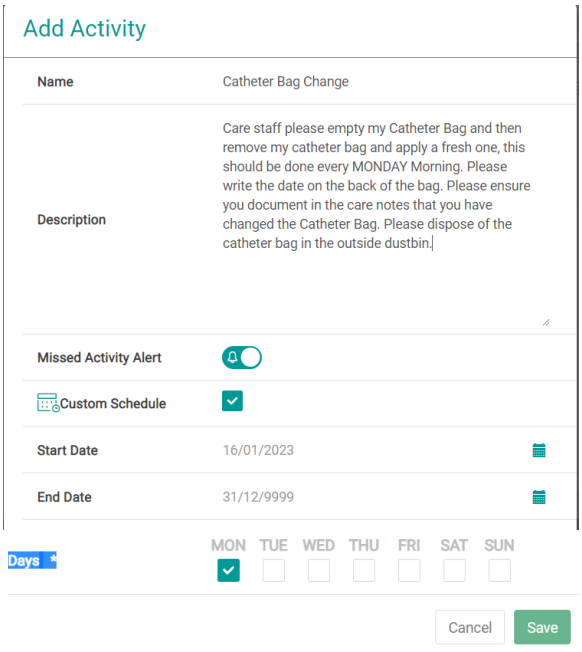
As you can see from the above you can write any relevant information that you need the carers to be aware of on each visit.

We can also make this a custom task, which means it will only show on certain days. We do this by clicking the box next to where it says custom schedule.



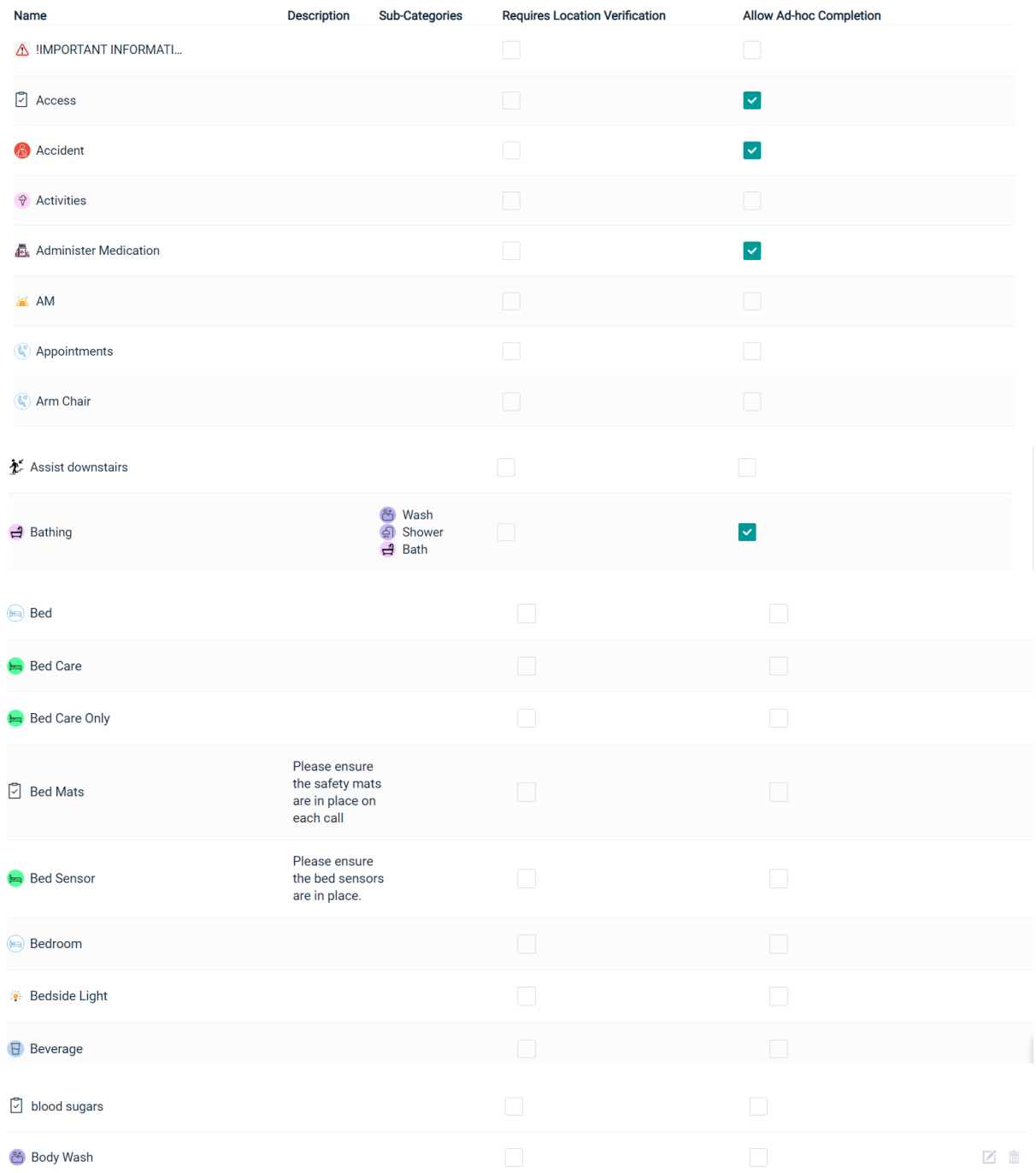
So if you need the warning on certain days or for a limited period we can set this by using the options above. All you need to do is click the days you need the warning to show, and if this is time bound enter the start and end date. Start and End date does not need to be added, you can just enter the days you want this to show.

We can do this with tasks as well, so if you want carers to change catheter bags on a Monday morning we would use the catheter task and set the options below:



As you can see from the above I have written what the person needs to do, explained the day that the change needs to happen on and then set the schedule, so this task will only show on a Monday. We can do this with medications as we will look at later.

This is a list of the different activities we can add to the care plan:



Name	Description	Sub-Categories	Requires Location Verification	Allow Ad-hoc Completion
!IMPORTANT INFORMATI...			☐	☐
Access			☐	☑
Accident			☐	☑
Activities			☐	☐
Administer Medication			☐	☑
AM			☐	☐
Appointments			☐	☐
Arm Chair			☐	☐
Assist downstairs			☐	☐
Bathing		Wash Shower Bath	☐	☑
Bed			☐	☐
Bed Care			☐	☐
Bed Care Only			☐	☐
Bed Mats	Please ensure the safety mats are in place on each call		☐	☐
Bed Sensor	Please ensure the bed sensors are in place.		☐	☐
Bedroom			☐	☐
Bedside Light			☐	☐
Beverage			☐	☐
blood sugars			☐	☐
Body Wash			☐	☐

Name	Description	Sub-Categories	Requires Location Verification	Allow Ad-hoc Completion	
Boots			☐	☐	
Bottle Water			☐	☐	
Breakfast			☐	☐	
Breathing Equipment			☐	☐	
Brush Hair			☐	☐	
Call			☐	☐	
Call Time			☐	☐	
Catheter			☐	☐	
Catheter Bag Change	Care staff please empty my Catheter Bag and then remove my catheter bag and apply a fresh one, Please write the date on the back of the bag. Please ensure you document in the care notes that you have chnaged the Catheter Bag. Please dispose of the catheter bag in the outside dustbin.		☐	☐	
Cats			☐	☐	
Ceiling Hoist Transfers			☐	☐	
Ceiling Track Hoist			☐	☐	
Chat			☐	☐	
Check			☐	☐	
Check / Ensure Pendant is...			☐	☑	
Check Appliances			☐	☐	
Check Bins			☐	☐	
Check Dates	Check expiration dates on food in cupboards and fridge.		☐	☐	
Check Food			☐	☐	
Check Fridge			☐	☐	

Name	Description	Sub-Categories	Requires Location Verification	Allow Ad-hoc Completion
Check list			☐	☐
Chip pan			☐	☐
clean carpet			☐	☐
Clean Dentures			☐	☑
clean my ears out			☐	☐
Clean Teeth			☐	☐
Clean Teeth			☐	☐
Cleaning tasks			☐	☐
Clear pathways			☐	☐
Clothes			☐	☐
Clothing/Dressing			☐	☐
Commode			☐	☐
Communication	I am Hard of hearing, care workers please speak clearly and look at me when speaking, Please ask me if I understand what is been said to me.		☐	☐
Contact family			☐	☐
Continence needs			☐	☐
Conversation			☐	☐
Cook Meal			☐	☑
Cooker			☐	☐
Covid-19 Precautions	Please ensure full PPE is worn at all times, Please ensure Donning and doffing procedures are adhered too.		☐	☐
Crumbs			☐	☐
Curtains	Open / shut curtains		☐	☑
dementia			☐	☐
Dentures			☐	☐
Diabetes			☐	☐
Dietary Intake			☐	☐
District Nurse			☐	☐
Dogs			☐	☐
Domestic - Clean Up / Wa...			☐	☑

Name	Description	Sub-Categories	Requires Location Verification	Allow Ad-hoc Completion

Domestic - Dry Clothes / ...		☐	☑
Domestic - Dust / Polish		☐	☑
Domestic - Empty Bins		☐	☑
Emergency Contacts		☐	☐
Empty / Clean Commode		☐	☑
Empty Bins		☐	☐
Encourage eating	Please encourage me to eat	☐	☐
Encourage Fluids	Please can you encourage me to drink my altra plan while you are there and encourage me to drink plenty of fluids at each visit	☐	☐
Encourage Walking		☐	☐
Ensure insulin has been ta...		☐	☐
Ensure managing diabetes		☐	☐
Ensure Walker is within re...		☐	☑
Ensure walking stick is in ...		☐	☑
Enter	Please enter via back door not front door	☐	☐
Enter Property		☐	☐
Equipment		☐	☐
Escort		☐	☐
Exercise		☐	☐
Eye Drops		☐	☐
Fall		☐	☑
Financial Transaction Rec...	Please complete the financial transaction record book. Ensure you have documented how much I have given you, how much you have spent and how much change I received. Place all receipts inside the financial transaction book and ensure it is signed by both of us.	☐	☐
Fire Blanket		☐	☐
Flannels	Please ensure to leave my Flannels over the bath near the soap box as these keep getting thrown away by mistake.	☐	☐

Name	Description	Sub-Categories	Requires Location Verification	Allow Ad-hoc Completion
Flask			☐	☐
Fluid - Output			☐	☑
Fluid for Chart	Fluids given to appear on a fluid chart		☐	☐
Fluids			☐	☐
Fluids - Input	Please provide me with a drink of my choosing and record what I have been given	Water Hot Drink Juice Other Multiple	☐	☑
Food Chart			☐	☐
Foot Care			☐	☐
Fridge Freezer			☐	☐
Gantry Hoist Transfers			☐	☐
Gas Fire			☐	☐
General chat			☐	☐
Getting out of bed			☐	☐
Glasses	Ensure glasses are being worn		☐	☐
Hair wash			☐	☐
Health Warning			☐	☐
hearing			☐	☑
Hearing Aid	Ensure hearing aid is being worn		☐	☐
Heating			☐	☐
Hoist Transfers			☐	☐
Hot Drink			☐	☐
Hot Water Bottle			☐	☐
Housekeeping			☐	☐
Hydration	Offer a drink	Water Hot Drink Juice Other Drinks Multiple Drinks	☐	☑

Name	Description	Sub-Categories	Requires Location Verification	Allow Ad-hoc Completion

Name	Description	Sub-Categories	Requires Location Verification	Allow Ad-hoc Completion
Hygiene			☐	☐
important information			☐	☐
Incontinence wear			☐	☐
Infection Control			☐	☐
Information			☐	☐
Inhaler Check			☐	☐
Into bed			☐	☐
Kettle	Please make sure my kettle is filled with water after use		☐	☐
Keysafe			☐	☐
Lamp			☐	☐
Laundry			☐	☐
Leave TV on until bedtime			☐	☐
Leg Bag			☐	☐
Light			☐	☐
Lighting			☐	☐
Lock Door			☐	☐
Lower Garments			☐	☐
Lunch			☐	☐
Mail			☐	☐
Make sure Door is secure			☐	☐
Massage			☐	☐
Meal Preparation			☐	☑
Medication			☐	☑
Medication check			☐	☑
memory			☐	☐
Microwave			☐	☐
Mobility			☐	☐

Monitor			☐	☐
Mood		Happy Indifferent Sad Anxious Irritable Depressed Tearful Agression	☐	☑
Moving and Handling			☐	☐
Night Bag			☐	☐
Night Checks			☐	☑
Night Sit			☐	☐
Night time routine			☐	☐
Nightwear			☐	☐
Nutrition (Meal)		Breakfast Lunch Dinner Snack Outing Other	☐	☑
Nutrition (Monitored)		Full Plate 3/4 Plate Half Plate 1/4 Plate	☐	☑
Nutrition Chart	Food to be logged for nutrition chart		☐	☐
Oral Care			☐	☐
Out Of Date Food			☐	☐
Oxypro mr 10mg	Take one tablet twice a day		☐	☐
Pack up for Dialysis			☐	☐
Pad change			☐	☐
Pad Check		Dry Wet	☐	☑
Pain			☐	☐
parkinsons'			☐	☐
Pendant			☐	☐
Personal Care	Please detail personal care required (i.e. Brushing teeth, washing / drying hair / nail care / dressing)		☐	☑

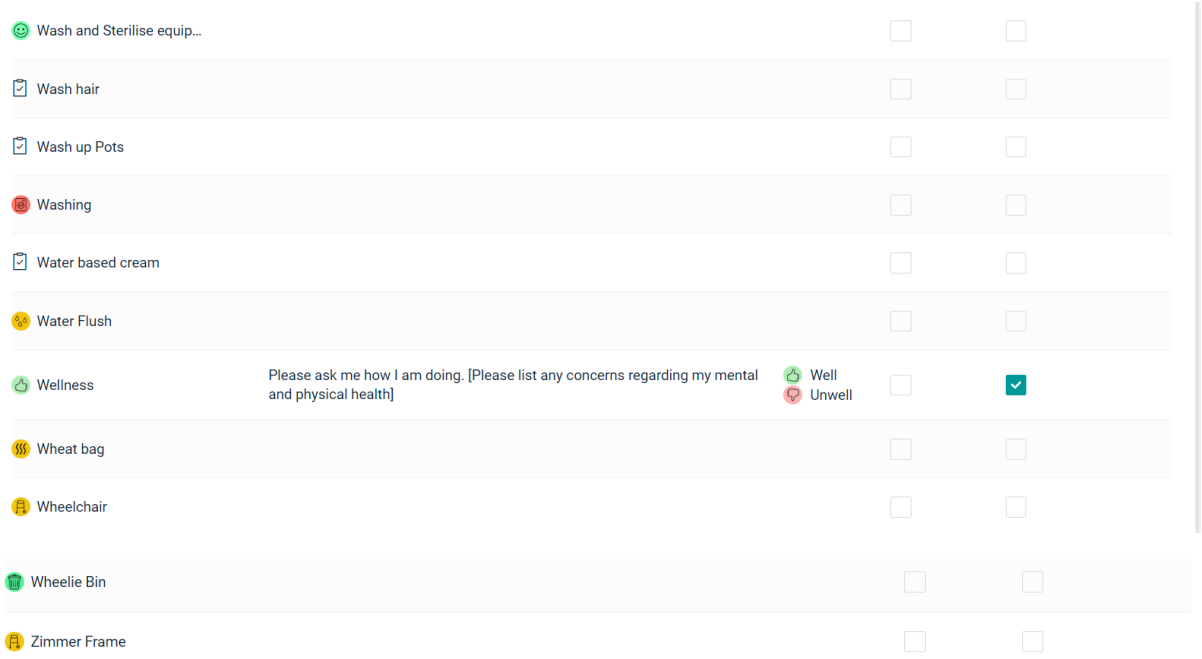
Name	Description	Sub-Categories	Requires Location Verification	Allow Ad-hoc Completion

Name	Description	Sub-Categories	Requires Location Verification	Allow Ad-hoc Completion
Pharmacy			☐	☐
Phone			☐	☐
Plug Sockets			☐	☐
PM			☐	☐
Post			☐	☐
PPE	Please ensure that you are wearing your PPE Properly and disposing it in line with DONNING and DOFFING protocol		☐	☐
Prepare Clothes			☐	☐
Prescription Collection			☐	☑
Pressure Boots			☐	☐
Pressure Care			☐	☐
Prompt Antibiotic			☐	☐
Prompt Ferrous Sulphate	Please prompt me to take my Ferrous Sulphate every morning and every night until course complete.		☐	☐
Prompt Furosemide			☐	☐
Prompt Inhaler			☐	☐
Prompt Macrogol Compo...			☐	☐
Prompt Medication			☐	☐
Prompt Steroids			☐	☐
Pull Cord			☐	☐
Pureed Diet			☐	☐
Push Fluids			☐	☐
Put to bed			☐	☐
Recycling			☐	☐
Refreshments			☐	☐
Remind	Please remind me to write/read my red book		☐	☐
Remote			☐	☐
Reposition			☐	☐

Name	Description	Sub-Categories	Requires Location Verification	Allow Ad-hoc Completion
Re-positioning	Reposition the resident and record any comments	Reposition - Left Reposition - Right Reposition - Up Reposition - Down Sit up in chair Sit up in bed Laid down in bed	☐	☑
Risedronte sodium			☐	☐
Rothercare			☐	☐
Rotunda Transfers			☐	☐
Safe			☐	☐
Safe property	Make sure the property is safe to leave		☐	☐
Salbutamol Nebuliser			☐	☐
Sandwich for lunch			☐	☐
Scabies Precautions	Please ensure full PPE is worn at all times, Please ensure Donning and doffing procedures are adhered too.		☐	☐
Seating Position			☐	☐
Secure Property			☐	☐
Security			☐	☐
Security Camera			☐	☐
Shaver			☐	☐
Shoes			☐	☐
Shopping			☐	☑
Shower			☐	☐
Skin Care	monitor my skin for any pressure areas, any issues record and report to the office. District nurses number 01709 426600		☐	☐
Skin Integrity			☐	☐
Snacks			☐	☐
Social Contact			☐	☐
Sores			☐	☐
speech			☐	☐
Stair lift			☐	☐

Name	Description	Sub-Categories	Requires Location Verification	Allow Ad-hoc Completion
Stand aid				
Sterilise equipment				
Stockings				
Stoma Bag				
Stool	Please monitor SU's stool.	Severe Constipation Mild Constipation Normal Lacking Fabire Mild Diarrhea Severe Diarrhea		☑
Stool Chart	Use the Bristol Stool Chart to Document	Type 1 Bristol Stool Chart, Separate hard lumps like nuts hard to pass Type 2 Bristol Stool Chart - Sausage Shaped but lumpy Type 3 Bristol Stool Chart - Like susage, but with cracks on the surface Type 4 Bristol Stool Chart - Like a sausage or snake, smooth and soft Type 5 Bristol Stool Chart - Soft blobs with clear-cut edges [passed easily] Type 6 Bristol Stool Chart - Fluffy pieces with ragged edges, a mushy stool Type 7 Bristol Stool Chart - Watery, no solid pieces, entirely liquid		☑	

Name	Description	Sub-Categories	Requires Location Verification	Allow Ad-hoc Completion
Strip wash			☐	☐
Supervise			☐	☐
supper			☐	☐
Support into the communi...			☐	☐
Support Stockings	Please assist with with my support stockings using the aid provided		☐	☐
Table			☐	☐
Tasks			☐	☐
Tea			☐	☐
Teeth			☐	☐
Television			☐	☐
Toilet			☐	☐
Toileting		Pad Change Bed Pan Emptied Taken to Toilet Commode Used Bowel Movement	☐	☑
Transfers			☐	☐
Tumble Dryer			☐	☐
Turn lights off			☐	☐
Turn off shower			☐	☐
Turn tv on			☐	☐
turtle			☐	☐
TV			☐	☐
Undressing			☐	☐
Up, Wash, Dress			☐	☐
Urine Bottles			☐	☐
Wake Up			☐	☐
Walking aid			☐	☐
Warning			☐	☐
Wash			☐	☐

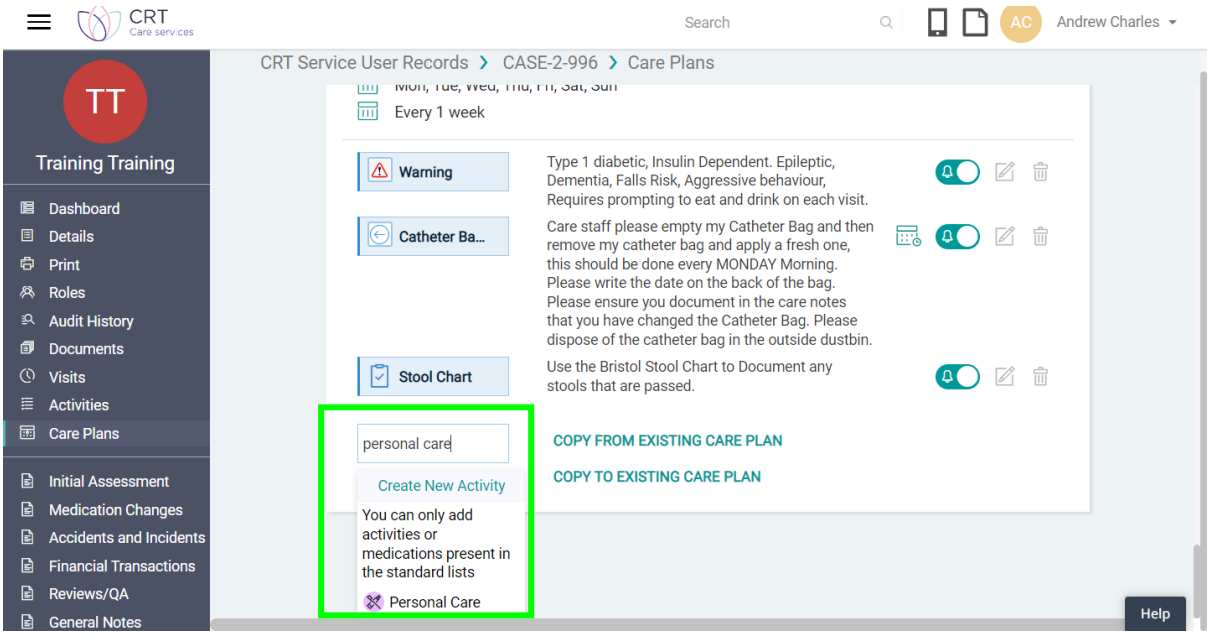


This is a list of the current icons and activities that you may use on Care Planner. If you need an additional activity these can be added, within Care Planner it's self.

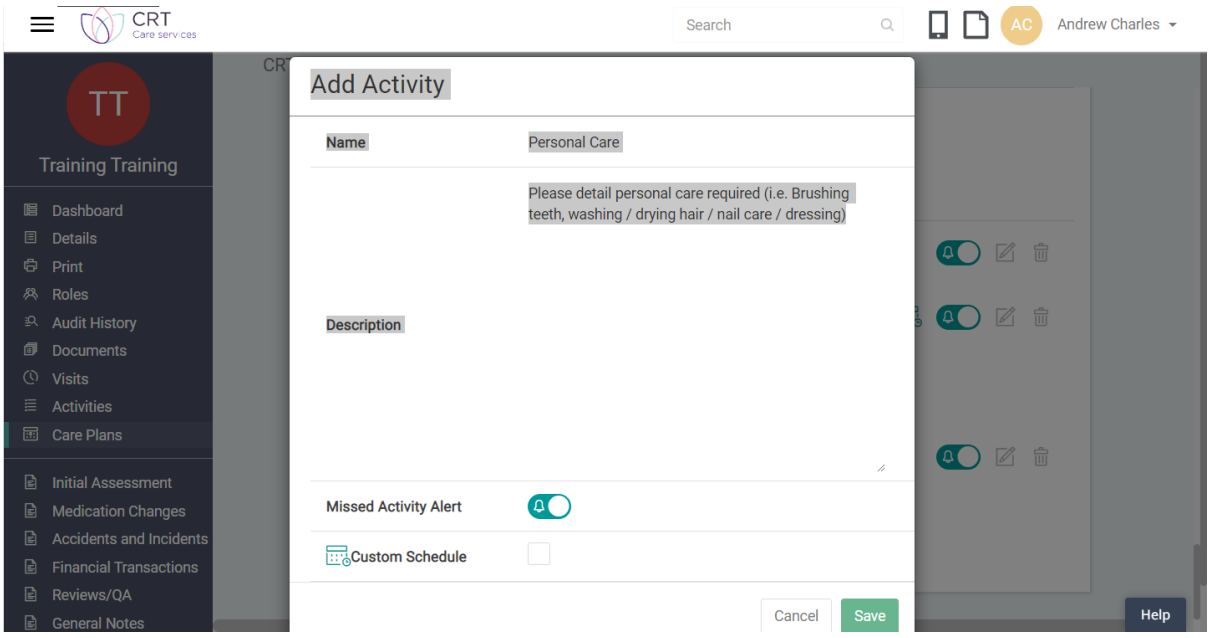
It is useful to have the list of activities near you when doing the care plan as it can serve as a prompt when writing your care plans.

We need to add the care tasks for each visit and what we are expecting the person to do, when they are carrying out the care.

If personal care is to be done we have a number of ways that we can add this, as can be seen on the previous pages i.e. Bathing, Showering, personal care. So if the person changes between a Strip wash [bed bath], bath or shower I would select the personal care option.

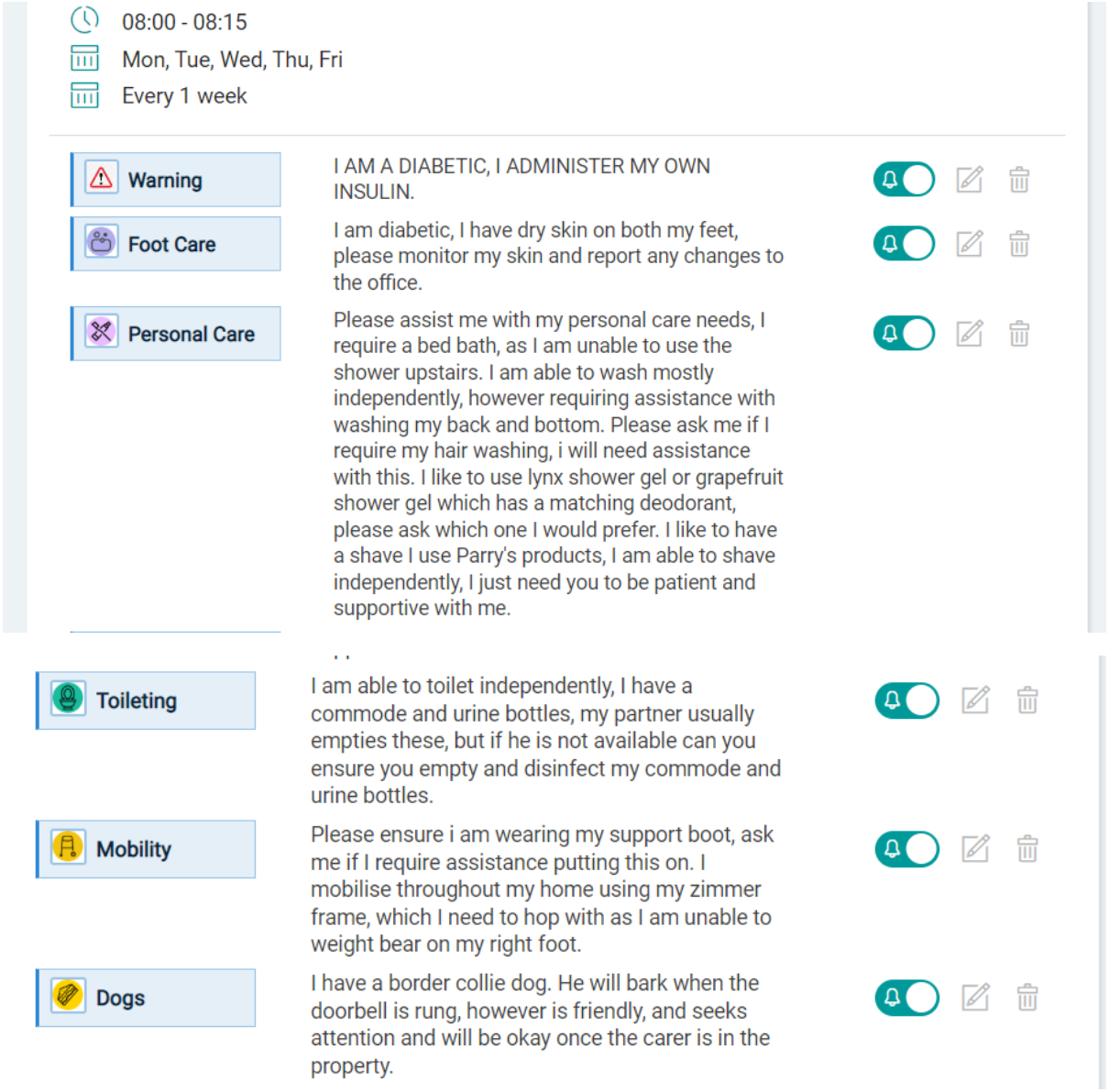


When you click on to the personal care icon, you will be presentenced with this screen:



You can add in all the person centred information that you have gathered from initial assessment in to this section now. Once you have completed this all you need to do is click save, and this task has now been added.

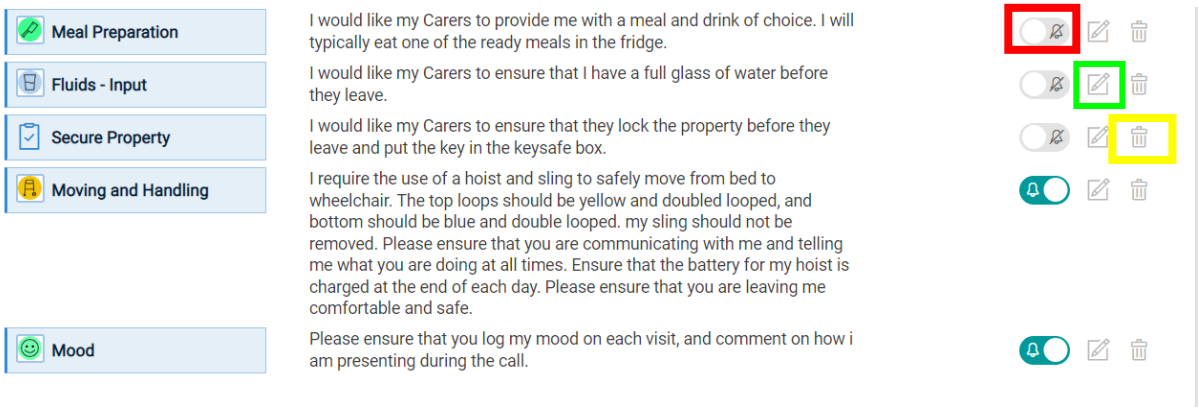
As an example I have completed two sections of this persons care plan.



As you can see it takes in to account the service users preferred products and items that he likes to use to make like easier for the carer so they know what to prompt.

We need to ensure that we have checked what the Local Authority are asking us to do on each visit, or if it is a private / direct payment package, what we are being asked to do at each visit.

Her are some more person centred examples for other tasks we can add across our care plans. Remember the detail needs to be in the documentation, not just a one line instruction as this can lead to mistakes and errors being made.



We can add the task and detail, when adding things about the hoist, ensure that you are including checking the sling, which loops to be used and hoist is charged and in good repair. A line about reporting any concerns or issues is also a point.

If you want Care Planner to alert you if a task has been missed then change the option above to green. [red rectangle]

If you wish to edit the care plan click the green square.

To archive the section of the care plan click the yellow square.

Remember the care plan is the guide that is going to get your carer through their call, so it should be written in plain simple language.

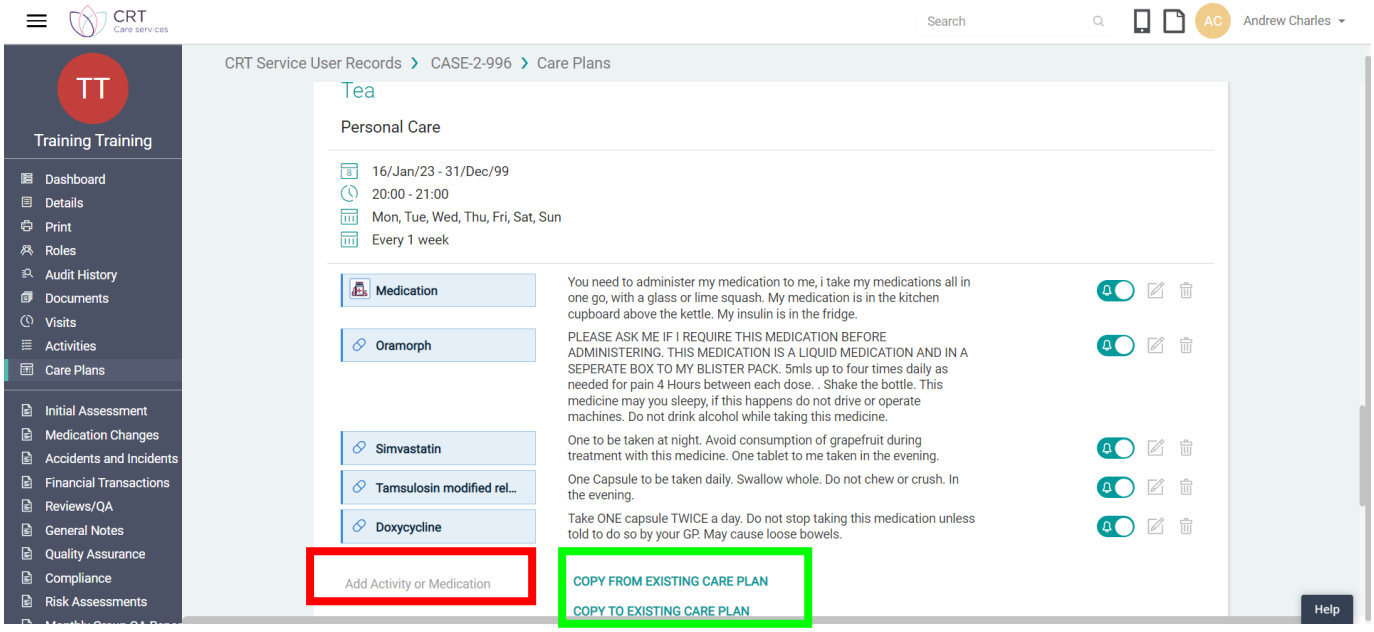
Do not use abbreviations, jargon, or medical text. This is also important as the Service user or their Family / advocate may have access to their care plans and need to be able to read and understand what has been written.

Also sometimes if we write too much information then the carer may not read this, and vital information may be missed.

You can Add Domestic Tasks, to the care plan, but don't add 10 different domestic tasks, keep it simple use one domestic icon and bullet point the tasks required in that section. And ensure that you write the note to explain that they can add what exactly they have done in the end of visit notes or under the task it's self.

If we don't get the initial assessment right from the start then the care plans will also be floored, this leaves us open to safeguarding concerns, and also puts

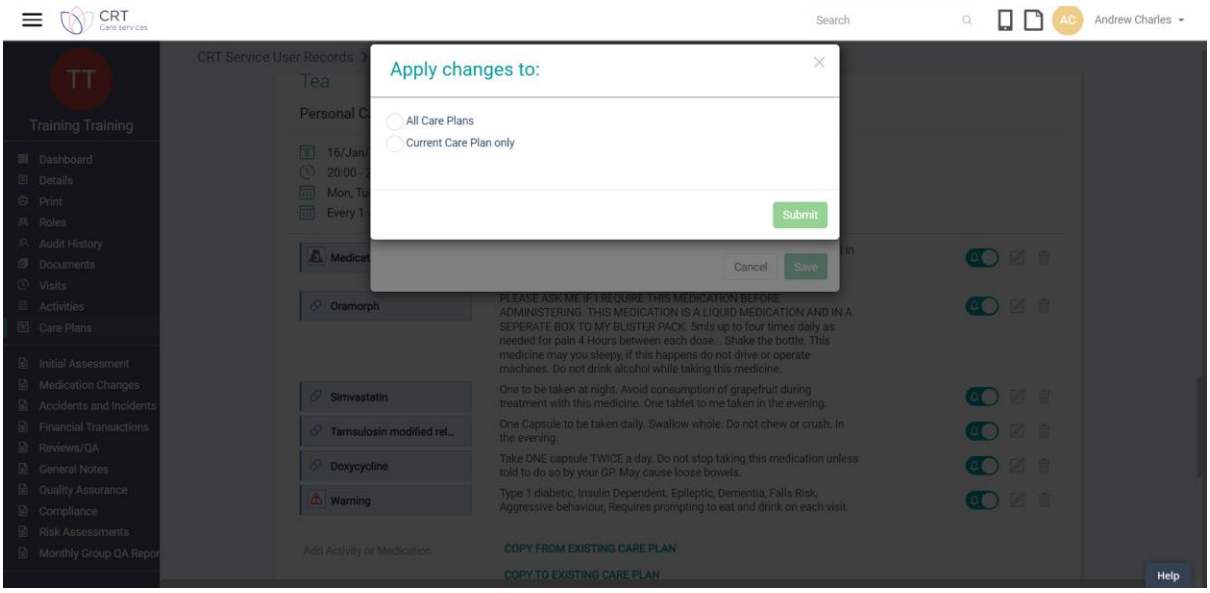
our CQC rating at risk especially in caring, safe and well leThere is another useful function if there is a care plan section that you wish to add to al the visits i.e. warning / critical information then there is a quick way of doing this.



Type the name of the activity you wish to add from another visit in the care plan, here and if there is a task with the same name it will appear at the top and the blank version will appear below. [red rectangle]

You can also use the copy from and to existing care plan options. [green rectangle]

If you need to tweak the option you can do this and the when you edit and click the tick to save it, it will show you the following screen.



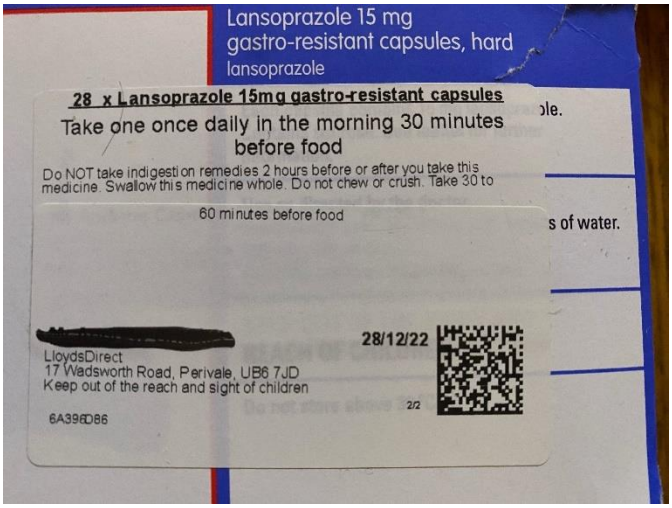
If you want to make the change to all care plans, click all care plans. Just this care plan to update, click current care plan only and click submit.

Adding Medications To A Call

When we add medication, it is important as explained in the initial assessment section that the medication needs to me added exactly as written on the label.

If we are unsure about when medications should be taken we need to talk to the issuing Doctor or Pharmacist to clarify when this should be taken.

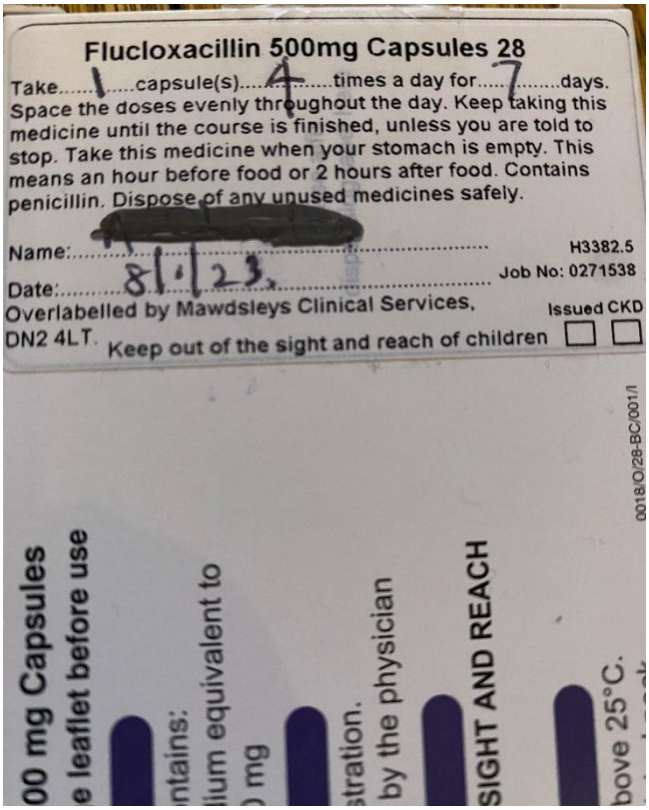
You will need to add a note to the service user record of who you spoke to and what you were told to do. This is to ensure that we have an audit trail, and can demonstrate that we have not made the decision ourselves.

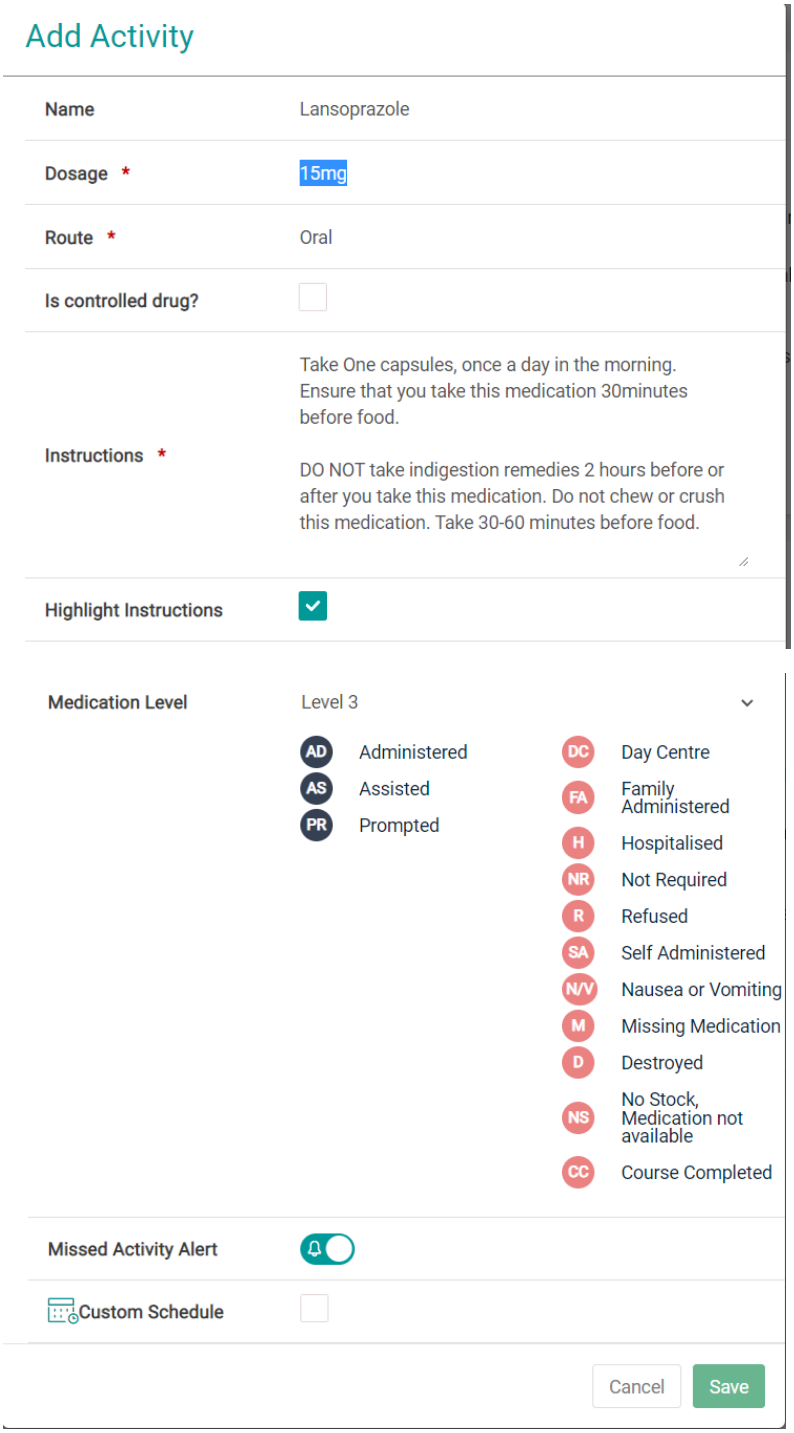


These medications are to be added to the persons care plan.

There is a lot of information that needs to be added, and as you can see one medication is a permanent medication and the other is a interim medication.

To add these medications we are going to use the add activity / medication option and type in the name of the medication, and then select the correct name and then you will see the following screen.





As you can see I have entered all the information from the label in to the correct areas. The name of the medication cannot be altered, you need to ensure the dosage has been entered. Always highlight the instructions so the medication stands out in yellow against a normal task. Also ensure that the missed activity alert option is activated.

The next section is the Medication Level, there are 4 options for this:

1) Level 1 – The service user self-administers their own mediation we may just prompt.

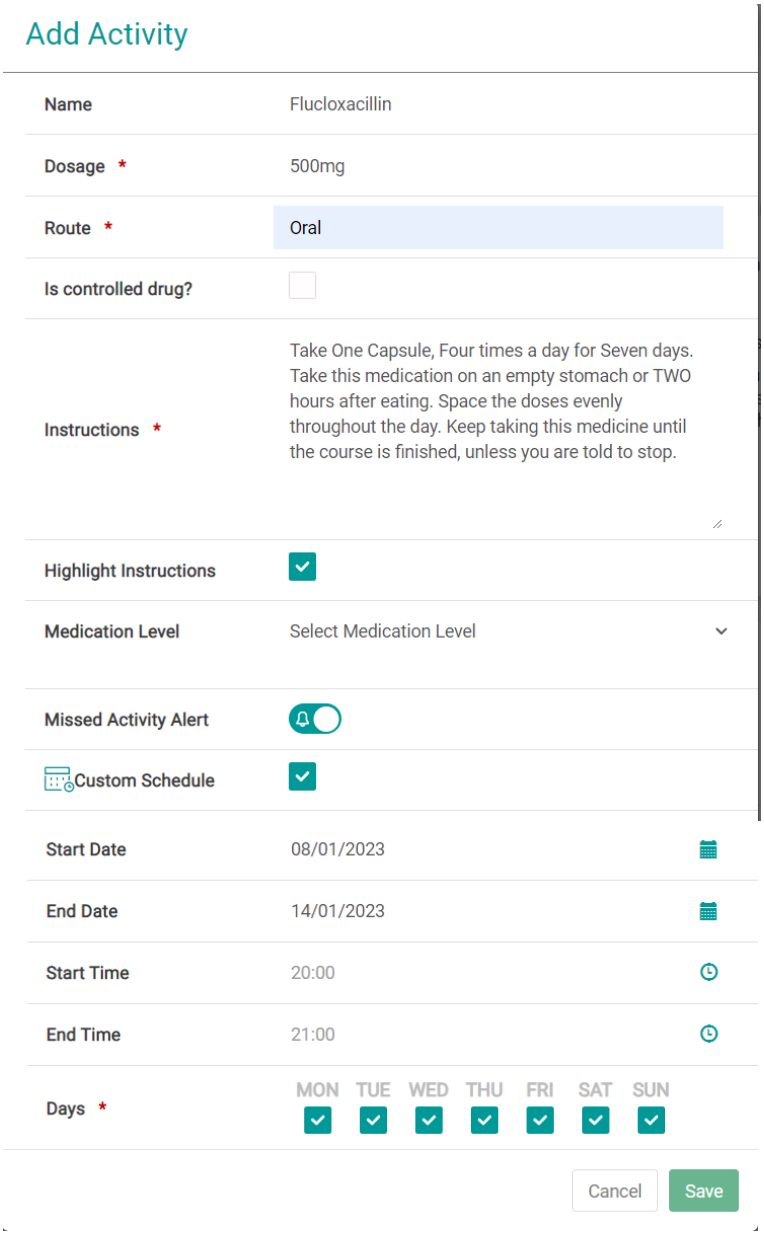
2) Level 2 – Assist, we gather the medication and a drink for the service user and then they administer their own medications.

3) Level 3 – Administer, we control all the medications for the service user and sign when they have been taken.

4) PRN – medications that do not have to be taken all the time i.e., pain relief, anxiety medications, and laxatives.

So if we have identified the level of support the service user needs that match's CRT medication policy, select the level that matches.

Next we are going to look at adding an adhoc medication in this case it is an antibiotic.

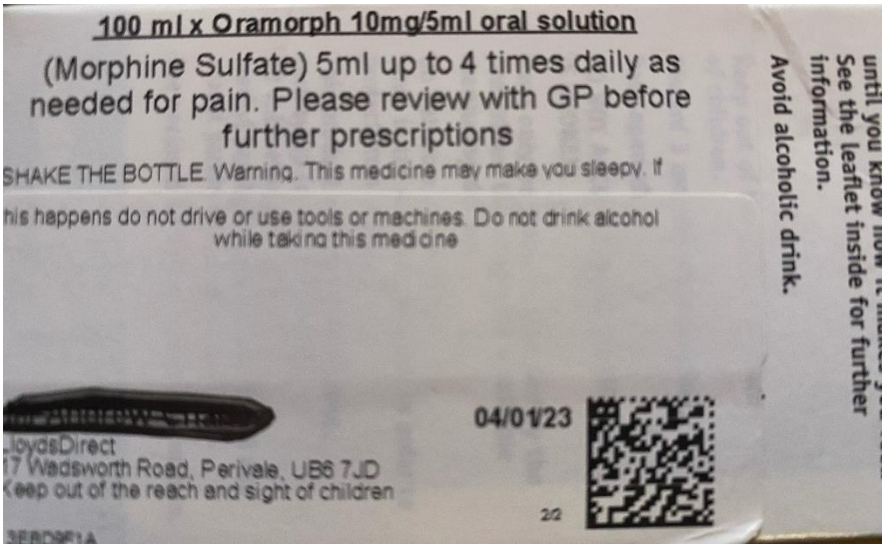


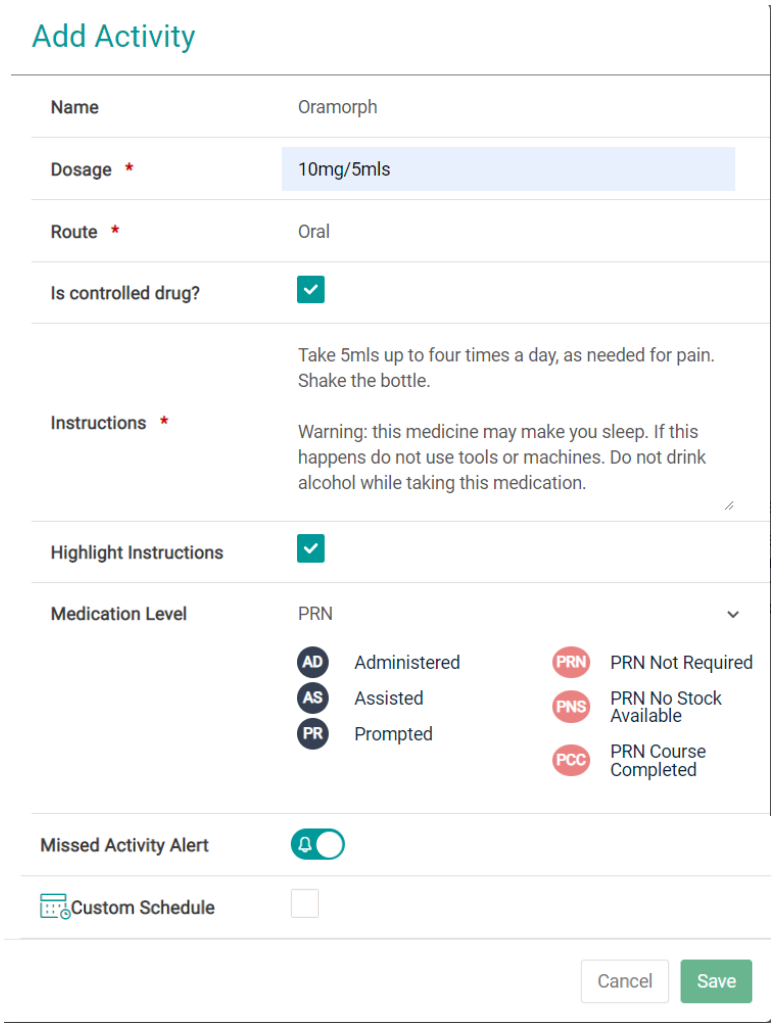
As you can see from the above the relevant information has been entered, the instructions explained. Again, the instructions have been highlighted, and the missed activity is activated.

As the person is only taking the medication for 7 days I have made this a custom medication and put the start date and the end day and highlighted all the days that the medication needs to be given.

This will take the medication off the system after the 8 days and keeps your medications up to date, and no out of date medications are on the system. This makes auditing easier.

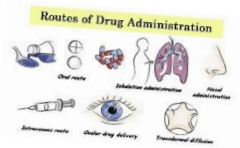
Now we need to add a PRN medication to the system.





If you are unsure if the medication is a controlled drug you can check the controlled drug document, or the packaging of the medication, as this should have on it POM – Prescription Only and CD – Controlled drug. As outlined in the previous sections all the relevant documentation, highlighted and missed alert have been ticked. And there is a PRN instead of the Level 1, 2 or 3.

Routes for Administration



As a reminder these are the following routes for administration

Abbreviation	Where Administered
ORAL	Via the Mouth
S/C (subcutaneous)	Under the skin
IM (intra muscular)	Into the muscle
IVI (intra venous infusion)	Into the vein
PR (per rectum)	Via the rectum
PV (per vulva)	Via vulva / vagina
Topical	Applied directly to affected area
Transdermal	Absorbed through the skin
Sublingually	Under the tongue
Buccal	In to the buccal cavity, between the gum and the cheek, towards the back of the mouth.
Endotracheal	into trachea / Lungs, only used in hospitals when a vein cannot be obtained, due to risk of causing pneumonia.
Intra-articular	injection directly in to the joint to relieve pain. i.e., patient with severe arthritis.

Remember that due to policy, we can not do injections, rectal, or vaginal medications. Highlighted in red above. If there are any medications listed that are delivered in a way that we cannot do and the service user does this for themselves please indicate in the instructions that carers are not to administer the medication for the service user. See example below.

NovoRapid

Fetch The injection which is located in the medicines box in the kitchen and a new needle. take to Andy for him to do his injection. PLEASE REMEMBER STAFF ARE NOT PERMITTED TO ASSIST WITH INJECTIONS.



Classification of Medications

Reminder of medication classifications.

- Prescription only medication (POM)

  Can only be dispensed with a valid prescription, written by a Dr, Nurse or dentist.

- General Sales List Medication (GSL) / Over the counter medication (OTC) These are the same.

  These are medications that can be brought over the counter, in shops / supermarkets.

- Pharmacy Only medications (P)

  Medications that can be brough only in a pharmacy when the pharmacist is present, as legally they have to authorise this. i.e. 500mg Paracetamol & 8mg Codeine tablets.

- Controlled drugs (CD's)

  Medications that can lead to addiction, these medications can only be dispensed on a prescriptions written by a Doctor. Prescriptions that are written for controlled drugs by law have to be written in words and numbers so there can be no mistake over what is being prescribed and dosages.

- Complementary/homeopathic remedies
  Complementary and alternative medicine (CAM) is the term for medical products and practices that are not part of standard medical care.

  Please remember the only medicines we are allowed to administer have to be prescribed, have a label and are in date.

Anticoagulant Therapy / Head Injury

Medication Generic Name	Medication Brand Name
Asprin	Zorprin, Aspirin, Durlaza, Asatab, Adprin-B, Alka-Seltzer Extra Strength with Aspirin, Alka-Seltzer with Aspirin, Arthritis Pain Formula, Ascriptin, Ascriptin Maximum Strength.
Apixaban	Eliquis.
Edoxaban	Lixiana
Foundaparinux Sodium	Arixtra
Rivaroxaban	Xarelto
Warfarin	Coumadin, Jantoven
Acenocumarol	Sinthrome
Phenindione	Dindevan
Heparin	Hep-Lock
Bemiparin Sodium	Ivor, Zibor
Enoxaprin Sodium	Lovenox
Dalteparin Sodium	Fragmin
Tinzaparin sodium	INNOHEP
Dabigatran	Pradaxa
Clopidogrel	Grepid, Plavix
Dipyridamole	Attia, Ofcram, Persantin, Persantin Retard, Trolactin
Selexipag	UPTRAVI®? UPTRAVI®

Medication Generic Name	Medication Brand Name
Cangrelor	Kengrexal
Ticagrelor	Brilique
Cliostazol	Pletal
Prasugrel	Efient

These medications work by altering platelets or thinning the blood to prevent clots from forming.

Due to the service user taking these medications the risk of brain bleed is increased.

The National Institute for Healthcare Excellence states:

People attending an emergency department with a head injury have a CT head scan within 8 hours of the injury if they are taking anticoagulants but have no other risk factors for brain injury.

The care plan should contain information about the medication that has the potential to cause the damage.

So the Warning Care Plan should be used or information written in to the critical information. The Warning is better as it can put in to each visit, and staff can sign to say they have read this.

If we are writing a specific falls care plan the information should be contained within there as well, as well as stating the correct procedures for dealing with falls.

i.e., not moving the person, call the emergency services, and if they are on any of the above medications then they should be taken to hospital to rule out head injury.

End of Life Care Planning

End of life care refers to the care and support people receive in the last days months or years of their life. The NHS considers people to be at the end of life when they have 12 or fewer months left to live. However, making such a prediction isn't always possible.

Palliative care is often included as part of end-of-life care. Palliative care is given to people with incurable illnesses in order to make people as comfortable as possible. It encompasses managing pain, psychological, spiritual and social support, not only for the person at the end of life, but their family and carers too.

Palliative care is said to be a 'holistic approach', caring for someone as a whole person not just treating or managing their illness or symptoms.

End of life care can be delivered in a variety of settings, including care homes, nursing homes, hospices, hospitals and in people's own homes. Where end of life care will take place depends on a person's preferences and their conditions.

What is a good death?

Regardless of where and how people die, all are entitled to receive person centred care, at a high quality. People often see the aim of end-of-life care as ensuring people's comfort and that they have a 'good death.'

A good death typically means the dying person was treated with compassion, dignity and respect, was kept clean, comfortable and in familiar surroundings with people important to them close by.

Essential to ensuring people have a good death, and experience as little distress and pain, as much comfort and joy as possible in their last months, days or years, is developing and documenting an end-of-life care plan.

End of life care plan

Person centred care planning is as important in end-of-life care as in any other form of health and care delivery. End of life care plans are not easy to create, difficult conversations will need to be had about difficult choices.

In addition to the common areas included in a care plan, an end-of-life care plan will often include additional areas such as their preferred care setting should their condition worsen, preferences for symptom management and care after death. Opportunities for discussion should continue so the plan can reflect changes in a person's wishes or needs in their final days.

Some people will already have what is called an advance care plan, which typically include:

- priorities and preferences for care and treatment
- decisions about resuscitation
- views about how and where they would like to be looked
- after in their last days of life
- who they would like to have with them
- any spiritual or religious beliefs they would like to be considered
- who they would like to make decisions for them if they become unable to make them for themselves

Creating a good end of life care plan

A study by Nursing Times developed a highly effective Personalised Care Plan for the Last Days of Life, based on five priorities developed by the Leadership Alliance for Care of Dying People. These Five Priorities of Care of the Dying Person, can help shape your end-of-life care plan approach:

1. Recognise The possibility that a person may die within the coming days and hours is recognised and communicated clearly, decisions about care are made in accordance with the person's needs and wishes, and these are reviewed and revised regularly
2. Communicate Sensitive communication takes place between staff and the person who is dying and those important to them.
3. Involve The dying person, and those identified as important to them, are involved in decisions about treatment and care
4. Support The people important to the dying person are listened to and their needs are respected
5. Plan and do Care is tailored to the individual and delivered with compassion – with an individual care plan in place

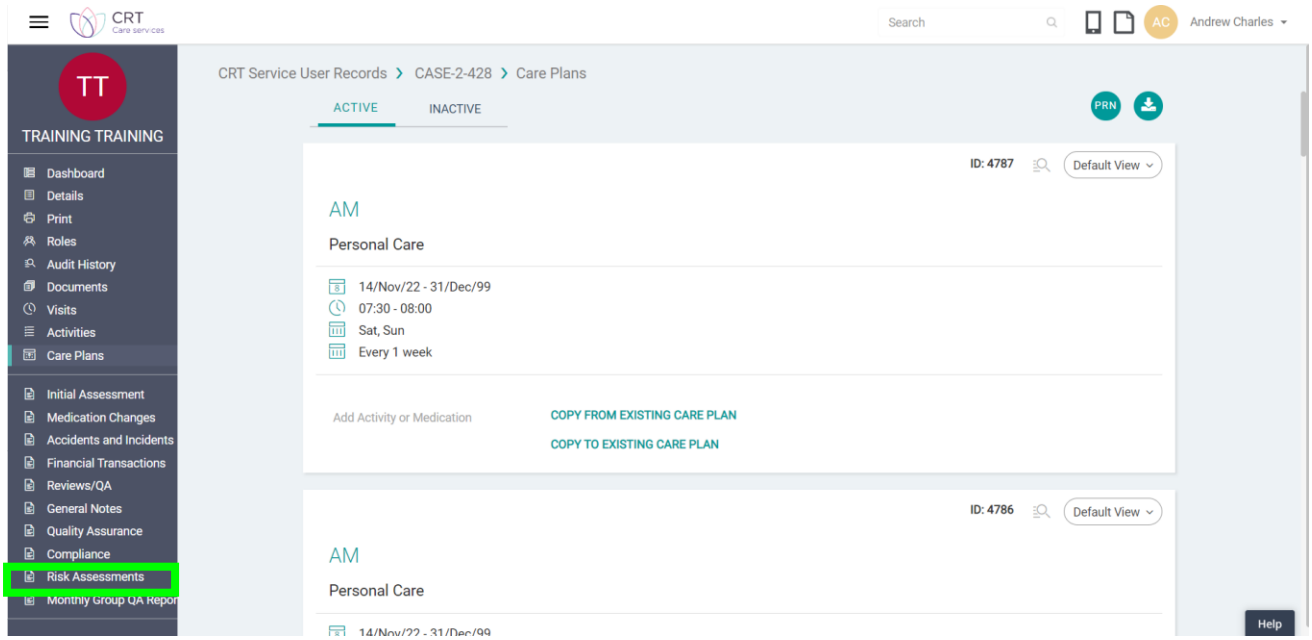
Meanwhile the Social Care Institute for Excellence identified the holistic assessment, as we've already discussed, as being essential to delivering the best end of life care.

We need to ensure that our care plans meet the needs of the service user that is on the end-of-life pathway, and that we are following their needs, and we have the relevant support structures in place to assist with this.

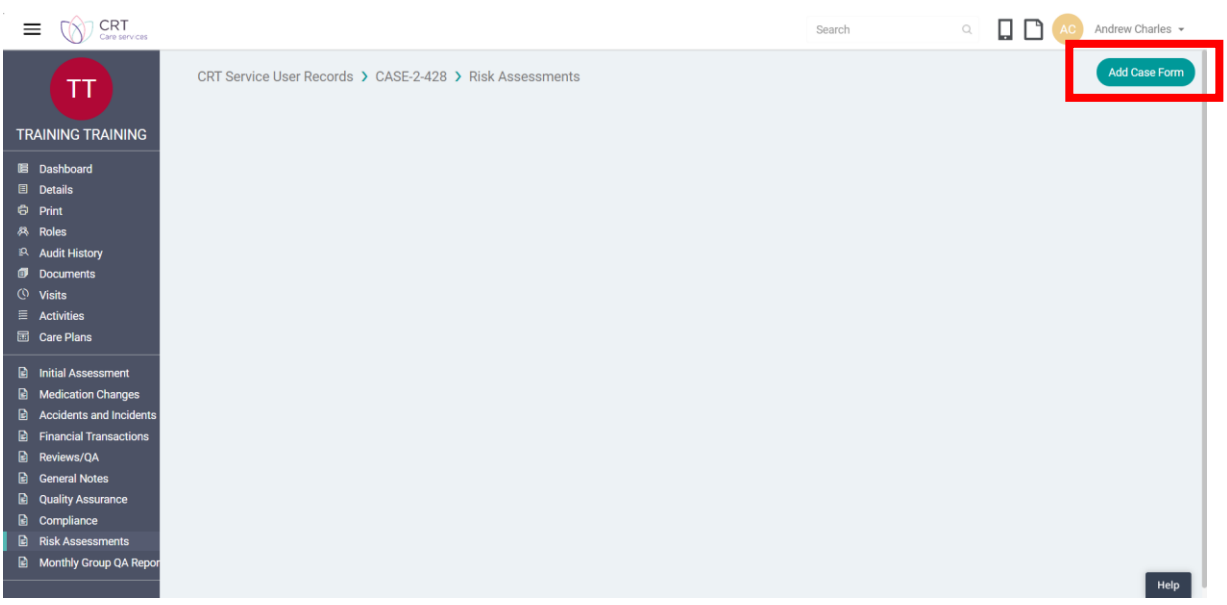
For example, focus may change with the care plan from rehabilitation to maintaining care comfort and keeping someone pain free.

Risk Assessments

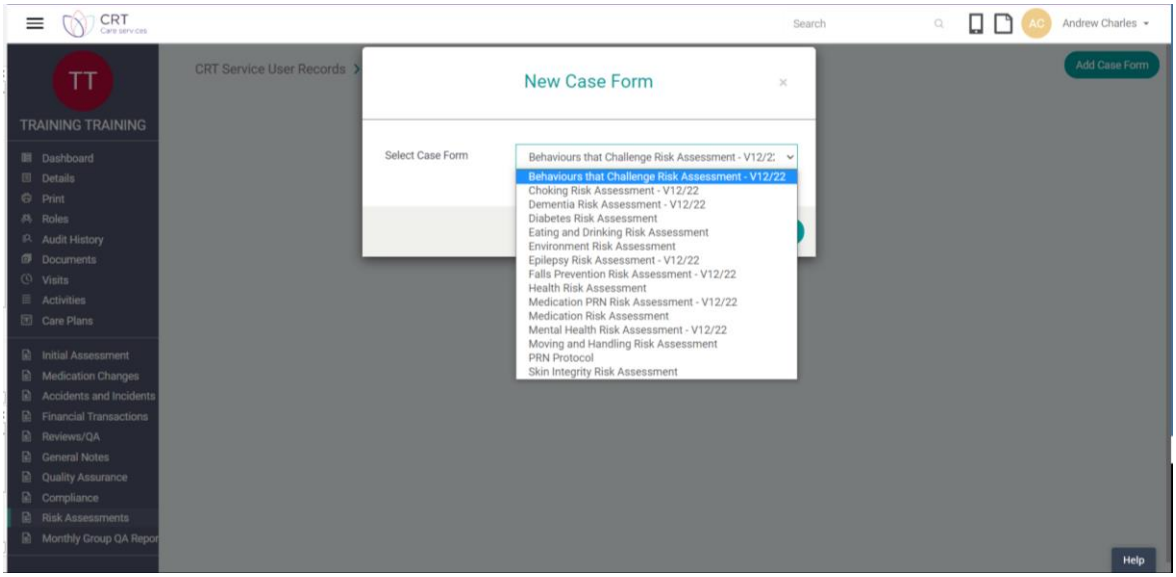
There are risk assessments that we have completed as part of the initial assessment, and there are other assessments that we can complete as part of the care plan to make the assessments more robust.



From the screen above select risk assessments and then you will be taken to this screen.



We then need to click the add case form and you will be taken to the following screen:



There is a list of preprepared risk assessments that you can choose from and complete.

The List includes is completed above.

Select the risk assessment and then click open.

Follow the guidance within each risk assessment and complete for the service user that you are caring for.

Important risk assessments should be:

- Environment
- Falls
- Behaviours that challenge
- Medication
- Moving and Handling
- PRN protocols

When completed, the carers can access the risk assessment from the service user case records on the application.

The risk assessment should backup and support the care plan that has been written.

It should be reviewed regularly, or when a change or accidents occur.

If you are unsure about anything in the risk assessment you can talk to your training manager, compliance manager or operations manager for support.

Appendices 1

Health and Social Care Act 2008 (Regulated Activities) Regulations 2014: Regulation 9

The intention of this regulation is to make sure that people using a service have care or treatment that is personalised specifically for them. This regulation describes the action that providers must take to make sure that each person receives appropriate person-centred care and treatment that is based on an assessment of their needs and preferences.

Providers must work in partnership with the person, make any reasonable adjustments and provide support to help them understand and make informed decisions about their care and treatment options, including the extent to which they may wish to manage these options themselves.

Providers must make sure that they take into account people's capacity and ability to consent, and that either they, or a person lawfully acting on their behalf, must be involved in the planning, management and review of their care and treatment. Providers must make sure that decisions are made by those with the legal authority or responsibility to do so, but they must work within the requirements of the Mental Capacity Act 2005, which includes the duty to consult others such as carers, families and/or advocates where appropriate.

Please see the glossary for important clarification of the terms "appropriate care and treatment"; "needs"; "preferences"; and "relevant person".

CQC cannot prosecute for a breach of this regulation or any of its parts, but we can take regulatory action. See the offences section for more detail.

CQC must refuse registration if providers cannot satisfy us that they can and will continue to comply with this regulation.

Appendices 2

Assessment of needs

Date of assessment:

Name:	
Likes to be called:	
Date of birth:	
Place of birth:	
Gender:	
Ethnicity:	
Faith:	
Preferred language:	
Past occupations:	
Address:	
G.P name & address:	
CPN name & address:	
Social worker name & address:	
1, Next of kin, address, telephone number & relationship:	
2, Next of kin address, telephone number & relationship:	
Where assessment took place:	
Who was involved in assessment:	

Why was assessment requested and who by:

Pen Picture This enables us to provide person centred care not just based on needs. Use your judgement; some people will be happy to tell you things about themselves, others may value their privacy and not want to divulge too much. The following are prompts only and you may prefer to do this section last or at any time during the assessment interview	
How long have they lived in ... (area); where were they before, where did they grow up?	
Who do they live with, do they have children, grandchildren, brothers, sisters etc?	
Do they have pets, what are their names?	
What was/is their employment, did they enjoy it?	
Where have they travelled, where do/did they spend their holidays?	
What are their hobbies/interests, do they go to clubs or groups?	
Do they attend church, are they actively involved in church groups?	
Do they enjoy television, what are their favourite programmes?	
Other – anything else they wish to tell you?	

Mental Capacity	
	Comments
Has an assessment been done by other professionals	Yes ☐ No ☐
Is the person able to make decisions about the care and support they would like to receive and able to contribute to this assessment?	Yes ☐ No ☐
If no, whilst no formal assessment has taken place at this stage, please comment whether there is evident confusion which may impact on the information provided for this assessment	
Please provide the name of the person who provided the information for this assessment or note whether some of the information was gathered from other records	
Is Mental Capacity Assessment required for best interest decision?	Yess ☐ No ☐ Target date:
Religion/spiritual/cultural	
	Comments
☐Language/translation needs	
☐Dress, Modesty & Privacy	
☐ Preferred or required gender of care worker	
☐Festivals and religious observations	
☐Holy books, texts and religious artefacts	

☐Local faith & community Name of minister/chaplaincy	
Communication	
☐Is understood without difficulty ☐Has slight difficulty in making speech understood ☐Has considerable difficulty in making speech understood ☐Relies on methods of communication other than speech (written word, sign language, Makaton etc)	Comments
Hearing	
☐ Hearing is good ☐Slight hearing impairment ☐Significant hearing impairment, uses hearing aids (please note provider/last hearing appointment) ☐Left/right ear hearing impaired (specify prominent side for communication) ☐No useful hearing/no aids	Comments
Vision	

☐Satisfactory eyesight with or without glasses ☐ Problems with vision ☐Registered partially sighted or blind ☐Glaucoma/macular degeneration/cataracts (specify – is there active treatment?) Date of last sight test Who organises this?	Comments Date: Name:
Literacy This section is used to establish the person's abilities as well as whether they require any documents provided in another format	
☐ Is able to read and write ☐ Is able to read and write using easy read publications and documents ☐ Is able to read and write using large print publications and documents ☐ Is unable to read and write ☐ Is able to read but cannot write due to physical limitations (see also section on dexterity)	Comments

☐ Is unable to read and write due to sight impairment (see also section on vision) ☐ Is able to read and write using Braille	

Health conditions Past and current diagnoses with dates where possible (Talk to GP for full health report)
Infections Are there any current infections? (Consider hospital acquired infections (MRSA, C Diff etc); are there recurring infections? Do they have a history of infections?
Allergies Do they have any? If so do they have rescue medication?

Smoking/alcohol How many cigarettes/cigars a day? If given up, when?. How much alcohol is taken each week?
Eating and Drinking Are there any special dietary needs or foods on prescription? are supplements taken or fortified foods/drinks prescribed? Any spiritual/cultural/lifestyle beliefs requiring a special diet? In a typical day how much is drunk? (tea, coffee, water, juice etc.)? Are adapted plates/cups/cutlery required? Is assistance with eating and drinking needed?
Likes and Dislikes and special diets Does diet involve cultural or lifestyle choices (vegetarian/vegan etc)?

Likes:	Dislikes:

Weight Recent loss or gain, appetite, does the person look thin or malnourished?

Weight : Height:

Chewing and swallowing

☐Can swallow food and drink without difficulty ☐Often cough or choke whilst eating or drinking ☐Swallowing is painful or food appears to stick ☐Can only manage a soft or liquid diet ☐Cannot swallow and is fed by other methods ☐ SALT assessment/safer swallowing plan is in place (note the date) ☐ Is a dietician involved	Comments Yes ☐ No ☐ If yes is there a plan in place? If no does a referral need to be made?

Current medication

Name of medicine and dose (include regularly taken homely remedies/vitamins/supplements/homeopathic treatments)	**What is the medication for?** Ask if the person knows what this is prescribed for	**How is it taken?** Please note whether independent, care workers to assist/administer, special technique, covert etc

Ability to take medication	
☐Able to take medicines correctly without help ☐Able to take medicines with support (name who currently helps) ☐Carer/family put my medicines in a 'dossette' box (inform person that CRT care cannot take responsibility for this) ☐The chemist sends medicines in a monitored dosage pack. ☐ The chemist sends medicines in original containers ☐ Medicines are collected from the pharmacy (name who currently does this) ☐Confused about when to take medication ☐Assistance/administration methods (pouring liquids, placing in mouth, applying creams, patches – specify if mix of administration/assistance, via peg etc..) ☐ If regular tests are needed (diabetes/warfarin), who carries out/arranges these test ☐Date last saw GP for medicine review (if unknown is it on a repeat prescription)	Comments

☐Special considerations such as fasting or vegetarianism that need be considered? Is a medication risk assessment required?	 Yes ☐ No ☐
Condition of skin Note eczema, psoriasis, ulcers, pressure sores or wounds?	
☐No current skin problems ☐Is not mobile putting pressure areas at risk ☐One or more areas of discoloured or sore skin ☐One or more areas of broken Skin (specify whether skin tears, pressure/diabetic ulcers, open wounds etc) ☐Already has pressure relieving equipment in place (specify what) ☐ A district nurse attends to my dressings/skin ☐Is prescribed cream/lotion (who applies?) ☐Currently uses/like to use a cosmetic/over the counter skin cream/moisturiser (who applies?) ☐Incontinent putting pressure areas at risk	Comments Yes ☐ No ☐ If yes what is the risk? Name: Tel:

Is a pressure care (Waterlow) assessment in place? Name and contact of health care professional (D/N, specialist)	
Breathlessness	
☐Is rarely/never breathless ☐Is breathless on exertion (dressing, mobilising, activity etc) ☐Is always breathless; has not seen GP ☐Is always breathless; has seen GP who is treating this (see medication section) ☐Can manage own inhalers (complete medication assessment)	Comments

Feet	
☐No problems/manages own foot care ☐Minor problems affecting one or both feet ☐Major problem affecting one/both feet (specify) ☐Sees a chiropodist regularly (who, if not, would they like to?) ☐Diabetic podiatrist (name/frequency/last appointment)	Comments
Oral health	
☐No problems with teeth or gums ☐Dentures (specify top plate, bottom plate, both) ☐Does not like to wear dentures – why? ☐Able to care for own teeth/dentures ☐Unable to manage own mouth care (specify support needed)	Comments

☐When was last dental appointment? (who makes appointments?)	

Personal Care	
☐Can manage own personal care ☐Can manage part of personal care (specify ie: can wash hands and face but needs assistance with back and feet etc) ☐Can manage own make up Jewellery ☐Needs assistance with makeup, jewellery ☐Can manage own shave ☐Needs assistance with shave ☐Can manage own hair care ☐Needs assistance with hair care ☐Can manage own finger nail care ☐Needs assistance with finger nail care	

Bathing and showering	
☐Can manage to bathe and/or shower independently	Comments

☐Needs assistance with bathing and/or Showering ☐Is totally dependent on another person for bathing and/or showering ☐Is unable to bathe and/or shower even with assistance; what is the alternative (strip wash/bed bath etc) ☐Prefers a bath (how often) ☐Prefers a shower (how often) (see also moving and handling section)	
Dressing	
☐Can dress and undress self ☐Can dress and undress self but finds it difficult and tiring/gets muddled ☐ Needs help to dress and undress ☐Is totally dependent on others for dressing and undressing	Comments
Toilet	
☐ Can use the toilet independently ☐ No problems remaining continent of urine	Comments

☐ Needs help to use the toilet (specify orientation, mobility, dressing, hygiene etc.)	
☐Uses a commode at night next to the bed	
☐ Is totally dependent on others when using the toilet	
☐ Has been assessed and uses pads (specify size/day/night, how many allocated and whether private purchase also needed)	Yes ☐ No ☐ Target date:
☐ Sometimes has problems remaining continent of urine and wear pads for comfort and security (how these are supplied or purchased?)	
☐Uses a sheath/bottle or other aid when in bed (males only)	
☐Has a catheter which is well tolerated with no problems (indwelling, supra-pubic, convene, obtain name of health practitioner who manages this)	
☐Has a catheter which is poorly tolerated and gives rise to problems	
☐Has no problem maintaining bowel control	
☐Is often constipated and requires regular laxatives (how often are laxatives taken?)	
☐Is rarely able to maintain bowel Control (is stoma in situ, who manages this?)	

☐Has an ostomy (specify who manages this and what is required, identify healthcare professional responsible) Is referral to the Continence Advisory Service/District Nurse required	
Mobility	
☐Is fully mobile without aids ☐Walks with aids independently (please specify whether stick, tripod, frame etc) ☐Walks with assistance of carer/care worker (please specify one or two, describe assistance required) ☐Walks with assistance of aids and carer/care worker (please specify one or two carer/worker, which aids and describe assistance required) ☐Uses mobility aids and/or require assistance when outdoors/indoors (please specify) Is dependent on a wheel chair which is: Self-propel ☐ Dependent on an assistance to propel ☐ Electric and self-operated ☐ A scooter outside ☐ ☐Can climb steps or stairs	Comments Yes ☐ No ☐ Not sure ☐ Date: Yes ☐ No ☐

☐Cannot climb steps or stairs ☐Uses a stair lift to manage the stairs ☐I am immobile and remain in bed Has an OT assessment been carried out? (Request details/copy/when this was done? Please note, if a person has equipment in their home they have probably been assessed as needing it. If it is evident that equipment was purchased privately, please request copy of use and maintenance manual) Is moving and handling risk assessment required?	
Ability to transfer	
☐Can transfer independently ☐Can transfer with help of one person ☐Can transfer with help of two people ☐Uses equipment to transfer (specify – stand-aid/rotunda etc) ☐Needs hoist to transfer (what size sling?) Which loops should be used for each transfer? ☐Is unable to transfer and remains in bed	Comments

☐I can get in/out of bath/shower independently ☐I need assistance getting in/out of bath/shower (please specify type of help/aids required) Last tine Hoist was Serviced: Last time Sling was serviced:	
Dexterity	
☐ Has no concerns about manual dexterity ☐ Has left/right sided weakness (specify how this affects the person's dexterity) ☐ Has no useful movement in arms/hands ☐ Has arthritis/rheumatism causing pain and making movement/dexterity difficult/painful ☐ Is able to hold a cup/cutlery/pen/and manage buttons/clips/zips etc	

Steadiness	
☐ Is seldom or never dizzy or unsteady ☐Is dizzy or unsteady in certain situations ☐Is often dizzy or unsteady (cause unknown) ☐Is often dizzy or unsteady and GP is treating this (see medication section)	Comments
Falls	
☐No falls in the last year ☐One fall in the last year ☐More than one fall in the last year ☐Has fallen in the last year sustaining a fracture/injury/hospital admission ☐If uninjured, is able to get self up from chair independently or with guidance using chair/stairs/leverage (see moving and handling assessment) Is a falls risk assessment required? Refer to Falls clinic (via GP)	Comments Yes ☐ No ☐ Yes ☐ No ☐

Chronic pain	
☐Is never or seldom in pain ☐Is sometimes in pain ☐Is constantly in pain ☐Takes pain relieving medication regularly/when required (how often? see medication section)	Comments
Sleep	
☐ Seldom or never has trouble sleeping ☐Occasionally has trouble sleeping ☐Has a longstanding sleep problem ☐I take sleeping/sedative medication Does anything aid sleep? Light on/off, music, reading, etc	Comments
Mental Health	

☐Mental health diagnosis (specify if known) ☐Hallucinations/delusions ☐Depression ☐Self Harm ☐ Eating disorder ☐ Obsessive compulsive disorder ☐ Violence ☐ Post traumatic stress disorder ☐ Dementia ☐ Anxiety ☐ Bi-Polar ☐ Personality Disorder Name of consultant Name of CPN	Comments (impact on lifestyle) Name & number: Name & number:
Confusion and orientation Difficulties paying attention or concentrating? Involved family concerns? who is concerned and why, orientation to time and place	
	Comments

☐Feels confused ☐Does not feel confused ☐Is oriented to time and place ☐ Has evident confusion (how is this evident?) ☐Has diagnosis of a condition causing confusion (such as dementia, multiple sclerosis) Ask the person if they know the day or date and what time it is (write comments opposite)	
Memory	
☐Has no concerns about memory ☐Friends/family have concerns about memory ☐ Has evident memory problems (how is this evident?) ☐Has a diagnosis of a condition causing confusion (such as dementia, multiple sclerosis, alcoholism, Parkinson's – please note name of consultant/health professional)	Comments
Social contact and leisure activity	
☐Sees friends and/or family frequently	Comments

☐Feels socially isolated ☐Is dependent on others for transport ☐Is housebound ☐Has one or more regular leisure pursuits (please note clubs, church, hobbies etc) ☐Has given up important leisure pursuits due to poor health or loss of motivation ☐Is content with current level of activity ☐Would like help accessing local clubs/day centres/shops/leisure activities Past hobbies and interests (list)	
Household	
☐ Can manage own housework (cleaning, laundry etc) ☐ Can manage own shopping	Comments

☐ Needs help with housework (specify what help; cleaning, laundry etc or all) ☐ Needs help with shopping (if yes, refer to financial section below) ☐ My family/carer does my shopping ☐Can manage all aspects of meal Preparation ☐Is dependent on others for meal preparation ☐Has community meals delivered	
Finance	
☐Manages own finances ☐Manages own finances with assistance ☐Requires care worker to assist with shopping and manage finances in relation to this (specify arrangements for handling money; remember, PIN numbers are <u>not</u> to be given to care workers or recorded) ☐Has appointed power of attorney/guardian/solicitor Copy of documents seen / on file	Comments Name: Tel:
Care support network	

☐ Is independent and arranges own support ☐Is supported by a main carer (family/private carer) who lives with me ☐Is supported by a main carer who lives elsewhere ☐Has no main carer ☐Is receiving one or more care service (community meals, live in care, other care provider) Name of main carer and the names of any other care provider/agency	Comments Name: Contact Tel: Name: Contact Tel:
Death and dying - This is a sensitive topic but it would help us to know whether you have considered any of the following	
☐Is there a living will/ an advance decision regarding wishes for the future ☐Arrangements have been made, solicitor/next of kin knows of the existence and location of will and advance decisions	Comments

☐Has not considered funeral planning and would appreciate some support/advice ☐Would like to be resuscitated in the event of heart failure ☐Would NOT like to be resuscitated in the event of heart failure (discuss in relation to agreed practice and need for GP to sign DNAR; if DNAR already in place, note who holds this, who signed it and the date it was signed)	

Other information:

Appendices 3

CRT
Care services

Case	TRAINING TRAINING
Case ID	CASE-2-428
Date of Birth	01/Jan/21
Printed By	Andrew Charles
Printed Date	12/Feb/23

TT

CRT

Email	-
Telephone	-
Website	-

Details

Last updated by Andrew Charles at 12/Feb/23 09:37

Fluid Target (in ml)	N/A
Area	Bolsover
Initial Assessment Due	Completed
Name of Assessor	
Initial Assessment Completed	23/Nov/2022
72 Hour Review Due	26/Nov/2022
72 Hour Review Completed	
6 Weekly Review Due	04/Jan/2023
6 Weekly Review Complete	
6 Monthly Review Due	25/May/2023
6 Monthly Review Complete	24/Nov/2022
Preferred Name	
Critical Information	Allergic to Ibuprofen and Diclofenac. DNAR / RESPECT FOM IN PLACE
NHS Number	
Date of Birth	01/Jan/2021
Gender	

Address Line 1	MERCURY HOUSE
Address Line 2	
Address Line 4	GAINSBOROUGH
County	
Postcode	DN21 1DY
Primary Telephone	07411754333
Secondary Telephone	
Commencement of Support Date	
Preferred Language	
Outline of Medical Background	
Support Required	
Key Safe Number	1234
Access Details	The house is located at the very end of the street and the keysafe is on the left side of the door.
DNR in use	Yes
Meds Safe Number	4563
Meds Safe Location	Medication Safe Located in the kitchen. Secured to the wall next to the microwave.
Power of Attorney	
Court of Protection	

Emergency Contacts (APP)	#1 Name: Andy Charles Address: Foxby Lane Lincolnshire Email: andrew.charles@crtcare.co.uk Telephone(Main): 07411754333 Relationship: NOK - Other Instructions:

Allergies

Active Care Plans

AM

Personal Care

ID: 4787

14/Nov/2022 - 31/Dec/9999

07:30 - 08:00

Sat Sun

Every Week

AM

ID: 4786

Personal Care

14/Nov/2022 - 31/Dec/9999

08:00 - 08:15

Mon Tue Wed Thu Fri

Every Week

Warning	I AM A DIABETIC, I ADMINISTER MY OWN INSULIN.
Foot Care	I am diabetic, I have dry skin on both my feet, please monitor my skin and report any changes to the office.
Personal Care	Please assist me with my personal care needs, I require a bed bath, as I am unable to use the shower upstairs. I am able to wash mostly independently, however requiring assistance with washing my back and bottom. Please ask me if I require my hair washing, i will need assistance with this. I like to use lynx shower gel or grapefruit shower gel which has a matching deodorant, please ask which one I would prefer. I like to have a shave I use Parry's products, I am able to shave independently, I just need you to be patient and supportive with me.
Toileting	I am able to toilet independently, I have a commode and urine bottles, my partner usually empties these, but if he is not available can you ensure you empty and disinfect my commode and urine bottles.
Mobility	Please ensure i am wearing my support boot, ask me if I require assistance putting this on. I mobilise throughout my home using my zimmer frame, which I need to hop with as I am unable to weight bear on my right foot.
Dogs	I have a border collie dog. He will bark when the doorbell is rung, however is friendly, and seeks attention and will be okay once the carer is in the property.
Medication	I am able to take my medication independently, please check I have taken these, and prompt me to take them if I haven't done so already.
Codeine	
Paracetamol	

TWO tablets to be taken FOUR times a day

Take TWO tablets FOUR times a day. No more than 8 tablets in 24 hour period.

PRN up to FOUR times a day

AM

ID: 1521

Personal Care

06/Dec/2021 - 31/Dec/9999

09:00 - 10:00

Mon Tue Wed Thu Fri

Every Week

Bisoprolol	one to be taken daily
Clopidogrel	one to be taken daily
Finasteride	one to be taken daily
Levothyroxine	one to be taken before breakfast
Levothyroxine	one to be taken before breakfast
Morphine	to be taken in the evening

Custom Schedule

Start Date: 15/Apr/2022 **End Date:** onwards **Days:** Mon Tue Wed Thu Fri

Paracetamol	Take two tablets 4 times a day. No more than 8 tablets in 24 hour period.

Ramipril	one to be taken each morning
Meal Preparation	

I would like my Carers to provide me with a meal and hot drink of my choice I typically like two slices of toast and a cup of strong tea with milk. I do not take sugar in my tea as I am diabetic.

Custom Schedule

Start Date: 15/Aug/2022 **End Date:** 17/Aug/2022 **Days:** Mon Tue Wed

take one tablet twice a day

Wash	I would like my Carers to assist me into the bathroom and help, where needed with getting washed. I can usually manage to wash myself but may need help reaching my back.
Mobility	I need support to mobilise and move around my home. Staff to ensure that my footwear is fitted correctly, and i am comfortable. Staff to ensure that my Zimmer frame is in good repair. and i have this with me at all times. Staff to encourage me to stand using ready, steady stand ensure that i have my balance before moving off.
Mood	Please ensure that you log my mood on each visit, and comment on how i am presenting during the call.
Stool	

If I have my bowels please document using the bristol stool chart as a gui

Initial Assessment

1.1 Initial Assessment

Last updated by Craig Topham at 24/Nov/22 14:19

Submitted Form Id: 2806476

Personal Information

Title
Mr

Service User Name:
TRAINING TRINING

Known as:
Preferred name(s)
top hat

Address: 28 the
park ave bolover
chesterfield
drebshire

Postcode: cfc
1886

Contact Number(s):
07771112223

Date of Birth:
28/Jan/1950

Spoken Language engilsh

My Individual Religious/Cultural needs and communication:
How I prefer to communicate, religion and visual needs etc. non
none

Assessment Information

Assessor craig
topham

Date of Assessment
23/Nov/2022

Were representatives provided with the opportunity to attend?	Relationship to Service User
Yes	jo bloggs

Present at Assessment: craig
topham jo bloggs training
training

Funder:
Local Authority

Please specify funder If Other is selected:

Care Worker Preference/Compatability:

Allergies:
I have a reaction to penicillin by no means i should take this

Medication Level
Administer - Level 3

Please list all medication including self-adminstered medication paracetamol 500mg/ tramadol 200mg/ampicillin20mg/fenbid gel 200%/amlodipine100mg

Contact Details

Please state all relevant contacts below:

Contact Details

Name of Contact:	Relationship	Number:	Address:	Email Address:
jo blogggs	NOK		10 hightfield road	jobloggs@tiptop.co.u k

In case of Emergency (Severe Weather/Epidemics)

- Red

Please state first contact in emergency situations jo bloggs

This is me

Who am I and what is important to me?
E.g. Where was I born, Where did I live and grow up? What was my occupation?

i love watching football on the weekend i like a bet to its really important to me that im not disturbed

What hobbies and interests do I have?
E.g. I like attending the day center two days per week

cross word and cards

What makes a good day for me?
A good day for me is when I go out with my Family?

my family come on sundays with my dinner it important to me i see my family together

My Goal Setting

My Goals *E.g. my goal is to improve my health*	How I will achieve my goals *E.g. with lots of encouragement from my family and care staff*	Potential Barriers *E.g. Health Concerns*	Time Frame *E.g. 6 Months*

Further Information

The Mental Capacity Act 2005:

The Mental Capacity Act 2005 includes the Deprivation of Liberty Safeguards (DoLS) a set of checks that aims to make sure that any care that restricts my personal liberty is both appropriate and in my best interest

COSHH and Riddor

I would like my care staff to adhere to Dale Cares policies and procedures around the handling of hazardous substances.

Cleaning products are in correct individual containers and are under the kitchen sink.

Potential risks are to be reported to the office.

All procedures in relation to COSHH and Riddor have been clearly explained to me during my intial assessment

INFECTION CONTROL MEASURES -COVID19

Carers have been trained to comply with policies and procedures relating to Infection Control measures. Should you consider that a carer is not complying to good infection control measures please inform the office immediately.

Carers have been issued with all PPE (masks, aprons, gloves, sanitiser) company policy dictates that all are worn while assisting with your care.

Carers are trained to wash hands thoroughly and we would request that there is ample supply of washing facilities.

Masks should be worn throughout calls at all times as an additional precautionary measure.
All PPE should be disposed of in the bin outside the property.

Where a Service User has tested positive PPE should be double bagged following every call, dated AND DISPOSED OF IN THE BIN OUTSIDE AFTER 72 HOURS.

Donning and Doffing should be followed at all times.
Signs and Symptoms of Covid
Temperature
Lack of Smell and Taste
Headache
Consistent Cough
SHOULD YOU OR YOUR FAMILY MEMBER DISPLAY ANY OF THE ABOVE PLEASE CONTACT THE OFFICE IMMEDIATELY.

Advance Decisions DNR (DO NOT RESUSCITATE) and Lasting Power of Attorney

Is an Advance Decision / Living Will in situ?

Yes

Where is the DNR located and what does this cover ?

my dnr is on my kitchen table with my notes if i have to go into hospital

Is there any other decisions to refuse (CPR, Resuscitation or Treatment)
Please specify all that are included, what they cover and where these are located

no my wises are not to be resuscitate

Is there a Lasting Power of Attorney in place?
Yes

Select all applicable POA options:
(Nothing selected.)

Contact Information
Contact information for the above:

jo bloggs

My Care Plan

My Health and Current Wellbeing
(For further information check section 1 of the Risk assessment)

I remain happily at home i walk with zimmer and have a stair lift to take me up and down stairs i have a two zimmer one down stair and one up stairs

Capacity, Decision making and deprivation of Liberty, Safeguarding:
The mental capacity act 2005 includes the deprivation of Liberty Safeguards (DoLS) - a set of checks that aims to make sure that any care that restricts a person's liberty is both appropriate and in their best interests.

yes i can make decision for myself

Trigger points to alert to potential changes:
If there are any concerns over the below trigger points then medical advice should be sought, inform line manager or out of hours and get advice from GP. to report to office and on call or management

My Medication Requirements
(For further information check section 2 of the Risk assessment)

i can not do my own medication i need carers to do this for me or family

Nutrition (Dietary needs or preferences)
(For further information check section 3 of the Risk assessment)

My nutrition needs are normal and no problems with eating i do love sweet thin/s so with every meal i would love a pudding

My Personal Care Needs
(For further information check section 4 of the Risk assessment)

I like to remain independent as much as possible.I like to get ready in my bedroom in a morning i only need assistances with standing and walking with my zimmer to bathroom and sitting on the toilet,I like to wash and part dress my self in the bathroom but i will need help with my bottom half with dressing and putting my belt on then i'm ready to go down stairs on my stair lift i need the carers to sort my medication and sort my breakfast out for me i like Weetabix's in a morning with a coffee 2 sugars

Mobility and Equipment / Methods
(For further information check section 5 of the Risk assessment)

I have two zimmers and a stair lift

Financial Arrangements
Also see - Lasting Power of Attorney section

I can sort my own money out at the moment and family can get me money out at my request if needed

Company Financial Policies and Policies (Please confirm below that all policies and procedures have been explained to the Service User)
It is our main objective to ensure that you are protected from any form of financial abuse and therefore we have developed robust policies and procedures. It is within this policy that we request that no employees are provided with a bank card unless you are in attendance when purchasing any shopping or withdrawing money. Please ensure that you do not provide your PIN number to any members of staff, All staff are required to complete a financial transaction document at all times following any transactional activity.

Yes

Finances: Potential Risks/Risk Assessments

Potential Risks/Low/Medium/High	Risk Prevention

low	

Risk Assessments: Section 1: MENTAL AND PHYSICAL HEALTH

Communication

Speech *Please select:*	Comments/Devices
Good	

Hearing

Please select:	Comments/Devices
Good	

Eyesight

Please select:	Comments/Devices
Fair	

General Health

Breathlessness *Please select:*	Comments
Independent	

Urinary Incontinence

Please select:	Comments
Use of Equipment	i use a urine bottle down stairs in the day and if i need the toilet i can go up stairs on my stair lift

Faecal Incontinence

Please select:	Comments
Independent	

Mental Health Assessment

Memory
Please select one from the dropdown

Good

Moods
Please select one option from the dropdown menu

Stable

Orientation
Please select one option from the dropdown menu

Full

Sleep Pattern
Please select one option from the dropdown menu

Fair

Behaviours
Please select one option from the dropdown menu

Good

Mental Capacity Assessment

Does the Service User understand the information provided? Yes

Can they retain the information long enough to make a decision?

Yes

Can they contemplate and discuss the pros and cons of the decision or action?

Yes

Can they communicate their decision by any means? Yes

If the answer is 'Yes' to all, the Service User has capacity.

if the answer is 'No or partially' to one or more of the above, please provide details of the person who supports with decisions in the Service User's best interest.

Actions/Comments/Discussions

Potential Risks/Risk Assessments

Potential Risks/Low/Medium/High	Risk Prevention
stair lift breaking down	make sure its serviced 6 monthly and on charge at all visits

Section 2: MEDICATION ASSESSMENT

Do you order and collect your own prescriptions? If not, who does?
The carers order them every month

Do you use a dosage system to assist you to take your medicine? If yes, record the system.

Who issues and maintains this system?
Discuss policies and procedures All
office staff

Where is the medication stored? And how?
On top of my dresser in lounge, the rest are stored in the dresser

Are you currently undergoing any investigations, treatment that may result in changes to your medication?
not at moment

Do you have any surplus/out of date medication currently stored in your property?
Discuss policies and procedures.

no my family take it back to phy

Do you have any concerns about your medication? no

Do you take regular homeopathic, herbal medicines or over the counter medications?
Discuss policies and procedures

no

Are you currently taking any PRN medication?
Discuss policies and procedures.

no

Do you have any problems with the following?

Picking up tablets and placing them into your mouth? YES

Opening medication boxes or bottles
NO

Opening and pouring liquids
YES

Using dosage system
NO

Applying creams and squeezing tubes
NO

Swallowing tablets
YES

Understanding the patient information leaflet or medication instructions? if yes, advise to consult GP/pharmacist or read it to them.
NO

Managing inhalers
YES

Instilling eye/ear/nose drops
NO

Additional Medication

Controlled Drugs: Name/Strength/Dose

Warfarin: Is it stable? Day Nurse attends/Bloods taken? Where is dosage documented and stored?
nurse updates yellow book every tuesday and every friday

Medication: Risk Assessment

Potential Risks/Low/Medium/High	Risk Prevention
high	we administer the medication

Section 3: NUTRITION ASSESSMENT

Do you require assistance from care staff with meal preparation?
Yes

Normal Diet

Please select:	Details of assistance required & risks involved
Yes	

Special Dietary Requirements

Please select:	Details of assistance required & risks involved
No	

Food Allergies

Please select:	Details of assistance required & risks involved
No	

Soft Diet

Please select:	Details of assistance required & risks involved
No	

Intolerance of food

Please select:	Details of assistance required & risks involved
No	

Tube Feed

Please select:	Details of assistance required & risks involved
No	

Supplementary foods/thickeners

Please select:	Details of assistance required & risks involved
No	

Food Preferences : Likes and Dislikes
I like bean egg on toast which is my favourite meal

Food Preferences : Portion Sizes
I like a normal portion of food fill my plate with normal amount of food
Food Preferences : Preferred time for main meal
I preference my main meal at lunch time

Fluid Preferences : Fluid Intake
I like a coffee with two surges and a pint of juices with all meals

Nutrition: Potential Risks

Potential Risks/Low/Medium/High	Risk Prevention
none	

Section 4: EQUIPMENT ASSESSMENT
Equipment

Equipment	Supplier *List name of supplier*	Contact Number:	Prev. Service Date	Next Service Date
Lift	mick meade	07875456456	1/2/1987	6/7/1987

Action/Comments (lack of equipment/faulty equipment)
Care workers are to monitor and report any concerns regarding my equipment.

carers to report any changers

Equipment - Potential Risks

Potential Risks/Low/Medium/High	Risk Prevention
medium do to walking with zimmer	

Section 5: MOBILITY ASSESSMENT

Weight Bear

Please Select	Details of assistance required & risks involved
Uses Equipment	

Walking

Please select	Details of assistance required & risks involved
Uses Equipment	zimmer

Use of stairs/steps

Please Select	Details of assistance required & risks involved

Uses Equipment	stair lift

Sit to stand

Please Select	Details of assistance required & risks involved
Independent	

Washing

Please Select	Details of assistance required & risks involved
Requires Assistance	carers to assist in washing

Dressing

Please Select	Details of assistance required & risks involved
Requires Assistance	require assistants with dressing bottom half

Oral Care

Please Select	Details of assistance required & risks involved
Independent	

Bath/Shower

Please Select	Details of assistance required & risks involved
Requires Assistance	family at this time are showering me remains that way at moment

Toilet/Commode

Please Select	Details of assistance required & risks involved
Requires Assistance	I need assistants in walking and sitting down on toilet i use a bar to get up

Use of Wheelchair

Please Select	Details of assistance required & risks involved
N/A	I use a wheelchair to go out with family but they deal this at this time

Transfers

Please Select	Details of assistance required & risks involved
Independent	

Move in bed

Please Select	Details of assistance required & risks involved
Independent	

In/Out of Bed

Please Select	Details of assistance required & risks involved
Independent	

Chair to Wheelchair

Please Select	Details of assistance required & risks involved
N/A	

In and Out of Car

Please Select	Details of assistance required & risks involved
Requires Assistance	my family help in and out of car at this time

Other transfers

Please Select	Details of assistance required & risks involved
Independent	

Able to access the community

Please Select	Details of assistance required & risks involved
Requires Assistance	my family take me out in my wheelchair when required

Use of Specialist Equipment

Please Select	Details of assistance required & risks involved
N/A	

Can be left alone

Select one:	Comments
Long Period	I remain independent at this time but still need help with my meals and medication

Prone to falls

Select one:	Comments
Occassionally	

Mobility: Potential Risks

Low/Medium/High	Risk Prevention
medium	walk with zimmer frame

Section 6: ASSESSMENT OF ENVIRONMENT ENABLING

Cooker
Fit for use in Care Plan

Microwave
Fit for use in Care Plan

Central Heating
Fit for use in Care Plan

Iron
Not included in Care Plan

Kettle
Fit for use in Care Plan

Toaster
Fit for use in Care Plan

Open Fire
Not included in Care Plan

Tumble Dryer
Fit for use in Care Plan

Free Standing Heaters
Not included in Care Plan

Floor Coverings
Fit for use in Care Plan

Hand/Grab Rails
Fit for use in Care Plan

Working Space
Fit for use in Care Plan

Cleaning Materials
Fit for use in Care Plan

Storage of Cleaning Materials Fit for use in Care Plan

Stairs
Fit for use in Care Plan

Surfaces
Fit for use in Care Plan

Electric Blanket
Not included in Care Plan

Bed
Fit for use in Care Plan

Plug Sockets
Fit for use in Care Plan

Electric Cables
Fit for use in Care Plan

Washing Machine
Not included in Care Plan

Smoke Alarms/Carbon Monoxide Detector
Fit for use in Care Plan

Actions Required:
If concerns are highlighted above:

Smoking Policy (Policies and procedures explained) Yes

Health and Safety

Fire Drills	Fire Hazards	Shared Access	Use of Pendant

Safe Working Practices - External Access

Please select:	Risk Assessment
Front	Fit for Purpose

Clear Pathways:

Please select:	Risk Assessment
Front	Fit for Purpose

Hand/Grab Rails

Please select:	Risk Assessment
Front	Fit for Purpose

Entrance Steps:

Please select:	Risk Asssessment
N/A	N/A

Lighting:

Please select:	Risk Assessment
Front	Fit for Purpose

Ramps:

Please select:	Risk Assessment
Rear	Fit for Purpose

Personal Security:

Please select:	Risk Asssessment
Front	Fit for Purpose

Parking:

Please select:	Risk Assessment
Side	Fit for Purpose

Actions Required:
If concerns are highlighted above:

Environment Enabling - Potential Risks

Low/Medium/High Risk	Risk Prevention *Please add notes*
low	

Section 7: SECURITY, ENTRY AND EMERGENCY ACCESS

Means of Access: *Select One*	Type of Property *Select One*	Enter Using *Select One*
Key	House	Front Door

Location of fuse box: above the front door on the left

Location of Stop tap/mains: under sink in kitchen

Location of gas shut off valve: in front room in cupboard next to tv

Security/Access - Potential Risks

Potential Risks/Low/Medium/High	Risk Prevention
low	

Consent to Care and Data Protection notice:

Following joint discussions, I give my consent to recieve the care and support that I require as stated in my agreed care plan.

I have been openly encouraged to participate in the care that I need and my preferences and choices have been discussed and agreed.

Addtional Documentation explained and provided:

Welcome Information Guide

Do you know who to contact if you wanted to raise an official complaint?

I (my representative/family) have been directed to the infomation booklet and it has been clearly explained of what procodure I/they should follow should I/they want to raise a complaint/safeguarding.

All emergency contact details have been provided and I am aware to contact the office if nesscessary (my family and respresentative have been provided with the infomation)

Cancellation of Visits

We understand that from time to time there may be occasions when you do not require our services, e.g. visiting friends of family for the day.

To prevent you being charged for the visits that have not taken place, we kindly ask that you give us 24 hours notice of cancellation.

Any visits where the cancellation period has not been given will be charged for at the usual rate.

I have read, understood and agree with the documentation jointly discussed in this Care Plan.

Date Completed:
24/Nov/2022

Name of Assessor: craig topham

Signature of Assessor:



1.2 Consent to Care

Last updated by TRAINING CAREWORKER-DUMMY ACCOUNT at 14/Apr/22 10:50

Submitted Form Id: 1250083

Consent to Care

Following joint discussions, I give my consent to receive the care and support that I require as stated in my agreed care plan.

I have been openly encouraged to participate in the care that I need and my preferences and choices have been discussed and agreed.

Complaints and Cancellations

Complaints Information

I (my representative/family) have been directed to the information booklet and it has been clearly explained of what procedure I/they should follow should I/they want to raise a complaint/safeguarding.

All emergency contact details have been provided and I am aware to contact the office if necessary (my family and representative have been provided with the information).

Visit Cancellations

We understand that from time to time there may be occasions when you do not require our services, e.g. visiting friends or family for the day.

To prevent you being charged for the visits that have not taken place, we kindly ask that you give us 24 hoursâ€™ notice of cancellation.

Any visits where the cancellation period has not been given will be charged for at the usual rate.

Documentation Explained or Provided:)

Please select all that apply

Welcome Guide

Complaints Procedure

Safeguarding Information

Date of Completion:

14/Apr/2022

Signature of Service User:

By signing this form I understand and agree with the statements above



Signature of Office User:



1.3 Data Protection Policy

Last updated by TRAINING CAREWORKER-DUMMY ACCOUNT at 14/Apr/22 10:51

Submitted Form Id: 1250091

Policy

Care Relief Team Limited, which trades as CRT (the â€œCompanyâ€?) and any other Group Company* will, where necessary for the effective provision of services to you, use the information it holds on you, whether from third parties or yourself, for the provision of safe care services to you. We may disclose your information to other professionals and third parties for these purposes for example the local authority, Social Services, your GP, hospital, emergency services and the District Nurse etc.

The Company (or Group Company) will from time to time also need to make some information available to legal and regulatory authorities (such as to CQC and to Her Majestyâ€™s Revenue and Customs) for legal and regulatory purposes and to sub-contractors, our staff, accountants, auditors, lawyers, other outside professional advisers and business contacts, and to other parties which provide products or services to the Company (such as IT systems suppliers) for the effective provision of services to you. This is necessary for the performance of our contract with you and/or the local authority, for the performance of our legal obligations, and in some cases necessary for our legitimate interests. It may also be necessary for the provision of healthcare, social care or treatment to you, in your vital interest or for reasons of substantial public interest. Alternatively, we may seek your explicit consent to the same.

We seek to ensure that your data is adequately protected and we will not retain it beyond what is necessary and in accordance with our data retention schedule. You have a right to request a copy of the personal data we hold about you and to correct any inaccuracies in your information. You also have the right to erasure, a right to restriction of processing, data portability and a right to object to our processing.

* â€œGroup Companyâ€? means the Company and its subsidiaries or holding companies from time to time and any subsidiary of any holding company from time to time where â€œsubsidiaryâ€? and â€œholding companyâ€? are defined in section 1159 of the Companies Act 2006 and a company shall be treated for the purpose only of the membership requirement contained in subsection 1159(1)(b) and (c), as a member of another company even if its share in the other company are registered in the name of (a) another person (or its nominee), whether by way of security or in connection with the taking of security, or (b) a nominee.

More information about all of the above is available in our Detailed Privacy Notice if required.

I give consent to allow photos to be taken and stored according to the data protection policy

Yes

Date of Completion

14/Apr/2022

Signature of Service User:

By signing this form I understand and agree with the statements above



Signature of Office User:



1.4 Fire Safety Referral

Last updated by TRAINING CAREWORKER-DUMMY ACCOUNT at 14/Apr/22 10:58

Submitted Form Id: 1250143

Fire Safety Referral

Type of Visit:

Fire Risks

Does the Service User have an increased fire risk?

No

Alarm Reactions

Would the Service User be less able to react to an alarm? Yes

Criteria

Please check all that apply

- Other

Mobility

Would the Service User have reduced mobility to escape? Yes

Criteria

- Restricted Mobility (frail or history of falls)

Equipment

Are there any smoke or heat alarms fitted in the Service Users' property? Yes

Comments:

Please include location, who maintains them and what type of connection (fixed or battery) In the ceiling middle of the room.

Are there any carbon monoxide alarms fitted in the Service Users' property?

No

Further Information and Consent

If there are any questions in sections 1â€“3 that have been answered â€˜Yesâ€™, or you have identified that there are no smoke/ heat alarms fitted, or if they are broken or poorly sited, this suggests there is a risk from fire. Immediate referral is required to ensure agreed safety measures are in place:

If you are a family member or an informal carer: Contact your local Fire Brigade to arrange for a free home fire safety visit:

- 01332 777850 Derby City and South Derbyshire
- 01159 326832 Erewash and Amber Valley
- 01246 223500 N-East Derbyshire, Chesterfield & Bolsover
- 01298 608720 High Peak and Derbyshire Dales
- Email reception@derbys-fire.gov.uk
- Web https://www.derbys-fire.gov.uk/keeping-safe/homefire-safety-check

As an employee you must ensure to:

Return this check list to your manager for a fire service visit referral to be conducted where necessary.

- Gain informed consent from the resident or other family members, (with LPA). Communicate the risks identified, if you are certain they will understand
- A copy of this document should be added to the care plan, and notes made in that plan as to the progress of the referral.

Ensure appropriate partnership referrals are made as required, e.g. local council, social worker and local fire service.

Based on the information gather in this from, it is prudent to have a fire specialist assess the situation and provide help where may be required or necessary. We would like your permission to pass your name contact number and address on to the local fire services to allow them to contact you or your designated next of kin, to ensure your ongoing safety from fire. We will not pass your details on to any other party or group and your information, once consent has be given, will only be used to in regards to your fire safety.

Please complete the below declaration if you wish us to make the fire safety referral on your behalf;

I would like my details passed to the local fire service for a fire safety referral;

Date of Completion:

14/Apr/2022

Signature of Service User:
By signing this form I understand and agree with the statements above



Signature of Office User:

